

**USING NONDUALITY TO AMELIORATE EARLY
CHILDHOOD TRAUMA: AN AUTOETHNOGRAPHY**

Nonduality in Acton – A Universal Healing Method

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Using Nonduality To Ameliorate Early Childhood Trauma: An Autoethnography

Nonduality in Action—
A Universal Healing Method

Judith Orloff

Abstract

The subject of this autoethnographic study is twofold. The first part of this study explores how early childhood trauma, whether emotional, physical, or sexual, can define the course of a person's future, leading to both physical and psychiatric disease. The second part explores a gap in a foundational core therapeutic model that could be used universally with all modalities of therapy, and potentially educational and social services training that could allow the healing of the traumatized. Through the use of a relational model based on nonduality, a Buddhist non-conceptual model of healing suffering, much of early childhood trauma caused by abusive behavior of parents could be ameliorated.

This study is focused on a woman in the second half of the 20th century, with postpartum depression and psychosis leading to a misdiagnosis which resulted in labels and inpatient psychiatric hospitalization. This autoethnography is the story of how a survivor of early childhood trauma achieved resilience and success and applied nonduality in a coaching, therapeutic, and consultative practice.

Key Words: NONDUALITY, POSTPARTUM DEPRESSION, PSYCHOSIS, EARLY CHILDHOOD TRAUMA, RESILIENCY COACHING, RELATIONAL THERAPY

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NONDUALITY IN ACTION

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Dedication

This dissertation is dedicated to all those who have struggled to overcome trauma and abuse,
whether it be sexual, physical, or emotional.

And

to my loving partner Jonathan, and my magnificent family. Without you all, this
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Table of Contents

CHAPTER ONE – INTRODUCTION TO THE STUDY.....	1
Background of the Study.....	2
Nonduality	4
Purpose of the Study.....	7
Theoretical Framework for the Study.....	13
Discussions of Both Non-Conceptual Nonduality and Conceptual Mindfulness.....	15
Difference Between Conceptual and Non-Conceptual Buddhist Mindfulness.....	18
Definition of Nonduality.....	23
Use of Nonduality as a Therapeutic Modality	27
Definition of Duality.....	29
Origin of Nonduality	29
Significance of this Study.....	32
Limitations of this Study.....	35
CHAPTER TWO – REVIEW OF LITERATURE	37
Introduction	37
Nondual Practitioner Scholars	38
Early Childhood Conditioning	54
Mindfulness	62
Mentors and Teachers: The Development of My Own Perspective.....	69
Summary.....	75
CHAPTER THREE - AUTHOETHNOGRAPHY AS METHOD.....	77
Overview.....	77

NONDUALITY IN ACTION

Research Focus and Data.....	77
Philadelphia Psychiatric Hospital.....	79
A Few Memories	82
Research Approach - Autoethnography	83
Summary.....	93
CHAPTER FOUR - MY PERSONAL JOURNEY.....	97
My Parents' Marriage.....	97
A Loving Tale About my Father and Me at 2 Years Old and Later.....	99
St. Agnes Hospital.....	101
Three Years Old.....	105
The Red Mitten.....	105
Seven to 12 Years Old	107
My Father's Heart Attack.....	111
Empath.....	112
Cinderella -- From a Journal, May 2012.....	116
Trolley Car.....	118
Elementary School.....	119
A Chance for Private School and Abuse	120
Ramona.....	123
Being Jewish in Philadelphia.....	124
Taking Sides -- The Beginning of Nonduality... ..	126
Teenager.....	129
Journal Entry, 1985.....	134

NONDUALITY IN ACTION

Teenager, continued.....	134
New York City.....	135
Philadelphia.....	138
Horizon House – My Healing Continued.....	140
University of Pennsylvania.....	147
Vermont – A New Reality.....	148
A Fly Cries.....	152
San Diego – A Paragraph.....	153
Move to Colorado.....,	156
Discovering Nonduality and Peter Fenner.....	157
The Last 12 Years 2006-2018.....	160
My Healing.....	161
CHAPTER FIVE – NOW WHAT? CONCLUSIONS AND RECOMMENDATIONS.....	164
Memories.....	165
Suggestions for Future Research.....	166
Implications for Practice	168
Example of Nonduality in Action.....	172
Final Reflection.....	175
REFERENCES.....	177

LIST OF FIGURES

<i>Figure 1</i> - Distinguishing Nondual Awareness from Conditioned Experience _____	22
<i>Figure 2</i> - The First Night Entering Philadelphia Psychiatric Hospital _____	81
<i>Figure 3</i> - Mom and Dad on Their Wedding Day _____	99
<i>Figure 4</i> - Judith as a Newborn _ _____	100
<i>Figure 5</i> - Judith and Carole Ann _____	104
<i>Figure 6</i> - A Moment of Happiness Except For My Eyes _____	112
<i>Figure 7</i> - Teen-Aged Punk _____	128
<i>Figure 8</i> - Beatnik Days in the Big City _____	135
<i>Figure 9</i> - Adult Judith and Mother _____	137
<i>Figure 10</i> - Choices International in Local Newspaper _____	155
<i>Figure 11</i> - Marriage to My Soulmate _____	157
<i>Figure 12</i> - Six Steps to Nonduality in Action _____	169
<i>Figure 13</i> - Another View of Six-Step Process _____	171

CHAPTER ONE: INTRODUCTION TO THE STUDY

For the benefit of all is the only way I know to save my own life. (Orloff, 2012, p.9)

In this study I investigate how early childhood trauma can define the course of a person's future, particularly women in the second half of the 20th century. This study is called for because there is a need for a foundational model that could be used universally in conjunction with all models of therapy. Also needed is educational training that allows for the healing of traumatized persons in a relational model that could ameliorate much of the early childhood trauma most of us experience. This introduction will explore trauma within the theoretical framework of nonduality (Fenner, 2007). Nonduality has been the theoretical framework of my life for the past 12 years as well as an integrated system of thinking about the world, human experience, and early childhood trauma. Nonduality is that which, while "fully accepting our finite and conditioned existence, we rest in a level of consciousness that can't be damaged or degraded by the presence of any thought, feeling, or sensation" (p.2).

I have chosen autoethnography as my research approach because through this lens I can share my healing process while looking at the larger, greater, socially relevant issues of healing. It is my hope that it will inspire others to see themselves as okay just the way they are. I believe it is from that perspective that transformation can occur.

This dissertation is the culmination of my search for liberation and freedom from the pain and lack of wellbeing brought on by a severely traumatic childhood. For much of my life I believed I was the cause of my pain and that I deserved the negative

treatment from my parents. When I left home at 19 and moved to New York City, I began a spiritual journey that ultimately led me to Fielding Graduate University. Beginning doctoral studies was a challenge in itself. During my studies I often asked myself, “What are you doing?” My answer and mantra each time has been a variant of knowing I am doing this work to save my own life through the healing and sharing of nonduality, for the benefit of all, a Buddhist non-conceptual practice. I call this nonduality in action.

It is my contention that nonduality is one essential pathway to revealing, clarifying, and mitigating early childhood trauma. Ira Glovinsky stated the following in a personal communication:

I see trauma as not being reversible because they are real events that happened to us. I am not sure you can make something that happened, not happen. I do think that therapy can help to manage the remnants of trauma, but I don’t see how you can take the trauma away. (August 2018)

Even so, I hope this autoethnographical study has the opportunity to show how it is possible to heal every day and, while doing so, allow the brain to create new pathways that take precedence over the old trauma-infused ones.

Background of the Study

Much of my life has been a continued search to understand why my childhood was so brutal and violent. Was I evil, as my father suggested when he was about to physically hurt me? Of course not. Was I “making him hurt me?”, as my mother told me? Of course not. Was I just a “big mouth child” who deserved to be stopped? Again, of course not. Well then, what could it be? I certainly believed he meant what he said. I searched and searched to discover what I did to cause his behavior as my mother

told me. I became an educator and psychotherapist with the outward reason to heal others, but my deepest desire was to find the reason my family was so physically and psychologically violent toward me.

I later discovered that my family's behavior had many aspects to it, depending how it was viewed, such as my father's myriad illnesses, including a heart condition, diabetes, arthritis, possibly high cholesterol—and more that I do not know about—that necessitated my father taking medication that changed his personality. They included Seconal, Prednisone, opioids, and other heart and various pain medications. There was a distanced relationship between my parents that was attributed to my father's gambling and relationships with other women (one of whom I met while living in New York). Both my parents used insults and name calling toward me when they were agitated. Some of the names, such as "harlot," verged on sexual aggression. I will elaborate further in my review of literature in Chapter 2 and Chapter 4 of my autoethnography.

For the past 45 years I have been exploring therapeutic approaches to mitigate early childhood trauma. Childhood messages stay subliminally hidden, unknowingly creating deeply reactive responses to situations that may unconsciously be reminiscent to the person or group of what their childhood conditions were. Survival strategies learned as children take center stage for the moment. The amygdala memories are expressed as an either/or way of thinking. These encapsulated and inaccessible memories take over, and as Jacobs (2009) has discussed, one goes into a "trance state." A trauma survivor cannot see what is occurring at that moment, but rather these experiences take the survivor unconsciously back to the traumatic event as if it were happening in the present moment. Working individually or with a group of children and adults, I have been able

to teach a process of self-awareness that allows people to have countless choices rather than succumb to a survival-based, either/or, trance approach of viewing the world (Jacobs, 2009; Orloff & Doubleday, 1991). Nonduality is the therapeutic modality I discovered in 2009 while participating in two, year- long programs and 6 years of nonduality training with Peter Fenner, PhD., a nonduality teacher from Australia, and other spiritual teachers who use nonduality as their modality.

Nonduality

Nonduality is a Buddhist non-conceptual (without feelings, thoughts, or mental concepts) process of healing suffering that encompasses the core essence of Buddhist, Hindu, Jewish, and other religious practices. The roots of nondual awareness, as experienced on the journey toward enlightenment, stretch deep into many ideologies throughout the course of human history. These include Hindu Vedanta, most schools of Buddhism and Taoism, mystical Christianity, Judaism and Islam (Prendergast, Fenner & Krystal, (2003). Non-dual awareness is experienced as the sense of a dissolved separate self and a fresh sense of connectedness and oneness that encompasses the extraordinary and the ordinary as one and the same without the need to contrast and compare. (Wilber, Engler & Brown, (1986), Wilber, Engler & Brown (1986) asserted that oneness attributed to nonduality can be viewed as a complete integration of formlessness and form that embraces all levels of consciousness so that an individual can enjoy all levels and let go of attachment to “existence,” no matter how it shows up. In essence, nonduality as a realization of who and what we are is a process where one comes to experience oneself and others as pure awareness (Fenner, 2007). It sounds so esoteric, but it is really so

practical. Nonduality as a state of consciousness has the power to relieve and heal fears of death in illness, fear of being alone in relationships, and fear of insanity.

In a practical sense, nondual practices can reeducate the brain to relax and, through this relaxation, be an aid to the mind, allowing negative thoughts and feelings to lose their power (Fenner, 2007). Blackstone (2006) described nonduality as the experience of oneness with all that is. She stated the “essence of an individual’s being is the same as, unified with, or dissolved in the essence of the cosmos” (p.26). When Blackstone discussed “oneness,” she meant oneness with the entire universe. I take that to mean an experience of merging with what one is experiencing at that moment.

When I was 28 years old and had taken LSD, I was on a cross-country trip with my boyfriend and we were staying in a log cabin on the Klamath River in Oregon. The cabin was rustic, with a deer head hanging in the living room. We took the LSD and after a while the wood in the cabin seemed to move and the deer head seemed to cry out to me. I felt so ashamed that someone hung a deer head on the wall and couldn’t stand how I felt. I went outside toward the river. It was a very short walk, perhaps 50 to 75 yards.

What happened next created a new paradigm for me. (See Michael Pollan’s book, <https://www.amazon.com/Change-Your-Mind-ConsciousnessTranscendence/dp/1594204225>. I experienced the earth breathing. And, I was walking on its body. Could I break the heart of the earth? Could I hurt the earth by walking on it? Was I the earth? Was I the broken heart I projected on the earth? These were the thoughts and questions I had at that moment of “oneness.”

It is a natural tendency as we evolve emotionally and spiritually to begin a radical inquiry into the question of “who am I?” Although I have come to live with the illusion that I am

separate many times during a day, I have had enough moments of nondual experiences to trust the realization that I am not separate and in fact nothing is. Watching small children begin their life in what I observe, and experience is nonduality, in the sense that within an hour or two they are experiencing what appears to be many emotions and states of being without pretense, and then seeing them begin to shift to a separate self, used to be exciting for me. Now, I recognize the possibility of teaching that separateness is not possible, but being unique is how we make our differences in the world. This realization can be used to look at the trauma of early childhood with a clear eye and open heart simultaneously. These were the thoughts and questions I had at that moment of “oneness.” From that moment I recognized that the earth was alive. I know that may not sound as profound as I experienced it, but that realization shifted how I treated all life. The acid wore off in a few hours, but my consciousness of that experience did not and grew to include all sentient beings. Yet, I had not given up the experience of a separate self until my work with nonduality. I have had that experience of oneness more and more through my work with nonduality without drugs, and it has fortified my resolve and mission to do this work at Fielding Graduate University.

By exploring nonduality as a healing model that can reach across time and culture, this study complements that which has come before me and fills a gap that will encourage other scholars and professionals in their work with early childhood trauma. We all deserve to be free to live a life of love.

Purpose of the Study

Without understanding emptiness, or nonduality, it is not possible to cut the root of human problems. (Lama Thubten Yeshe, 2001, p.34)

My purpose is to bring to light and to the mainstream this healing process so that individuals and groups can find a way to heal themselves from trauma-based childhood hurts and wounds. Many different therapy models are working diligently to understand and release trauma. Some of the pitfalls, in my opinion, are that many of us do not wish to risk projecting our hurt on our families and the external environment. We have learned to be excessively cautious in how we speak of our experience. S., S. H. (2010) posited the dangers of political correctness are the dynamics that underlie it (p.17). I use the term *political correctness* to mean descriptive language, policies, or measures that are intended to prevent offense or disadvantage to members of specific groups in society. S., S. H. (2010) claimed there is a resultant self-destruction of the core processes in an organization (p.48).

I experience that political correctness makes difficult and intimate conversations awkward and abstract, and our resultant confusion allows for miscommunication and easily creates a disabled culture, just as a family's miscommunication can create a disabled family. The limited language "allowed" makes it very difficult to share an experience of an individual's reality. In so doing, the world is redefined (p.197). Also, I experience deep separation with those who have felt the everyday racist undertones of our culture. I remember being in Paris in 1960 and hearing and observing how the French White people characterized and treated the Algerians in their midst. I did not see myself as part of that dynamic at the time. That is no longer true. The argument that, for me, allows me to be silenced is that I will never know what it is like to be a person of color in our society, and I will never be able to understand what that pain is. However

true that may be, the resultant experience is separateness and continued trauma. The language and communication methods we presently use as efforts to create a bridge are still complex and not clear at this time.

Deeply relevant to my autoethnographical study is Jung's work relating to a shadow self. Jung (2001) asserted that over the course of our individual development the repressed characteristics of ourselves coalesce to form the shadow, which he called the "dark" side of our being. Our lack of self-awareness allows us to see negativity in others rather than in ourselves.

Jung's work has informed me during my tenure as a therapist and a student. His work on the shadow self of each of us is another way of discussing our projections on others, our extreme judgments of others and, for this study in particular, how trauma occurs in a family through attempts to change a child's experience from what is to what the parent wants it to be. I will use the idea of the shadow peripherally as this study is focused on nonduality as a healing modality, and not Jung's work.

In this autoethnography, I explore my personal journey of healing my suffering, from the time stretching between the 1950s and '60s to today, with the intent and objective of offering practical ideas and curricula to mitigate early childhood trauma in present time. My journey recovering from postpartum psychosis highlights how nonduality transformed my suffering through continually resolving the trauma that led me to a mental hospital at age 26. This study also focuses on issues particular to women, such as post-partum depression and psychosis, and how that was misdiagnosed and treated as mental illness, rather than hormonal imbalance.

To this day, I do not know whether if I had been treated for post-partum depression rather than an adjustment disorder, an emotional response to a stressful event, my life would not have included as much suffering and also as much growth. It is a state of subjective distress and emotional disturbance, which arises during the course of adapting to stresses of significant life changes, stressful life events, serious physical illness, or possibility of serious illness (Patra & Sarkar (2013). O'Hara (2009) declared post-partum psychosis as a disabling disorder occurring once in every 1,000 deliveries, and one where intensive treatment, usually on an inpatient basis, occurs. Patra and Sarkar (2013) discussed adjustment disorder as a common diagnosis in psychiatric settings which carry a significant rate of morbidity. When I research the term *morbidity*, one of the symptoms is a preoccupation with death and other gloomy emotional responses to stressful situations. When I feel frightened at times, this symptom is perfectly descriptive of my experience.

Another issue illustrates how being a single mother during that time, with the politics and laws that dealt with child support and maintenance, made it almost impossible for me to pursue a career that brought my unique gifts and talents to the public arena. To this day, I find it extremely difficult to share the responsibilities of childrearing and the day-to-day household maintenance with my partner. I can see how my experiences of early childhood abuse by my father continues in my response to everyday circumstances with my sons and partners, other men in my life. At times, I want to gain their approval and find myself attempting to speak in a way that I know will please them. At other times, with no apparent reason, I am resentful and angry at them. When I inquire why I might feel that way, I recognize I am experiencing a trance state in which I

am not really in the present, and not seeing them as who they are. Rather, I am back with my father and I am behaving like my mother. I know this is a reaction, and I know at the moment I did not have the capacity to shift my behavior.

I believe that having a memory and an experience of the trauma-related events allow for a more resilient response to present day situations. In a personal communication from Ira Glovinsky (July 2019) he stated, “Here is where we have to be careful. You can get a resilient response from a resilient person, but a person who is not resilient will not respond the same way. Resilience is an important variable in recovery. There is still so much we don’t know. What some of the neuroscience research is showing is that each of us has a ceiling in areas of our functioning.” The definition of resilience is one’s ability to recover quickly from difficulties. I do not know what causes one person to be resilient in a particular situation and not another. I can say this is still being explored.

Brewin, Huntley, and Whalley (2012) suggested that a child’s awareness and memory of the events may be necessary for symptoms of trauma to be examined and alleviated. The question of the correlation between traumatic events in early childhood and resilience in adulthood has been investigated (Masten, Best, & Garmezy, 1990). Although I have suffered from anxiety much of my teen and adult life, I am an example of a resilient survivor of trauma. Many of these transformative shifts have occurred through the experience of nonduality.

My autoethnographic study seeks to discover how nonduality can provide a foundation for healing that other healing modalities will be able to work with. The ethical stance that I am taking is crucial for the autonomy and integrity of a researcher while dealing with a secularly cleansed model of healing.

Autoethnographers challenge canonical ways of doing research and representing themselves and others through a political and socially responsible, socially just, and socially conscious lens (Ellis, Adams, & Bochner, 2011). The self-reflection and exploration of my unadulterated truth has been a strenuous, emotional, and intellectual challenge each day. When I am able to both show and tell an event with cultural and social meaning that extends to others, I feel no resistance to this work; rather, it becomes a labor of love for want of a more academic turn of phrase.

Many of us have encountered early childhood trauma, from the benign to the severe. Some children can escape most of the negative long-term results of external trauma, from something as simple as a clock ticking all night long in an infant's bedroom (Blackstone, 2007, 2008a, 2008b). Others suffer greatly from any kind of continued negativity, whether parents or caregivers are conscious of the effect or not (Eliot, 2000).

Although we are each unique, we are also all human with the shared capability to experience human emotions (Gazzaniga, 2008, 2009 p.7). It is common knowledge that at times we wish to forget uncomfortable experiences. So, we often obfuscate them or create pleasant illusions to cover up the memory and pain. When our feelings and memories are diluted and dulled, the experiences do not disappear from our bodies (Van der Kolk, 2015).

As we know from our instantaneous, hair-trigger reaction to situations that seem innocuous, our reactions do not seem appropriate for what is happening in the moment. A serious disadvantage of not experiencing our feelings in difficult situations is that we often misinterpret what is occurring in the present based on what occurred in our past personal childhood conditioning (Orloff-Falk & Doubleday, 1991). In speaking with

Glovinsky (2018), he asserted that one's "internal working model" begins to get laid down very early in development, and then we develop cognitive biases that drive our thinking in the direction of these models (I. Glovinsky, personal communication, August 30, 2018).

Our internal working models become solidified with memories that seem to prove our points of view. Two siblings present at the same event have different interpretations of the event. Is either one inaccurate? These early childhood memories, whether repressed or malignantly experienced, continue to impact us throughout adolescence and adulthood. The quality of care during the child's first 2 years corresponds with the development of adolescent troubles and can become a "vicious cycle" (Van der Kolk, 2015, p.327). Van der Kolk (2015) also considered an informal discussion he had with Alan Sroufe, a researcher working with him, who believes that resilience in adulthood could be predicted by the love or lack of love that mothers or caretakers felt toward their children by age 2 (Van der Kolk, 2015, pp.160-161).

In my exploration of the literature, I discovered there are more anxiety-related disorders in girls compared to boys. It may be that girls have more difficulty regulating negative emotions, and this difficulty to regulate emotions is more predictive of anxiety in girls than boys (Bender, Harris, Reinholdt-Dunne, Esbjorn, & Pons, 2015). This has been true for me, personally, since I was a small child unable to have a voice when my parents were in conflict situations. They had conflicts with each other, with me, or with a server in a restaurant who did not trim the fat from my father's steak before it was cooked. In most cases the literature discussed marital violence between men and women,

not fathers and daughters. Yet, in my case, both my parents and my sister participated in the abuse toward me by blaming me for my father's behavior.

When daily emotional issues that are problematic to parents overflow into their interactions with their children and each other, the effect on the other members of the family are long term (Almeida, Wethington, & Chandler 1999; Almeida & McDonald, 2004). "Anxiety disorders are among the most common psychiatric disorders affecting children and adolescents" (Albano, Chorpita, & Barlow, 2003, p. 292) The experience of anxiety in childhood and adolescence can lead to a variety of negative outcomes including antisocial behavior, academic problems, and the development of other mental health problems (Ginsburg, La Greca, & Silverman, (1998). Ira Glovinsky in a personal communication (July 2019) stated, "Some disorders emerge in phases. We refer to this process as staging. Anxiety is often the first signal in the staging process. We experience anxiety and then symptoms, leading to other disturbances of emotions."

Theoretical Framework for the Study

. My autoethnographic study seeks to discover how nonduality can provide a foundation for healing that other healing modalities will be able to work with. The ethical stance that I am taking is crucial for the autonomy and integrity of a researcher in dealing with a secularly cleansed model of healing. Nonduality does not offer a religious or spiritual model for how to be as a therapist. I need to assure myself and those I work with that the nondual model of healing is not at odds with any spiritual or religious practice they are identified with. I must not in any way, while working with a client or organization, exaggerate my memories, accomplishments, failures, transformational processes or narcissistic tendencies.

Foster (2007) Parsons (2004) and Theriault (2005) declared the central tenet found in nondual realization is complete freedom from the loneliness of a separate self. When I first heard the idea that we were not a separate self, I felt totally scared and uncomfortable. I wanted my separate self. I wanted to be a special separate self. I didn't want to merge with anyone or anything even though my experience with LSD was a merging of myself and all that is. Quite a paradox. After study with nondual teachers I recognized I would not have to give up my personality or uniqueness and therefore was able to lean into the idea without experiencing the fear and anxiety that I would die if I weren't me. In a course in Systems Theory at Fielding, I learned that Wheatley (2002) had stated,

As leaders, we act on this truth when we're willing to notice how a decision might affect others, when we try and think systemically, when we're willing to look down the road and notice how, at this moment, we might be affecting future generations. (We Are Interconnected to all Life, para. 20)

Katz, (Ed.) (2007), a nondual practitioner, stated that deconstruction, when used in nonduality, means the undoing of the false self, which he calls the separate self. What does it mean not to have a separate self? What does it mean to be at one with the universe? Is it possible? Philosopher Tomas Metzinger (2004) said, "Nobody ever had or was a self" (p.362). I have not met anyone who is egoless or who does not live their everyday life in a way that demonstrates their conditioning. Fenner (2007) proclaimed our everyday world is dualistic, filled with either/or choices, not nondual, where there is a panoply of choices for each decision and they come without effort.

Many aspects of nonduality are paradoxical (Fenner, 2007). In non-conceptual

nonduality something can be true and not true at the same time, good and bad simultaneously. Our preferences for order, structures, concepts, and so forth, do not matter during the time we are resting in nondual awareness (p. 10). When we can see many alternatives, not just an either/or, our choices can be transformative. For the purposes of this study, the healing of early childhood trauma, I will be exploring the Buddhist Eastern non-conceptual mindfulness as defined by Peter Fenner and other expositors in this tradition.

Discussions of both Non-Conceptual Nonduality and Conceptual Mindfulness

Mindfulness is important in this discussion because, as Kabat-Zinn's You Tube, (2017) definition of mindfulness stated, it is "awareness that arises through paying attention on purpose, in the present moment, nonjudgmentally, in the service of self-understanding and wisdom." Many people think mindfulness is nonduality, and I believe it is important to make the distinctions clear. I believe the distinctions are crucial for this study. I have not experienced being nonjudgmental as an easy task as defined by Kabat-Zinn (2017). Yet, even with that caveat, Western conceptual mindfulness has taken hold in our mainstream culture by its practice in hospitals, retreat centers, universities, yoga classes, self-help books, and more. The empirical literature on mindfulness training can be useful in many treatment options (Baer, 2003). Although it is demonstrably beneficial in psychological interventions and medical conditions such as stress-induced pain, diabetes, and so forth, it differs from nonduality, the traditional Eastern Buddhist model, which is "non-conceptual" (Weick & Putnam, 2006). Its focus is on the present moment, and its results show it can ameliorate stress-induced symptoms (Baer, 2003). I have often questioned whether mindfulness is a precursor to nonduality. Weber, in his January 7,

2017, YouTube video, stated unequivocally that mindfulness is useful for “toning” down symptoms, yet nothing fundamentally changes. In order to create a fundamental change, individuals need to “let go” of who they think they are and be willing to wake up to who they really are. In nonduality that means our idea of who we are is an illusion and rather we are pure awareness that does not know ourselves to be that. He does not believe mindfulness is a precursor to nonduality. (For a detailed discussion on mindfulness and its uses and value in different stress-related diseases and symptoms, please see Baer, (2003).

I explore mindfulness in my review of literature in a limited manner to determine if it is, in fact, a precursor to nonduality. Opinions differ on this. I have concluded it can be under specific circumstances. However, whereas mindfulness has been shown to produce change in negative behaviors and stress produced, my focus remains on nonduality as the fundamental source of consciousness in order to affect a permanent transformation and dissolve childhood trauma. Gill, Waltz, Suhrbier, and Robert (2015) asserted the practitioner/client relationship shifted if the therapist meditated and lived a mindful life. One participant in their study, when discussing inherent goodness and non-judgment asserted,

This concept of essential goodness, or that someone’s essential nature is fundamentally whole and intact. ... The Buddha doesn’t speak about someone being “damaged,” and there’s a real emphasis on a nonjudgmental stance towards clients. And that resonates very strongly with the values that arise out of my mindfulness practice; which is everything is connected, and everyone is connected, and there is fundamentally no difference between you and me. (p.714)

Gill et al. (2015) initiated a discussion of non-duality and mindfulness and stated emphatically, “The practice of mindfulness itself is very much linked to directly abiding in non-dual, non-conceptual awareness. It is a helpful explanation of how difficult it is to speak about non-dual consciousness” (p.718). This article declared a direct connection between the therapist’s knowledge of nonduality and his or her ability to become a spiritual teacher. One therapist in the article stated that “In the therapy world we’re talking about structure and ego, and all of these things. In my spiritual practice we’re talking about losing the self and there is ‘not self’, and to know the self is to forget the self, so the emphasis is totally different in one way, yet there are some commonalities” (p.719). This illustrates a difficulty of knowing the differences between mindfulness and nonduality. Mindfulness and nonduality are two different modalities of support for healing, and generally speaking, comparing them takes away from both. Yet, for purposes of this study, I believe it is important to make a few distinctions between the work when the actual training is completed. I personally am uncomfortable with this perspective as I believe it is a disservice to both mindfulness and nonduality.

While nondual practices are centuries old, they have only recently become prevalent in Western medicine and psychology under the name of *mindfulness*. Zinn (1990) and Langer (2014) introduced mindfulness to the West. They adhered to the conceptual mindfulness framework. In the case of mindfulness practices, conceptual means noticing and naming the objects in the room, toward positioning the practitioner right here, right now (e.g., focusing on the present moment nonjudgmentally); Decharms, 1997; Vogus & Welburne, 2003; Leventhal & Rerup, 2006; Weick & Putnam, 2006; Wishall, C., (2004).

As long as there is recognition, there is thinking and conceptualizing. According to Davis (1999), “We cannot know without conceptualizing” (p. 50). Nonduality is a literal translation of the Sanskrit word *Advaita*, which means *not two*, and points to the essential oneness (wholeness, completeness, unity) of life, a wholeness which exists here and now, prior to any apparent separation (Foster, 2007). In a personal communication, Alan Davis (2018) stated the following:

It seems to me that this is only true if you *define* “knowing” as involving conceptualizing. I would argue that you can know pain, joy, love, or how to walk or ride a bike without conceptualizing. Pain, joy, and love are feelings. Feelings don’t have to be named or categorized in order to be experienced. Knowledge of how to walk or ride a bike exists unconsciously in the body without interpretation or conceptualization. I’m not arguing with the distinction between conceptual and non-conceptual. I’m arguing against the word *knowing* as key to the difference.”

Difference Between Conceptual and Non-Conceptual Buddhist Mindfulness

How do we really know the difference between conceptual and non-conceptual Buddhist mindfulness? And, how do we describe an experience that defies definition? These questions are crucial for this research project. Pure experience without naming or conceptualizing is a descriptive used for the experience of nondual awareness by many practitioners and clients (Fenner, 2015). What is that exactly? Nonduality is not just the absence of thoughts and feelings and concepts in themselves; rather, it is a state of being that cannot be described in words. This is a paradox and a contradiction, because in order to write about something we use words, pictures, videos, and other methods of communication. And so, as I write about this experience that defies definitions, I will

provide a short example of how to speak from the nondual according to Peter Fenner (2015). His preface of this short communication is to make the point that instead of speaking from our conditioned mind, which will want to make a point and discuss concepts, we speak to what is going on as we listen to the other person, rather than making a point. Our speaking is continually adjusted as we listen to the other person, focusing on the here and now.

Student: How do you talk about nothing?

Peter: Well, we can begin by talking about something that appears to be something, something that seems to have some characteristics.

Student: Like unconditioned awareness.

Peter: Perhaps. What is unconditioned awareness?

Student: Well you've said it is a state of being completely at home with ourselves, not rejecting anything.

Peter: Yes, I think at times I have. But that sounds like a conditioned state to me. We're at home with ourselves in contrast to being somewhere else. And, we're accepting rather than rejecting. It doesn't sound unconditioned.

Student: What doesn't sound unconditioned?

Peter: What you're talking about.

Student: What am I talking about?

Peter: Your understanding of unconditional awareness.

Student: Yes. Well, I'm not sure what it is.

Peter: What is?

Student: Unconditioned awareness. Are we still talking about that?

Peter: I'm not sure what you are talking about. Do you know?

Student: I don't know. Do you know? Are we talking about anything?

Peter: I don't know. Do you know?

Student: I don't know.

The nondual state does not exclude anything, including our relative state of everyday life. While resting in the nondual state, we notice the objects around us, as we are also in the relative state as well. Once something is described, it is conceptual. Yet, it is obviously clear, Fenner (2007) has said, and as I stated above, duality is our everyday way of living. We compare, we judge, we name, we interpret. Through my practice of nonduality all these years with myself and my clients, I quickly recognize that there is no problem with duality. Rather, my life is less chaotic and conflicted when I am able to abide in nonduality for short moment after short moment.

At times, when I am working with clients or just accessing this nondual state, my inner experience is one of "not knowing." I am comfortable just letting the experience unfold as it naturally does. The client lets me know where to go next. I am listening with a clear mind and can take awakened action when appropriate. When the mind is clear, we can begin to ameliorate the anxiety of negative childhood trauma. I have found that occurring when I am not experiencing emotions or regretting the past or worrying about the future.

The Cambridge Dictionary (2018) defined *conceptual* as "based on ideas and principles." The differences between conceptual and non-conceptual mindfulness or nonduality, as formulated by Fenner (2007), are represented in Figure 1. The top half of the figure represents what Fenner called nondual awareness, which is another name for

non-conceptual Buddhist mindfulness. Contents of awareness such as feelings, thoughts, and sensations are not in the top half of the diagram. There are concepts in the top half, such as “Witness Consciousness.” Yet, to the nonduality practitioner or student, these concepts, when experienced are devoid of the very concept they represent, whereas the bottom half of the figure represents conditioned awareness, which includes feelings, sensations, and thoughts.

Fenner (2007) asserted that all feelings, thoughts, and sensations are conditioned through our early years and, therefore, open to interpretation. Nondual awareness is not open to interpretation, as there are no thoughts, feelings, or sensations (Fenner, 2007, 2015). In this context, “conditioning” is what we were taught from when we were in the womb and heard our mother and others speak and reacted to her experiences, to when we realized we had real choice, which is when we began to recognize who we really are. In a way, it is similar to McLeod, S. (2015) discussing Skinner-operant conditioning as early as 1938.

. Although there is not a bell that rings that brings us to a “Manchurian Candidate” behavior, it is a quickening that causes us to react in the moment to incidents that really have nothing to do with what originally came about (Orloff-Falk, Doubleday, 1991).

Figure 1 is a handout given to Fenner’s students, of which I was one for many years. If examined, the top half, which Fenner calls nondual awareness, is devoid of the objects of awareness, which are thoughts, feelings, and sensations. It is from that nameless space that I believe healing from trauma and transformation is possible.

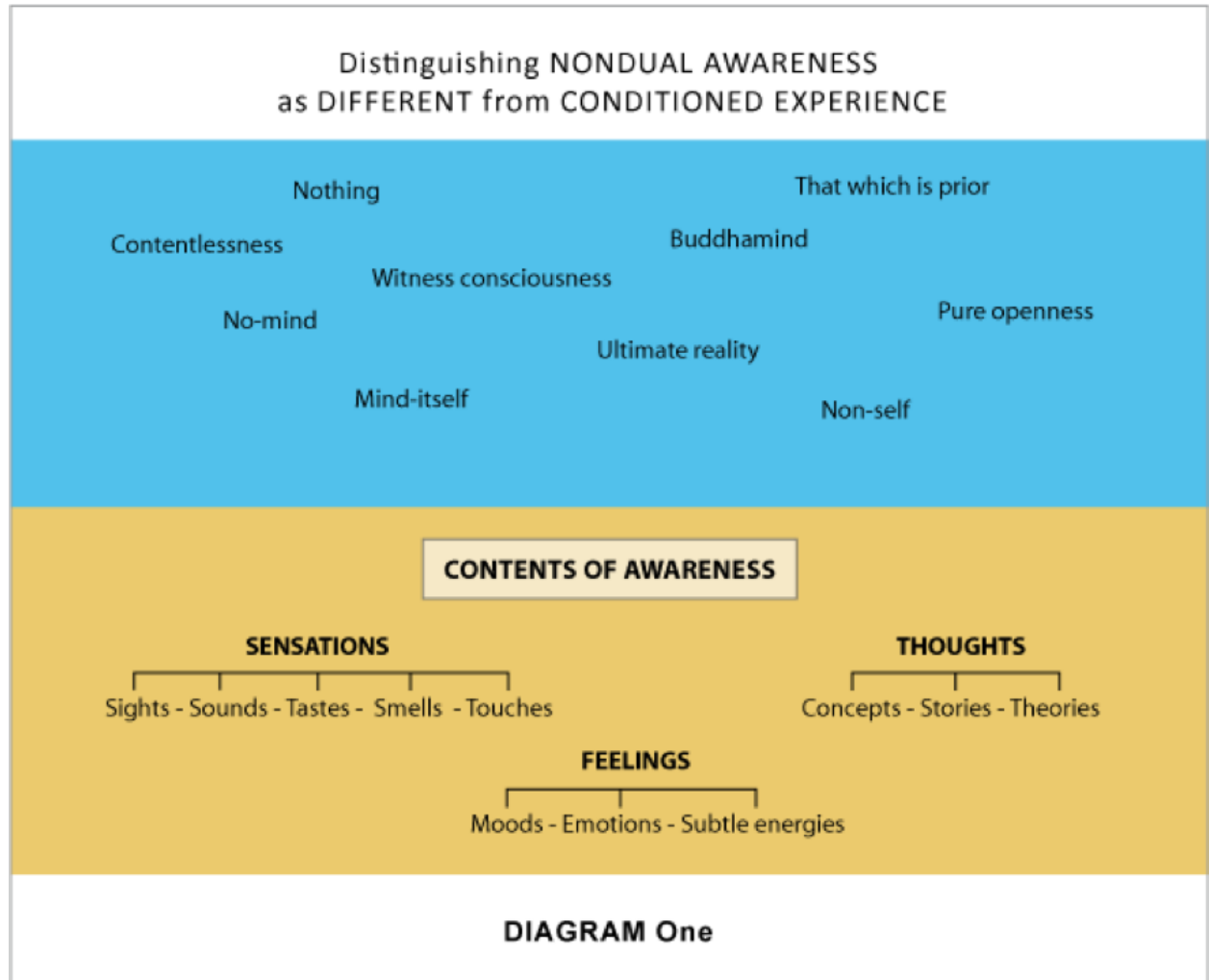


Figure 1. Distinguishing nondual awareness from conditioned experience.

Non-conceptual Buddhism can be best described as “mindfulness-as-process” (Weick & Putman, 2006, p.286). I call what I am writing about *nonduality-in-action*. I call it *nonduality-in-action* because without the action of noticing there is no moving toward unconditioned awareness in an effortless manner. This method of healing does not ask the participant to attempt to do anything, including paying attention or giving up a

judgment. Rather, in my experience of 12 years of practice, one sits with whatever is arising in the mind at any moment and does nothing.

Doing nothing, for many people, including me, can be difficult and disconcerting. In this process of “doing nothing,” we experience whatever is occurring in the moment, whether it appears as worry about a child, relationship chagrins, fear of not having enough money for the mortgage payment, until this concern diminishes in intensity and disappears, albeit temporarily at first. If there is a judgment, the nondual support is to not judge the judgment. If there are tears, nondual support is to just let there be tears. What is most exciting to me and for this study is that Weick and Putman (2006) asserted this form of healing could potentially improve mental skills that are generalizable across different domains, including emotional, physical, and intellectual. In my experience of raising my own children, once they achieved mastery in a skill, such as soccer, it was during that time they were open to achieving mastery in emotional and academic skills. That certainly happened for me. In this dissertation, I examine how nonduality could diminish negative childhood messaging and trauma through generalizing changes in attitudes and abilities of an individual.

Definition of Nonduality

Nonduality is that state of being described as the unconditioned dimension of a person’s existence and the deepening and stabilizing of that experience (Fenner, 2007). Unconditioned awareness is described by some as “content less” awareness and is one where nothing needs to be done and nothing needs to be thought about and understood (Fenner, 2007). How can awareness be contentless? Fenner (2011) explained contentless awareness as follows:

Our conditioned experience unfolds in time—it is always changing. We can touch, feel, sense, and think about it. Nondual awareness, on the other hand, doesn't have any of these characteristics. It's not a "thing". We can't see it; we can't even think about it because there is nothing to think about. Nondual awareness is completely unrelated to you and me as different embodied minds. It's unrelated to the circumstances of our lives or the condition of our bodies and minds. We are born and we will die. We have gender, age, race, etc.; nondual awareness has none of these. It is ahistorical, transpersonal and transcultural. (p.12)

Unconditioned awareness is paradoxical, as it contains and yet goes beyond all forms or structures of experience (Fenner, 2007). It has been called source consciousness, as it is believed it is that which everything that is the content of our minds appears to arise from and return to (Spiro, 2017a). Unconditioned awareness could be likened to a movie screen, in that nothing that is projected on the screen disturbs the screen from its original shape and color; yet, when a film is projected on the screen it seems as if that which is projected actually saturates the screen. When the film is over, the screen is just as it was before the film was shown (p.17).

Blackstone (2008b) declared unconditioned awareness our fundamental consciousness. She discussed that at times there is a peak experience which can seem instantaneous, in which a person seems dazzled by his or her shift in consciousness. Spiro (2017a), Fenner (2007), and Blackstone (2008a) all concur that these short moments of unconditioned awareness usually come and go until they become stabilized. All of these definitions of unconditioned awareness concur with my work in nonduality.

As I clarified previously, unconditioned awareness is void of content, so putting words to this wordless experience feels and is contradictory.

A personal experience of nonduality occurs for me as an internal vision. *I see myself walking in a forest. As I create a path, I begin to eliminate the debris on the path. All the debris represents my regrets of the past and fears of the future. Eventually, the debris on the ground is gone. Then thoughts and feelings are like birds flying through the air. And they don't stick to me or leave a trace as they fly through the air. Shortly thereafter, there are the multivariate sounds of silence. This eventually subsides and there is what I call no-thing. All is well. The thoughts and feelings have disappeared, and the illusion that I needed to do something to change what was occurring also was gone. What was left was this ineffable experience that all is okay.*

Lindahl (2015) asserted that Eastern nonduality which he called mindfulness contains within its experience a concept known as “right mindedness,” which provides an ethical-based view of being present, whereas Western mindfulness has been secularized purposefully (Germer, 2013; McMahan, 2008; Miller, 2014). Wilson (2014) argued that an important appeal of mindfulness is a deliberate process of removing and obfuscating its connections to Buddhism, religion, and ethics. It is my personal observation that this ethical, spiritual practice of right mindedness is missing from most non-religious healing practices.

One of my reasons for choosing the framework of nonduality is that it shares the ultimate goal of Buddhism. Although nonduality is only one aspect of Buddhist practice, its ultimate goal is the same as the ultimate goal of Buddhism, which is the alleviation of suffering. This goal occurs through sustainable changes in a person's cognitive and

emotional capabilities, which leads to irreversible changes in behavior (Slagter, Davidson, & Lutz, 2011). I personally have experienced these irreversible changes. For example, I am aware when I am confronted with deeply uncomfortable situations and my inner experience supports me to go ahead with my decisions while experiencing the discomfort. I rarely panic when I make a mistake--I said rarely! Rather, I am able to watch my thoughts and feelings about the mistake and through the watching, the paying of uninterrupted attention, my negative thoughts and feelings tend to disappear.

I am still deeply attached to having my committee really like my work. I use references during narratives. When I thought about this, I realized I wanted to be sure I was believed. This need has not dissolved. When this need arises, I stop my work and remember what it was like as a child. When the sadness leaves, I move on. Continued short moments of a nondual experience are a simple way to sustain the experience of “all is well” at the moment. The relationship between the practice of nonduality and diminishing suffering is not a mystery. When our assumptions about another’s motivations and judgments begin to lessen in intensity, we can see the past through a new lens. For me, rather than view the behavior of abuse that was levied on me as a child as “my fault,” through the practice of nonduality I was able to see my parents’ behavior from a different view. That view no longer blames my parents for their behavior. One of the beliefs in most therapeutic interventions is that our parents did the best they could with what they had available to them at the time. I believe that is true: I can see it is true for me as a parent as well.

Use of Nonduality as a Therapeutic Modality

Kiloby's (2014) main premise is that nonduality does not promise a better future, but is an invitation to look deeply into our present experience and realize the healing we are so desperately seeking is already here. When one accesses nonduality, it is impossible to create problems. However, as a rule we tend to identify with highly conditioned states of consciousness and want our "now" to be different from what our now really is. This is a really important point, and people who have a weak capacity to mentalize make their "now" a reality when their "now" is really their feelings going on inside them (I. Glovinsky, personal communication, August 30, 2018). Our attachment or aversion to the circumstances of the present can make our life filled with suffering in the form of frustrations, disillusionment, pain, and anger. Kiloby (2014), agreeing with other nonduality teachers, declared nonduality as the profound medicine of healing that makes a difference in everyday life.

Through my review of the literature, I rediscovered that nonduality is an experience that two people could share without their personal thoughts or history interfering, and in that space profound healing occurs (Blackstone, 2008; Fenner, 2007). Fenner (2007) declared that the nondual experience is the only experience that two people can have that is exactly the same. That does not mean that an illness like cancer will disappear or that a broken bone will miraculously repair itself. Personally, I am able to experience a deep sense of contentment even though my body may be in pain or my personal life is conflictual at that time. In a personal communication, Alan Davis (August 2018) asked me the following:

Is the observer/therapist surrendering or entering into the client's account of the experience without allowing her own interpretation or evaluation to intervene, thereby "sharing" the experience akin to having the experience herself? This would make sense to me. During a session, what happens after the experience has been shared. What does the therapist say or do?

This is an important question that cannot be answered with a singular response. In my work with clients, once they share their direct experience, there is nothing to say most of the time. I will describe some nondual therapy sessions and their results in Chapter 4.

When daily emotional issues that are problematic to parents overflow into their interactions with their children and each other, the effect on the other members of the family are long term (Almeida, Chandler & Wethington, 1999; Repetti & Wood, 1997; Almeida & McDonald, 2004). "Anxiety disorders are among the most common psychiatric disorders affecting children and adolescents" (Albano, Chorpita, & Barlow, 2003, p. 292). The experience of anxiety in childhood and adolescence can lead to a variety of negative outcomes including antisocial behavior, academic problems, and the development of other mental health problems (Ginsburg, La Greca, & Silverman, (1998). Ira Glovinsky, in a personal communication (July 2019) stated, "Some disorders emerge in phases. We refer to this process as staging. Anxiety is often the first signal in the staging process. We experience anxiety and then symptoms, leading to other disturbances of emotions."

Definition of Duality

Before further exploration of nonduality I will provide a definition of duality. Fenner (2007) asserted that duality appears through fixations which he calls the source of suffering (p.88). The suffering comes from fixed opinions or beliefs. This can lead to a “rigid and inflexible” interpretation of our experience. When that occurs, there is usually an either/or way of looking at the world, comparison of good vs. evil, right vs. wrong. The aspect of dualism that seems most salient here is mind-body dualism. This relates to a separation between mind and matter, beginning with the thesis that mental phenomena are not physical. So, suffering is the unwillingness of our finite selves and the human condition to accept what is. I believe that this is the human condition. According to nonduality we are both finite and infinite. When faced with the question of “What is the purpose of suffering?” Spiro (2017b) tells the questioner in his venue that the real purpose of suffering is to motivate inquiry as to what will bring happiness from inside. Spiro (2017b) believed that suffering and happiness go hand in hand and living a life based on nonduality can provide the alleviation of suffering.

Origin of Nonduality

There are many different names and sources for nonduality. Although many individual practitioners may not agree, it is a critical component of many religions, including Judaism and Christianity (Prendergast, Fenner & Krystal, 2003). The roots of nondual awareness as experienced on the journey to awakening are connected at the core of ideologies throughout our human history. These include Hindu Vedanta, and most religious schools Fenner & Krystal, 2003). In essence, nondual realization is a process of

mature consciousness (O’Keeffe, 2014). This maturity allows for a deep and broad-minded acceptance of self and others just as they are.

Nonduality is seen as early as 800 BCE through Advaita Vedanta, a school of Hindu philosophy and religious practice, one of the classic Indian paths to spiritual realization. Advaita, a Sanskrit word, means nonduality, and is found in verse 4.3.32 of the Brihadaranyaka Upanishad (~800 BCE), and in verses 7 and 12 of the Mandukya Upanishad composed between 500 BCE to 200 CE. It is actively being practiced today in many spiritual programs; it has many layers. I experience it as the silent essence of no mind. Some names for nonduality are Awareness, Consciousness, Nirvana, Brahman, oneness, even God (Sugunasiri, 1995). The nondual is constant, ever present, and the essence of existence; not just for humans, but for all sentient beings, those capable of feeling (Sugunasiri, 1995). While I will pursue a few philosophical precursors of nonduality in the review of the literature, this study focuses on nonduality as taught by the spiritual teachers and psychotherapists who are scholar-practitioners in the present and recent past. I do not research the origins or uses of nonduality from its roots, except in a cursory fashion.

There is another model of the unconscious that has roots in the cultural experiences of the last hundred years of one’s individual family’s geographical location. Hopper and Weinberg (2011) called this the *social unconscious*. Although I will not be exploring the social unconscious, I do wish to make a point here before moving on. Foulkes (1983) adopted Erik Erikson’s suggestion that an individual’s mastery over his neurosis begins when he is in a position to accept the historical reality and necessity that makes him who he is today. Although the family certainly takes center stage when it

comes to how children are taught their beliefs, values, and influencing ways they share their feelings, it is impossible to leave out the contextual issue of the history of their ancestral consciousness over 100 years (Foulkes, 1983) or the cultural learning that occurs in the 9 months in the womb.

Orloff and Doubleday (1991) asserted that when one is in the experience of nonduality there is clarity to choose without being bound by early childhood trauma. The either/or dichotomy disappears and there are many possibilities to make new choices that can redirect our life. Rediscovering that was deeply rewarding and reminded me of one important reason why I returned to Fielding Graduate University to earn my doctorate. Through my immersion in studying and practicing nonduality, I understood that without this scholarly research, deep study, and collaboration with fellow colleagues and faculty, I would not be able to experience the confidence to share this wellspring of wellbeing in the world.

I am concerned that traditional therapy and alternative healing techniques cannot access the connection needed for core healing to occur, such as my continuously occurring PTSD brought about by my childhood and then again by postpartum psychosis. This is because of the relationship of the therapist to the client, the person behind the technique, and also because of the singularity of the methodology of the model used.

Prochaska and DiClemente (1982) created the stages of change transtheoretical model showing how there are five areas they found were most common in the therapies they studied. These areas can be compared positively to my Mindfulness in Action patent. The transtheoretical model posits that healthy behavior change involves progress through six stages of change: precontemplation, contemplation, preparation, action,

maintenance, and termination (Prochaska & DiClemente, 1982). This information was garnered by the authors studying many different therapy models and taking the best and most common from the models. When speaking with Ira Glovinsky, he stated that it is usually impossible for psychologists to avoid all contact with clients outside their offices and that relationships that do not hinder psychologists' performance or harm their patients are ethically acceptable (I. Glovinsky, personal communication, September 2018). I contend that much of a therapist's training is designed to create some distance and objectivity in their work with his or her clients. The nondual model of psychotherapy shifts that to where the therapist and the client are one (Blackstone, 2007, 2008, 2009; Fenner, 2015).

Significance of this Study

The life-altering positive impact of nonduality practice is supported by much anecdotal evidence narrated by scholar-practitioners (Blackstone, 2006; Fenner, 2015; Hanh, 2001; Nixon, 2013). Eisner (1988) asserted that just being able to know that you are not your thoughts can make a big difference in everyday life. My nonduality process and practice are shared in my autoethnography so that there will be an explicit explanation and description of how that works.

Bojuwoye (2013), Edwards (2013) asserted that generational and ancestral consciousness permeates our DNA and our experiences. They declared that addressing these beliefs in traditional cultures makes a difference in working with clients regarding attachment issues, particularly when working with African Americans. I have had experiences with clients in my Choices, Radical Love, and Radical Choices seminars where the participants claim they are being nurtured and directed by "spirits" whom they

consider their ancestors. Is it really possible to connect with ancestors? Is it possible to retrieve past life experiences? In my work over the past years, there have been many incidents that would lead me to the conclusion it is possible and, in fact, is happening without our awareness. That connecting with the wisdom of the past is not imagination; rather, it is a natural process that we all can access. I dive into these experiences in my autoethnography.

A search of scholarly databases revealed research, poetry, novels, and film express a deep, abiding curiosity toward early childhood conditioning. Scholars from diverse disciplines such as history and archeology, medicine, language and literature, education, social services, and public policy are exploring early childhood conditioning. What I am referring to with the word *conditioning* corresponds to how the word is generally used. It refers to what we were taught to believe, behave, think, and feel based on how our parents believed, behaved, thought, and felt (Fenner, p.160). In the Fielding library there are 15,831 books, 15,662 journal articles, 12,387 dissertations/theses, 1,557 chapters in books, and 6,311 newspaper articles found under the heading of “early childhood conditioning.” The primary interest in this developmental literature has focused on how early childhood conditioning and the cultural and social milieu sets the stage for an individual’s future (Marusak, Martin, Etkin, & Thomason, 2015; Wilborn 2015; De Young, Kenardy, & Cobham, 2011).

Childhood abuse and neglect “increased the likelihood of an arrest as a juvenile by 59 percent, as an adult by 28 percent, and for a violent crime by 30 percent” (Widom & Maxfield, 2001, p.1). Repetti, Wang, and Saxbe (2009) also asserted there is evidence that family systems are coregulatory in such a way that the behaviors and illnesses of one

or more family members affect the well-being of other members of the family. Due to the frequency and depth of the discussions of how any one family member can affect the lives of the entire family, my family is just one example of this morass. My work in psychotherapy and programs across many demographics, from Caterpillar to Bank of America and Fleet Bank, Coors to small groups taught in private homes and small venues had convinced me that I wanted to create a model for healing that could reach many different cultures.

Why the need for a universal principle for healing? I have been wanting to connect at the core level of everyone for the benefit of all. I believe the practical application of nonduality in action is a healing medicine that can allow for that universal connection. There is a myriad of competent models of therapy and healing, both traditional and alternative. Because of my belief in the possibility of discovering a healing modality that can reverse this childhood cycle of violence and pain across cultures, I have been studying Buddhist-based nonduality and its effect on the inner life of individuals. The resultant behaviors of individuals living a nondual lifestyle may be able to co-regulate the family unit to which they are a part resulting in healthy supportive reconditioning. By participating in nondual living, the co-regulation in the family system is fostered in a positive, healthy manner. I also can see the ripples of this model of living moving into the organizations people populate and set a new tone in behaviors and connection. I know there are individual differences in all of us, yet the experience of unconditioned awareness, nonduality, may be able to work across cultures, races, and ethnicities as the foundation for other therapeutic models to have a strong healing effect.

My autoethnographic dissertation is emerging at a perfect time in our collective history. It encompasses the last 65 years of my social, psychological, physical, and emotional history, and how my personal paradigm of transformation coincided with paradigm shifts that our country was experiencing. Of course, these changes were not just occurring for me and my close associates. I believe these possibilities were universal. One way to describe one of these choices is drinking alcohol as opposed to smoking marijuana; listening to Neil Diamond as opposed to the Rolling Stones; going to art theaters or Elvis Presley movies. We each followed our choices with simple steps, on at a time, until we were met with bifurcations and more major choice points. It is my supposition that the state of our country and world is a result of the infinite opportunities and choices we have collectively made (Wilber, 2017). I wish to make my contribution to the work of healing that so many have been undertaking at this time.

Limitations of this Study

The very subject matter of this dissertation indicates the limitations inherent in any study of nonduality. The nature of a nondual awareness experience by definition indicates it is not an easily explainable concept. It is non-conceptual. Also, nonduality as a psychological intervention is not a therapeutic modality offered in universities as a healing therapy. So, peer-reviewed studies showing the validity of nonduality as a healing modality in early childhood trauma are in their infancy. In 2011 the Nonduality Institute was founded by Zoran Josipovic, PhD. He is the principal investigator and the husband of Judith Blackstone, PhD, a well-known nonduality practitioner whom I discuss in Chapter 2. So far, the research has been confined to states of consciousness.

I, personally, am also a limitation of this study. An expression of that is my haphazard recordkeeping of the thousands of hours of work and training in which I have taught, facilitated, and been a participant. I have used many journals, letters, and class notes. Yet, many of these are not dated. And, I have recognized during the last years working to achieve my doctorate, how unwilling I have been to be public with my work on nonduality. I have attempted to discuss and present my work in a way as to be seen as mainstream. I strive to be as honest and truthful as possible. I recognized the requirement that practitioners do their own work which includes looking at and doing their best to heal their past. This is a big ask, and I am aware not everyone will want to or be able to accomplish this. This work demands authenticity from the therapist. This might be difficult for many practitioners as our training as psychotherapists asks us for objectivity.

CHAPTER TWO: REVIEW OF LITERATURE

Introduction

This review of literature draws upon the salient work of those scholar practitioners who provide the conceptual foundation of this autoethnography. This study's overarching research question is: How does the use of nonduality assist in the amelioration of early childhood trauma? Such an application could make a unique and significant contribution to the therapeutic methods intended to alleviate the diminution of early childhood trauma. I am not suggesting that nondual therapy can heal childhood trauma by itself. I am suggesting that the experience of nonduality is a core, universal aspect of all healing that can be used with other therapist training that goes beyond theoretical modeling (Haley, 1963; Glovinsky, personal communication, 2018).

I have divided this review of the literature into four sections:

- In the first section, I briefly describe what is presented by the scholars of nonduality and then explain why this research has the potential of filling in gaps on behalf of bringing forth nonduality practices into healing early childhood trauma. The authors I have chosen are scholar-practitioners in the field of nonduality. Their work is seminal. This area of research on the efficacy of nonduality as a treatment for trauma is new and needs more

investigation. There is not much peer-reviewed literature on this subject; therefore, I have analyzed the work of nondual practitioners and discuss how I might approach the situation both similarly and/or differently.

- In Section 2, I focus on early childhood conditioning. I briefly describe appropriate studies relating to early childhood trauma, the results of the studies, and my observations on the efficacy of the work as it relates to my concerns.
- In Section 3, I will describe my personal vantage point and consider my research in conjunction with the perspectives set forth by a variety of scholars. Some of the studies I have chosen will refer to mindfulness. Although this study is not about mindfulness, I will look at them to determine if they are a precursor to nonduality and healing early childhood trauma.
- In Section 4, I will examine some of the work of my mentors from the 1960s and how their body of work still has sway and influence on my nonduality-in-action programs.

Nondual Practitioner Scholars

I have chosen some well-known, respected scholar practitioners and a few spiritual teachers. The scholar practitioners work with individual clients or families, while the spiritual teachers usually work with groups, sit in the front of the room, present a short lecture, and then take questions from participants. This is called *Satsang*, which means a spiritual discourse given in a sacred gathering. They rarely work with

individuals and do not see themselves as therapists although their words can be taken as advice as from a therapist.

Blackstone (2007, 2008, 2009) affirmed how we are shaped in childhood by our responses to “small, ordinary” traumas as well as other more extreme, intolerable situations or intense physical punishment that freeze our experience and disrupt our ability to connect inwardly. This inability to connect with ourselves creates a barrier between ourselves and others, and so begins the illusion of the duality of the self and others (p.76). Another way of saying this is that when we lose our connection to our feelings we begin to see ourselves as a separate self. Therefore, suffering begins! This perspective is shared by Fenner (2007) in that he discusses our suffering as not being willing to accept what is and rather attempt to deny or make promises to ourselves or God, to relieve the pain and suffering we experience. If we can feel what is really happening, whether it be pain or joy, we learn our feelings are like the wind. They are temporary and blow from many different directions and from many different circumstances. I believe it is only when we refuse to experience that which is, we embody the feelings we do not want to acknowledge, and then believe that our feelings are permanent and define who we are. Our suffering can abate when we are able to see what is really true and not identify with the feelings we are experiencing at the moment.

Blackstone’s (2006) method of healing trauma is for the therapist to support individuals to experience their feelings, senses, their body, to become embodied. Her perspective of healing through nonduality occurs when the therapist and the client have realized nondual consciousness together. This occurs when both the therapist and the client have let go of the concepts and problems that brought the client to the therapist

originally. In this moment, there is nothing to say and nothing to do. It is then they can discern and release the holding patterns they have accumulated since birth (Blackstone, 2006). Although Fenner (2007) did not discuss early childhood conditioning or trauma directly, he is essentially saying what Blackstone is saying. This realization produces a lasting effect, and the therapist and client are able to observe each other as both separate and connected, without the present experience contaminated by past trauma. This is usually a short moment followed by another short moment, and then another. These short moments can evolve into minutes, hours, and days.

I believe if we continue this practice without explicit goals to end suffering, and just continue noticing, noticing, and more noticing, we can live our everyday life with a new perspective of seeing and living what is really happening and ameliorate our addiction to attempting to control our external environment. Of course, with the explicit goal to end suffering, I cannot predict what will occur. I wish to reiterate how impossible it is to change what is.

Blackstone (2007, 2008, 2009) came to this realization through the relationship between the therapist and the client using somatic/energetic holding pattern release exercises and deep sharing and connection. She called that the Realization Process (p.14). This release occurs when an individual can remain in the nonduality state and still relate to another simultaneously (p.14). Blackstone's Realization Process is a series of nondual meditations and movement exercises designed to bring clients into their body to directly realize the nondual dimension of consciousness (p.79). Blackstone (2007, 2008, 2009) emphasized that nondual consciousness can and does arise spontaneously as we become more open to experiencing life as it is (p.123). As this occurs, memories also

appear and when experienced fully, tend to disappear for a period of time. I believe the cellular structure that holds the trauma begins to thin out.

After reading her book, I would like to meet Judith Blackstone. I am in alignment with her work and have had the experience with clients that once a joint opening of nondual consciousness arose with both of us, the next time we saw each other we commenced where we left off.

Bodian (2014) asserted that an expressed separation between the self and others is a painful and “seductive illusion”. He used the nondual no-separation direct approach for his own spiritual awakening. The direct approach is described by Fenner (2006) as working at the result level. Bodian (2014) claimed we live our lives through the lens of past traumas and hurts and rarely live in the present (p.28). Bodian’s approach went beyond what started as mindfulness to a direct awakening (e.g., nonduality) to what he called an “abiding ground” of awareness. He offered many meditations throughout his book designed to open fully to this self-awareness. A meditation example is “being aware of awareness,” or “find a quiet, comfortable place to sit for ten minutes,” or “take a few deep breaths and allow your thoughts and feelings to shift to bodily sensations” (p.9).

Bodian (2014) recognized the limitations of Western mindfulness, in that he thought mindfulness is only a doorway into nondual awareness where profound healing can occur. Bodian is not talking about curing illnesses. Instead, he is talking about the ultimate healing of the person. He calls this healing our true nature (p.6). I believe our true nature continues to exist inside the burden of early childhood trauma. After each guided meditation, there is a discussion. Much of the language in his meditations, such as the use of the word *allow*, are difficult for participants to accomplish in a meditation.

These meditations then become mind-based and rarely reach the body where, in my view and the views of others, such as Levine (1997) and Glover, early childhood trauma is stored. There has been the mind-body dilemma where the mind is seen as separate from the body rather than looking at the mind as embodied and the person as whole (I. Glover, personal communication, September 1, 2018).

The concept of “to allow”, is an action that, in my experience, needs a physical movement whether internally or externally. I think about the idea of allowing something to occur. In order to allow, one must give permission and time for someone to accomplish whatever is being asked of that person. I have used guided meditations in my work in the 80’s before I realized I was creating a dilemma for the client. Asking a client to do something such as to allow, first needed the concept to be understood and accepted. Once it was understood and accepted, it was possible to move through the protection of a trauma in the body and allow it to thin out and potentially dissolve. Our trauma can be precious to us as a known survival strategy and, I believe, the nondual therapist needs to have a deep understanding of how and where a trauma resides in our cells (Valent, 2007). In a nondual therapy session, the client leads the way and opens the possibility for allowing a penetration through the years of layered protection (Van Der Kolk, 2015).

Peter Fenner has been my main nonduality teacher since 2006. His teachings and his writings have been my bible for nonduality. Fenner’s work offered perceptions in the pure form of nonduality. For example, “the experience of unconditioned awareness is called the ultimate medicine” (p. 17). Fenner (2007) proclaimed that other types of healing have limitations, in that they work for some people and not others (p.134), whereas nonduality can work for any demographic. This understanding has provided me

with the willingness to consider nondual therapy as a universal healing possibility.

According to Fenner (2007), when we are experiencing unconditioned awareness, we are already healed. This space of unconditioned awareness comes and goes with daily practice.

Fenner was not suggesting that all our illnesses and worries are permanently gone; he asserts that when in that state of consciousness, we are no longer in conflict with what is occurring in the present (p.17). Our anxiety about what might happen, our regret about what has happened, and our fear of death are just some examples of conflicts that temporarily disappear when in the nondual state. I personally have experienced extreme fear of death from minor infections to serious illnesses. Although this preternatural fear has been present since the trauma I experienced as a young teen, I am now able to make distinctions between reality and my terror most of the time.

As Fenner's student, I learned to access that state of unconditioned awareness for short moments over a period of 10 years. In those moments, I understood there were actions I can take such as ownership of my life, and unlimited options for the future, rather than be constrained by either/or thinking. It seemed logical to me that my issues in relationships with men were initiated by my experience with my father. In this autoethnography, I share how these insights personally occurred and also how to share this wisdom in a simple format for others to consider using. I also realized that the behaviors and ways of being that occurred during those short moments of unconditioned awareness were transferable to my personal and professional life.

Johnson (2017) shared my perspective about how duality takes a prominent role in trauma-based conditions. For example, those persons who have had an early

childhood trauma such as dealing with continued humiliation present themselves to the world as arrogant or proud (p.25). Johnson (2017) named trauma-based behavior as energetic, whereas my program, Infinite Choices, calls it reactive. Johnson's points of view about traumatic ancestral lineage, personality, and ego are familiar to me in my experience of nondual therapy. Her method of nondual therapy included working in family systems and calling in past lives, including ancestral narratives (p.36). Johnson (2017) supported going directly to the source, the deepest aspect of the self, and used the metaphor of walking directly into a forest and cutting the bramble along the way (p.48). This coincided with Fenner as he begins his work at the "result" level (Fenner, 2007). The result level Fenner (2007) refers to is there is nothing to be enhanced. We are already residing in and are pure awareness.

Johnson's work with clients appears to be directed by her profound knowledge of nondual nature, and her spiritual work is deep and resonates with ancestral and generational influences. In her "Spiritual Media" blog, Johnson (2017) asserted the following:

When a sense of self opens beyond personal identity or psychological patterns, there is always a healing opportunity. There is a kind of psychic reset, in which first we realize our existence beyond all forms, and secondly we gain insight into the range of possibilities and attitudes we can have in our daily lives. Between unbounded consciousness and the relaxation of being somebody with the challenge of human responsibility are contractions of suffering. It is here that nondual therapy seeks to bring relief.

Her sense of a spiritual awakening into nonduality is that it is a continuous movement; it does not only occur once and then there is freedom (Johnson, 2017). I used to believe that once I had an experience of oneness, I would be healed from my early childhood trauma. After 12 years of study and practice in nonduality, I am still surprised every time I have a reaction to something that seems so trivial. Then I experience an “aha,” realizing I am still thinning the invisible line between my illusionary and natural self.

Jordan (2008) examined how cultural and personal denial of fear and vulnerability contributed to a sense of isolation. This dynamic is particularly present in hierarchical settings in order to preserve existing power arrangements. Jordan likened this denial of fear and vulnerability to the paradigm of slavery, referring to slave master giving a prescription for the insidious management of slaves. Some examples are to establish and maintain strict discipline, awe them with a sense of their master’s enormous power, and impress the slaves with their own helplessness, which conditions them to perfect dependence upon their masters. This way is another perspective on individual and cultural trauma. “The emotions of shame, disrespect and humiliation all are based on fear, while vulnerability is vigorously shamed in our culture” (Jordan, 2008, p.238). In my perspective, disrespect is a second-level description of an emotion that is a response to a basic emotion. It is usually expressed as a behavior. Subsequent to that discussion of cultural trauma, Jordan (2008) claimed that the dominant culture is invested in a denial of that fear that was conditioned throughout life, particularly in boys (p.238).

Jordan (2008) disputed the theory of the separate self-model of who we are and created the relational-cultural therapy model that works to shift from the separate self-

model, both individually and culturally, to one of connection (p. 240). Her therapeutic model depended on evidence-based models of attachment theory for both the therapist and the patient (Schore, 2003, p.245). This sense of presence for both the therapist and the client leads to what she described as peace and healing, and eventually a social revolution (pp.251, 252).

I am deeply moved by the article, the discussion of relational-cultural therapy, and its implications for the profound healing that nonduality can bring to any circumstance. Jordan (2008) has brought in many political, social, gender, and racial issues, as well as psychological issues. If, in fact, nonduality, as a therapeutically sustainable model can move across cultures and disparate points of view, it can serve as an impersonal perspective that can be shared by everyone (Fenner, 2007).

One of Kiloby's (2014) main premises was that nonduality "is not a promise of future fulfillment" (p.3). It is an invitation to look deeply into our present experience and to realize that the wholeness and healing that we seek is already here (p.2). Kiloby (2014) asserted that by allowing all emotions, sensations, thoughts, and concepts to be as they are, which is to appear and disappear inseparably into awareness, the state of nonduality is reached. This awareness is the experience of nonduality. He does not discuss the difficulty of experiencing the resistance of our need to conceptualize, analyze, figure out, and reach a conclusion.

He also emphasized, along with Fenner (2007, 2015), that this experience provides for a natural healing of trauma and a state of emotional and mental balance. Kiloby (2014) presented a method he called the "Unfindable Inquiry" to have this profound experience. When there is an identification with a negative thought or feeling,

part of his model calls for asking one to find out where that feeling or thought is in space, or in one's body or mind.

With addiction and recovery programs testing the efficacy of this model of healing, studies will be able to be done that can follow patients after they leave the facility. The road to self-awareness in this process is small steps such as notice your judgments. Although I have made a few attempts to discuss the status of Kiloby's (2014) addiction and recovery programs, as of this date I have not had the opportunity to have a conversation with him personally.

Levine (1997) focused on the trauma cycle and its connection to memory. For Levine, talk therapy and drugs are of little use in healing trauma unless the somatic issues of the body are addressed (p.3). Levine's (1997) method of healing, "shock" trauma, which is when we experience unexpected, potentially life-threatening events which overwhelm our ability to respond effectively, is through somatic therapy consciousness work. Shock trauma is quite common and can result from everyday events. What makes an event a shock trauma is that it was unexpected and overwhelmed one's ability to cope with it. This can occur through a car accident or a hit on the head.

Levine's (1997) therapeutic approach is to have a "felt sense" of awareness to listen to "the organism" (p. 73). That is also true of Blackstone (2006). Levine's focus on what is, rather than our interpretations, is also close to Fenner's (2007) radiant mind work. He asks the participant to focus on and describe what is experienced rather than what one thinks it means. I find this a difficult focus for most adult clients. How can anyone share an experience without a container to put that experience in? In my experience, thought just is. It presents itself without intent. Levine (1997) provided

many examples of opening up the senses with exercises that gradually begin to release elements of shock trauma in short moments. He works with the body and sensations and is very gentle in his instructions. He was critical of intense emotional release therapies and preferred that people stay with the sensations they experience, rather than attempting to change them (p. 215). Levine's (1997) work was done in phases and seemed to trust the participant (e.g., the reader of the book) to make decisions as to how to manage reenactment of the trauma (p.237).

Hanh (2001) used the metaphor of the wounded child that we all carry within us (p.43). His main method of healing that wounded child is to listen with compassion. This metaphor of a wounded child has been used by many authors for many years, yet when I read excerpts from his book I begin to experience the work he is speaking of as understanding Jung (2001). He spoke of the energy of compassion as the only way to heal anger. Compassion is the antidote to anger (p.85). Hanh (2001) achieved such compassion by listening without correcting any perception that appears, which allows the "real" person to emerge. This listening can take hours or days. Nondual healing employs the same practice as Hanh's (2001) "forever" listening with the message that one can listen for infinity. Of course, this is a lifelong process, and once begun it becomes a natural way to confront whatever traumatic release one encounters. Although difficult for me, I have taken that idea of forever listening to heart. My children have benefited from this and can talk with me with ease.

Hanh's (2001) work with the wounded child is not a structured process. The idea of compassion being the antidote to anger resonates with me personally and with my research question--although this technically cannot be classified as nonduality. Pure

listening can only occur when the receiver of the communication is in a nondual state of awareness (Fenner, 2007).

Nixon (2013) searched for his personal healing from trauma and used several methods, such as psychodrama, catharsis therapy, and somatic experiencing which were helpful initially but could not expedite a full recovery. He went to India in 1980 and studied with many nondual teachers. Through the experience he eventually discovered that the separate self is an illusion and decided to live his nondual passion. He was able to integrate transpersonal and nondual work in his work with himself and his clients. Nixon (2013) called his work choice less awareness. He tested his work on himself and recognized it was not the trauma itself but his interpretation of and stance against what occurred. He contended that a response to new trauma does not disturb him because he had let go of his need to survive (Nixon, 2013). He then began to work with past life trauma and again contended that after he accepted there was no way out, he was able to see himself as part of a new generation of psychotherapists who “non-judgmentally embrace the traumatic experience” (p.10).

I recognize all that Nixon (2013) discussed. I can imagine how his work and how its emotional acceptance of death can facilitate healing from trauma. Yet, I personally have not experienced that myself, as of this time. I have not found other practitioners or authors who say what Nixon (2013) asserted. Death is a concept I have been obsessed with since I was 5 years old and was told by my mother that if I “upset my dad, he could die.” As discussed earlier, he was recovering from a heart attack and we were very careful where we stepped. We did not want him to die, and for sure we did not want to be the cause of it. I still experience the fear of death regularly.

Van der Kolk (1989) discussed how some clients have a compulsion to repeat and revictimize themselves through a reenactment of the trauma (pp.389-411). He asserted that traumatization occurs when both our internal and external access to support are inadequate to cope with external threat. Van der Kolk (1989) continued by asserting uncontrollable disruptions or alterations of attachment connections lead to the development of post-traumatic stress syndromes. Individuals look for augmented connection when faced with seemingly external danger. Adults, as well as children, may develop strong emotional ties with people who intermittently harass, beat, and threaten them (Van der Kolk, 1989). I have seen this phenomenon with myself and my clients throughout my teenage years and throughout my therapy practice. This distinction between self-discovery and provoking the original trauma depends on what and who brings about the “aha.”

According to Glovinsky, (2018) childhood trauma can be thought of in terms of making friends with one’s enemies. It reminds him of the difference in the philosophy of karate vs. aikido. He prefers the philosophy of aikido because it emphasizes “joining” rather than opposing (I. Glovinsky, personal communication, September 2018). The agenda for nonduality in action is practical and simple, one step at a time, one memory at a time. There is no opposition as in karate; rather there is using the position of the memory as in aikido.

Prabhu and Bhat (2013) used Eastern philosophies such as yoga and Vedanta, and quantum physics that approach consciousness studies from a nondualism foundation, and compared them with the Cartesian model. In their work, they advanced reasons why they see the current mind-brain model in neuroscience as not supporting dualism. Although

this article supports my position that an orientation towards nonduality is becoming established in neurosciences as an accepted model, it does not offer practical insights about how one can learn to see the world accordingly, nor why doing so can prevent mental illness. Yet, many nondual practitioners practice yoga.

Therault's (2010) experience of his own personal transformation into nonduality led him to embrace nondual psychology as his preferred methodology of working with clients. He discussed the limitations of conventional forms of trauma resolution. He was thoroughly familiar with many methods of therapeutic interventions and discussed the benefits and limitations of each therapeutic approach, such as CBT, psychodrama, Peter Levine's somatic release work, meditation, and contemplative study.

Therault (2010) asserted that "nondual wisdom and psychotherapy is not for everyone when attempting to heal the scars of trauma" (p.9). When I read this, I do not believe Therault (2010) really understands and has experienced nondual states. Fenner (2007) and Blackstone (2009) support my own personal experience] as well as other practitioners who speak of this state as one without content. Therault (2010) expressed the fear that some people may not be ready for this experience. I believe we can meet each other at the level where we are, so the idea of readiness is not really meaningful in my work of nondual therapy. In conventional interventions, the therapist is separate from the patient/client and observes and analyzes the responses from the client to assure feelings of safety for the client. The idea of safety runs prominently in many books and articles.

I believe the concept of safety is a duality perspective and not applicable to a nondual state of experience. There is an opposite to safety, which is danger. In a nondual

state there are no opposites and, therefore, danger is not a possibility. Ira Glovinsky, in a personal communication (2018), asked the following:

How would this statement fit with current neuroscience of sympathetic-parasympathetic and hypothalamic-pituitary-adrenal functioning? It would seem that there would be a difference in brain imaging as the amygdala would be lit up in danger and that is a physiological response built into our systems. The same neurological response would not happen with a feeling of safety.

Glovinsky's question is an important one to explore for researchers who are working with current brain neuroscience. I do not have the data to say what happens in the amygdala when the subject is in a nondual state. However, when I am accessing a nonduality state, fear is not possible. Instead, in situations where I would ordinarily have my heart beat faster and my brain confused, I perceive what the right action is and act accordingly. That is not a guarantee that my heart will go back to a normal rhythm. Rather, it reminds me I am in a trauma-based reaction and that I need to stop and breathe.

Van der Kolk (2015) raised the question of how feelings can return to people who have a negative sensory response to trauma. Also, how do people regain control of their life when their animal brain, the amygdala, is in charge of their everyday life? Every moment can seem like a fight for survival for the person who has not recovered from and is living his or her early childhood trauma through regular PTSD responses. When trauma is not resolved, the body is the field for a war. Van der Kolk (2015) has three main goals for recovery from early childhood trauma. They are the following:

- to draw out the sensory information that was blocked in childhood
- to befriend our inner experiences

- to complete the thwarted physical actions due to terror

Van der Kolk (2015) assured that self-awareness allows people to become whole again. Although he used many traditional therapies to achieve this wholeness, he favored sensory self-awareness. Sensory self-awareness is a precursor to unconditioned awareness or nonduality. Although he does not specifically mention nonduality as a method of achieving health after trauma, the results of his work mimic the work my research intends to provide (p.98). That is, my experience and my research are based on six principles that are part of every trauma resolution if the steps in the process are followed. It is my conjecture that Van der Kolk (2014) experienced an awakening through healing his own childhood trauma, whereas my autoethnography shows how I have been doing that since my teenage years.

The final scholar/practitioner, Werner Erhard, was an inadvertent pioneer in the world of nonduality. Erhard combined his Erhard Seminar Training (EST) with Zen ideas about *satori*. Laycock (2014) asserted in Buddhism, *satori* means sudden enlightenment. These ideas are followed in the EST training. There were moments in the EST trainings when I thought I was enlightened. They didn't last. In one of my meetings with Peter Fenner he shared that before he participated in the EST trainings, he did not see how he could take his years of training as an academic and 9 years as a Buddhist monk and present a program that would successfully teach nonduality. After working with Werner Erhard in EST, he saw a way to present his work to a Western world.

I have been to three of Erhard's classes. My original EST training was in 1983. I then did a 6-day EST training in 1984 and most recently, in 2015, I participated in a 10-day leadership training. During these trainings, Erhard and his associates used loud

voices, seemingly harsh words, which do not show obvious compassion to many participants who are crying or sharing their stories and their emotions. Why is that? Over 800,000 people have participated in these programs, and the number keeps growing. I believe EST used a disruptive learning technology which goes beyond the words and penetrates the conditioning to access what I call *the broken heart of our inner child*.

For Erhard, suffering is created by dualistic thinking and attachment to concepts that have no corresponding reality. He describes the alternative to this worldview as enlightenment. Within enlightenment (or getting it) is a heightened state of awareness in which dualistic thinking is overcome. All of EST—the long seminars, the exercises intended to confound discursive logic, and the authoritarian attitude of the trainers—is presented as technologies that lead the trainees to this singular experience of getting it. Fenwick (1976) believed that in addition to relieving suffering, getting it is also intended to make the EST graduate a more effective and successful person. Ethnographic studies of EST have attributed these ideas to Buddhism. Fenwick (1976) believed that the EST commentary on acceptance became increasingly Eastern in philosophy. Stephen (an EST trainer) warned of the dangers of being “attached to things”.

Early Childhood Conditioning

The following group of articles reflect the work of academic scholars whose studies and insights on early childhood conditioning are important and relevant to this study. They explore early childhood trauma and the reactive response it produces that can negatively affect an individual’s life. Some of these articles will refer to mindfulness as a way to mitigate early childhood trauma. I include them in my review of literature because, as of now, I have not discovered articles where nonduality is used as a healing

modality for early childhood trauma. Rather, mindfulness, transpersonal psychology, and other Western conceptual models are represented. Although I am not looking at mindfulness as a proxy for nonduality, I am sharing some of the research that has been used in mindfulness that could be relevant to nonduality.

I group the mindfulness articles together so that it will become clear that mindfulness is still in the stages of experimentation as it pertains to nonduality and it may or may not be a precursor to nonduality.

Bittner et al. (2007) correlated childhood anxiety disorders and adult psychiatric chronic behaviors. In particular, they focused on testing the predictive specificity of separation anxiety disorder (SAD), overanxious disorder (OAD), generalized anxiety disorder (GAD), and social phobias. These untreated anxiety disorders lead to depression and alcoholism (p.1174). This study is only one of a few longitudinal studies of childhood and adolescent psychopathology in a community setting. Data came from the Great Smoky Mountains Study (GSMS). A representative population sample of children – ages 9, 11, and 13 years at intake – was followed to age 19. Diagnoses of both childhood (before age 13 years) and adolescent psychiatric disorders (age 13 to 19 years) were available from 906 participants (Bittner et al., 2007). Many of these children were Native Americans and rural subjects from the Smoky Mountain areas.

The researchers paired different anxiety disorders with psychiatric disorders, such as depression and alcohol use. This study suggested that inhibitions and separation anxiety in early childhood were associated with the later development of anxiety disorders in adulthood. The authors contended that inhibitive behavior in young children where fear and withdrawal are displayed deserved more attention (Bittner et al., 2007).

Although childhood anxiety disorders are predictive of many adult disorders, childhood SAD did not predict adult separation anxiety.

They examined possible gender differences, and all associations were tested for interaction with sex of respondents. They found only one formally significant sex-by-predictor interaction: The association between childhood generalized anxiety disorder and subsequent substance use disorder was significantly higher in girls than in boys (Bittner et al., 2007). This study suggested more attention needs to be paid to these childhood anxiety disorders based on predictions for future depression, disability, and chronic impairment.

In my own case, there was physical abuse by my father and emotional sexual abuse by both parents. I was unusually resilient until I gave birth to my first child, after which I began a long history of a psychosomatic and fearful relationship with my body, which eventually ended up with a short mental hospital stay and a prolonged psychiatric journey, which I will describe more fully in Chapter 4.

Gagnon, DePrince, Chu, Gorman, and Saylor (2016) hypothesized that both mothers' and children's experiences in betrayal trauma, such as abuse by a caretaker, and the subsequent child symptoms following that trauma, can be significantly mitigated through learning the language and understanding of emotion. Betrayal trauma includes interpersonal violence that occurs in the context of a close relationship (Freyd, 1996, Brown & Freyd, 2008). Freyd (1996) posited there is a "social utility" for a person to remain unaware of abuse or disassociated when the perpetrator is a caretaker.

I have seen this phenomenon with our son who was adopted when he was 3 years old from an orphanage in a country besieged with poverty. He was there from the

time he was 20 days old to when he was adopted at 3 ½ years old. Throughout his life, from the time of early adolescence, he has asserted that if only he could remember what happened in that orphanage, his life would be more successful. This study offers a suggestion as to why he has not been able to remember any of his time before we adopted him.

Children who are exposed to this betrayal by a loved one are at a great risk for developing both internalized and externalized symptoms. Internalized symptoms include withdrawal and memory loss, and externalized symptoms include negative acting out behavior. When a mother has been exposed to betrayal trauma in her own childhood, the possibilities of developing PTSD, dissociation, and depression increase (Gagnon et al. 2016).

The researchers examining the link between betrayal trauma and psychological wellbeing in children and mothers tested emotional skills in the following studies.

1. Emotion language: the use of emotion-based words and behavioral expressions of emotions (Denham, Cook, & Zoller, 1992)
2. Emotional understanding: the ability to identify emotional expressions and situations that elicit them (Denham et al., 2003).

Gagnon et al. (2016) examined these two abilities in parents and children.

Conversations between parents and their children about emotions and how to use them are crucial for children to understand what is going on around them (Denham & Auerbach, 1995). In essence, parents and caregivers are the direct educators and serve as “external regulators” of the children’s emotions. Denham and Auerbach (1995) believed that to the degree that parents have these conversations directly with their children, the

children gain the ability to discuss and regulate their own emotions. As yet, there are no data to determine what other influences contribute to betrayal trauma.

Denham and Auerbach (1995) used a simple behavioral task in which emotional language and emotional understanding were reliably coded based on the parents' and children's performance on a facial affect perception task. Denham and Auerbach (1995) had a small sample size ($N=47$), and since there is a dearth of research in this area a new coding system was developed. Lower scores on EU (emotional understanding) and EL (emotional language) were associated with greater symptoms in the children. The betrayal trauma history variables included demographics, and there were no significant differences based on age and racial mix. As expected, the stronger the language of emotions discussed, the less symptomatic the children. I believe early childhood trauma in many cases inhibits the experience of emotions and the memories that are bonded to those emotions in some children.

With respect to the research itself, I am not sure if the tests developed in the early '90's are still relevant and culturally relevant. Cultural significance has to do with the values, norms, art, literature, and traditions of a particular community. The '90's and today are culturally different through the structural change made by the Internet. It seems obvious to me that when parents discuss emotional issues and what emotions are with their children, the children and the parents can learn the language and understanding of emotions to a greater degree than if there is silence or concealment. Since each participant dyad (parent and child) was a subject of betrayal trauma, and the parent showed symptoms of this condition such as silence, depression, disassociation, and so forth just discussing the issue, a sense of relief can occur. I assert emotional

intelligence is no longer a luxury for families. I postulate it can save lives through allowing communication to create language that each of us can connect with, the language of emotion.

Lang and Connell (2017) wrote an article on the development of a childhood trauma, which posited that childhood exposure to trauma is a major public health concern. Nationally, 71% of all children are exposed to one or more traumatic post traumatic events (PTE), which includes physical or sexual abuse and domestic and community violence (Finkelhor, Turner, Shattuck, & Hamby, 2013). This exposure has permanent lifelong effects, leading to substance abuse, health problems, premature death, and behavioral problems (Adams & Boscarino, 2006; Felitti, et al., 1998). The economic impact of childhood trauma has been estimated at \$103 billion annually (Fang, Brown, Florence, & Mercy, 2012). This impact was based on lack of work productivity along with increases in health care and social service costs. I find this statistic staggering in that we do not acknowledge a lack of work productivity due to PTSD or trauma. Rather, we create trainings and interventions based on more conventional explanations and models.

With the recent recognition of the prevalence of PTEs, there is a need to create “trauma-informed systems designed to use best practices to prevent this trauma exposure and to provide services to children” (Ko, et al., 2008, p.396). Child trauma screening is increasingly used in many different settings. There are many different screening instruments and processes; however, they tend to occur in health facilities after a traumatic event has already occurred. For example, Ko et al. (2008) developed the Child Trauma Screen (CTS) as a short, no-cost screen that can be used by people outside clinical settings. Two studies were conducted to test this screening measure. The

conclusion of these studies conducted by Ko et al, (2008) was that the CTS fills a significant gap to apply a very brief trauma screen that can be used very quickly in many different settings. The goal is to eventually use this or an adaptation of this tool as a universal trauma screening mechanism to address childhood trauma. The authors conceded that although this screening is not as comprehensive as longer measures, it is easy to use, and many people can administer it (Ko et al., 2008). The 71% of children who have experienced a traumatic event in early childhood is either too large or too small, based on what is defined as trauma and who is making the definition. In communication with Ira Glovinsky, he stated “This is exactly right and crosses over to numerous areas where people speak to each other about a topic and they see the topic differently” (I. Glovinsky, personal communication, September 2018).

I do not know anyone who has not experienced a traumatic event in childhood. The cost to society is evident to me, based on our politically and culturally divided country and how we react to circumstances day to day. I believe the authors and their research are limited by what is defined and what is not defined as trauma and trauma-related events in a child’s life (Ko et al., 2008). It is my observation that many children and adults have experienced trauma-related events and we do not recognize and acknowledge these events. According to a survey by the National Survey of Children’s Health (NSCH, 2013) , nearly 35 million children in the US have experienced one or more types of childhood trauma. This is almost half the nation’s children (National Survey of Children's Health (NSCH), May 13, 2013).

On the other hand, perhaps we can provide parental training before and upon pregnancy so that the family can understand what happens to a child when it is put into a

world where the parents are not aware of the effect of their behavior on that child, each other, and the community.

Shukoor (2015) discussed the refugee PTSD perspective of children traumatized both before, during, and after their journey to Australia from Iraq and Iran, using two in-depth case studies. He hypothesized that PTSD damages individuals' mental and physical health causing future psychological disturbances with similar neurological effects across age groups, no matter their race or culture. Although this research on refugees is still at an early stage (this article has not been peer reviewed as of now or submitted anywhere else), there is abundant research on the effects of trauma in general. He has also presented a recovery model and proposes several recommendations to working with non-Western children and their families.

Shukoor (2015) also asserted, as of now, financial and social entitlements from refugee claims from Australia are growing in that these needs are costing the host country more money than anticipated. Over 4,000 children are living in fear of being sent back to the country from which they recently escaped (Shukoor, 2015). There is a prevalence of PTSD which can occur from naturally occurring disasters or human causes such as rape, war, physical and psychological assault, sexual abuse, and more (Shukoor, 2015). Refugees displaced from war-torn countries are most vulnerable groups for PTSD. Shukoor (2015) described three stages of traumatic experience ranging from what occurred in the original country leading to the first loss during their flight to safety, tolerating exposure to danger, and separation from family and loved ones, and needing to prove their eligibility for asylum status while integrating into their new society.

Of course, there are practical issues in the counselling of refugees that are different from non-refugees. They include language issues to overcoming trust, cultural divides, and family perceptions of mental distress. Asian Pacific, Arab, and Middle Eastern refugees are overly sensitive to any suggestion of mental issues (Nasser -McMillan, 2003) including depression, and this can prohibit families from getting the help they need. Fazel, Panter-Brick, Reed, and Stein (2012) made the connection that Hanh, T. N. (2001). *Anger*. New York, NY: Penguin. I believe with them that adverse events and ongoing stress from parents and caregivers were strongly correlated with their children's mental health.

I found Shukoor's (2015) article beneficial to the study of refugee PTSD and PTSD in general. Besides being well written, easy to understand, and meaningful and germane in today's world, it can support using Infinite Choices and the Six Principles in order to dissolve negative childhood conditioning. The recovery model that Shukoor (2015) proposed is one that, with a few changes in focus, can be adapted for different variants of PTSD. His requirement that caregivers become aware of their personal values, beliefs, and cultural points of view is one to which I strongly subscribe. The framework of recovery model by Victorian Foundation for Survivors of Torture (1998) involves the entire family that includes young children and the community.

Mindfulness

There are many studies on the value of mindfulness and there is a debate as to whether mindfulness serves as a precursor to nonduality. I will discuss a few articles related to mindfulness that I believe are important in this study, primarily to differentiate mindfulness from nonduality. For a detailed discussion on mindfulness and its uses and

value in different stress-related diseases and symptoms, please see Baer (2003).

Mindfulness practice and meditation are not in any way conflictual with nonduality. I believe there is an invisible aspect to mindfulness programs that has not been able to be measured, as far as I can surmise. I am very interested in measuring that invisible essence that is being discussed in each article I have read so far. The experience of nonduality may be the unrecognized aspect heretofore not named in studies of mindfulness.

Rahl, et al. (2017) has demonstrated how mindfulness meditation diminishes loneliness and alleviates stress in older adults. It can reverse these patterns, even during stress itself. Davis and Hayes (2011) showed how chronic mood disorders can be reduced when regularly practicing mindfulness. There are positive results based on stress-related diseases, PTSD, ADD, and other related disorders. As an example,

Twenty nonclinical beginning meditators who participated in a 10-day intensive mindfulness meditation retreat were compared to a control group on mindfulness, rumination, affect, and performance tasks for attention switching, and working memory. Following the meditation retreat, the meditation group had significantly higher self-reported mindfulness, decreased negative affect, fewer depressive symptoms, and less rumination compared to the control group. In addition, the meditation group had significantly better working memory capacity and greater ability to sustain attention during a performance task compared to the control group. These results are but one of many that can be found in Davis and Hayes (2011).

Braun, Levy, Collins, and Mogilner (2014) have written a very simple

mindfulness study (Braun et al., 2014). However, I feel this study is very relevant as the authors of the study felt that the teens were at risk for future negative lives. I am interested in how therapists can find innovative ways to foster and maintain trust as they work with clients to ameliorate early childhood trauma.

The results were statistically significant, showing a decrease in self-perceived stress by the participants (Braun et al., 2014). Positive side benefits included confidence to handle personal problems and each participant's ability to feel confident in his or her capability to handle conflict. Braun et al., 2014) asserted, based on these results, that introducing teens to yoga and mindfulness can be used to stop a cycle of hurtful behavior with adolescents. Braun et al. (2014), made the very important point that they cannot identify which activity in the program made the difference. Was it the yoga, meditation, snacks, etc.? This point has been made in each study about mindfulness I have explored. It was important to the researchers that participants trusted in the medical profession and the community center environment where the program was conducted. The researchers wanted to be trusted themselves.

The Braun et al. (2014) article is the first I have found that used mindfulness on groups other than White middle class. I believe there is a transmission from a person that lets another know nonverbally what is occurring in the space that implies safety or danger. Perhaps by doing yoga and meditation and eating healthfully, the mind enters a nondual space for short moments and that is what makes a difference. Perhaps there can be more studies of this nature with people of color and high-risk populations that are not usually offered this opportunity.

Krageloh's (2016) discussion of the importance of morality in mindfulness practice presented an important explication of mindfulness and its practitioners and the ways mindfulness has been used in different models of psychotherapy. It also looked at the differences between Eastern (Buddhist) practice of mindfulness and Western (e.g., Kabat-Zinn) practices of mindfulness. Some of these are acceptance and commitment therapy (Hayes, Strosahl, & Wilson, 1999); dialectical behavior therapy (Linehan, 1993); mindfulness-based stress reduction (MBSR, Kabat-Zinn, 1990); and mindfulness-based cognitive therapy (MBCT; Segal, Williams, & Teasdale, 2018).

Although a large body of empirically substantiated literature associates mindfulness with positive health and psychological outcomes (Baer, 2003), it is not clear why. In a personal communication from Alan Davis (July 2019) he stated,

"To me, this suggests that there must be critical aspects or processes within the general label of mindfulness that are essential to its therapeutic effectiveness, but they haven't yet been identified." I agree with him and suggest this essentiality is the state of unconditioned awareness of the practitioner and how he or she transmits it to the client/patient.

Perhaps the Dalai Lama's practice of kindness may be a partial answer to why nonduality and mindfulness have such an effect on both the practitioner and the client. Typically, there is an educational element followed by guided exercises. There are no guidelines to insure the mindfulness-based interventions (MBI's) are developed and delivered by competent counselors (Brown, Marquis, & Guiffrida, 2013). However, the techniques used in MBIs are adaptations of Buddhist traditions that have been westernized.

Krageloh (2016) discussed the secularization of mindfulness practices and how religious and ritualistic aspects of Buddhist-based mindfulness have been removed from the training and teaching. The rest of the article questioned the issue of practitioner morality and how that makes a difference in mindfulness practice (Krageloh, 2016). Without evaluation and personal ethics of the practitioner, there was a concern that unwholesome states of mind of the practitioner can corrupt the practices (Bodhi, 2011; Dreyfus, 2011, Kang & Whittingham, 2010). I believe Krageloh (2016) touched at the very heart of mindfulness and nonduality practice today and how it is being used and taught by many people who do not have the depth of training in its core of knowledge and are not actively involved in their own practice. The issues of morality and ethics need to be raised and seriously questioned. I believe Krageloh recognized that each practitioner would be best served by understanding that the secularization of mindfulness does not remove the need for personal awareness of the values and ethics the counselor and practitioner need to bring to their work with students and clients.

The idea of values and ethics is not at odds with the essence of nonduality, which is oneness. We live in a relative world in which values and ethics are crucial for the therapist and a nondual world of oneness in which the values and ethics are naturally communicated simultaneously. When I am working with a client, and I have an agenda for that client, that lets me know I am not in unconditioned awareness and my conditioning is being projected on the client.

At this time, there is not a certification process to become a mindfulness or nonduality practitioner, nor is Krageloh petitioning for one. Yet, both he and I are suggesting that the field needs new ways of looking at these teachings (Krageloh, 2016).

Without personal self-awareness, the practitioner, just like the parent or caretaker of the child, cannot know if there is long-term damage being foisted on the child/patient (Krageloh, 2016).

Meiklejohn, et al. (2012) reviewed how mindfulness practices have entered K-12 education over the past decade after their success in medicine and stress reduction. Meiklejohn et al. (2012) suggested integrating mindfulness training into K-12 education, believing it will foster the resilience of teachers and students. They make the case to continue to research and test for evidence of the benefits of mindfulness-based interventions in K-12 education. Since 2005, 14 studies of programs that train students in mindfulness have demonstrated benefits in areas of cognitive, social, and psychological benefits to elementary (6 studies) and high school (8 studies) students.

The improvements are manifested in many areas, including memory, attention, academic and social skills, emotional regulation, and self-esteem (Meiklejohn et al., 2012). Even with all of these successes, they suggested the need for even more scientific evidence of the benefits of mindfulness-based interventions (Meiklejohn et al., 2012). They also make the point that all the mindfulness programs have been stripped of any religious or spiritual aspects of the original intent and are secular in nature.

These studies are relevant to nonduality practice by bringing up both morality and a spiritual relationship with the therapist. I believe that when the therapist and the client share from a holistic perspective that includes the spiritual rather than a secular perspective that is devoid of the spiritual, there is a deeper connection. In my experience that is when the merging of experience can occur through content-less awareness or nonduality. There is extensive peer-reviewed literature exploring the nature and

application of the potential efficacy of mindfulness-based practices, especially in areas of cancer, fibromyalgia, chronic pain, anxiety disorders, and depression (Shapiro, Carlson, Astin, & Freedman (2006); Shapiro & Carlson, (2009) Davis & Hayes, (2011); Segal, Williams, & Teasdale, 2018;). There is also convincing data that in adults, the neuroendocrine and immune system function evolves. The point of this review is that by supporting K-12 educators and students in direct and indirect ways, the educator can influence the students in their classes to be more mindful and, in my opinion, more successful in their everyday lives (Meiklejohn et al., 2012).

Meiklejohn et al. (2012) are convinced they needed to continue to prove that mindfulness training is an answer to anxiety in the classroom and a benefit to overburdened educators. So far, in the literature I have explored, no one knows why mindfulness works or does not work. I contend that without a spiritual and ethical component to mindfulness and education in general, we are missing the mark. When volunteering in a fifth-grade classroom in a Boulder elementary school last spring, I discovered that all the students wanted guidance in how to behave and always wanted to know *why*. Bringing another component such as mindfulness that affects the personality and points of view of the students into the classroom without the teacher or practitioner knowing what they are energetically teaching is confusing (I. Glovinsky, personal communication, September 2018). Exactly! When we teach kids, we tend to focus on the cortex rather than on the brainstem and limbic system. Having book knowledge of an area is not the same thing as experiencing it spiritually.

Blackstone (2007, 2008, 2009) asserted that the teacher or therapist shares her inner experience with the student or client without words. We may not be aware of what

our inner life is experiencing when we are working with students or clients. When I engaged with fifth-grade students in one of my praxis semesters, they matched my inner dynamics in their behavior each week I was working with them. When I was without judgment or agenda, they opened up and shared the content of their lives without prodding or questions from me. I experienced gratefulness that I was able to share my time with them and how I became part of their lives one day a week. I was also able to answer their questions and provide meditations for them to take home to their families. Mindfulness with spirituality and/or a nondual perspective of a middle way may be difficult to bring into the classroom where it can have deep and permanent positive results for the teacher and the students.

Mentors and Teachers: The Development of My Own Perspective

The next area of studies to be reviewed is that of my past mentors and teachers who have influenced my work over the past 50 years and who continue to have an influence and make a difference in how I see the world.

As the director of the Child Guidance Clinic, Salvador Minuchin began his work with welfare clients, both African American and Latino. In the mid-1960s, Dr. Minuchin was the director of psychiatry at Children's Hospital in Philadelphia, director of the Philadelphia Child Guidance Clinic, and a professor at the University of Pennsylvania Medical School. He received a grant to train people of color as paraprofessionals. Most of the staff at Horizon House, like myself, were paraprofessionals. We worked personally with him and his trainers once or twice a week during my tenure at Horizon House. His trainers were Jay Haley, Braulio Montalvo, Marianne Walters, RD Laing, and others. They all developed sterling careers of their own.

Jay Haley, one of his trainers, was enthusiastic about the idea of training people who had not been “contaminated” by academic knowledge. The training programs were inclusive and successful. I was fortunate to be one of the trainees, although I was not African American. In Chapter 4, the autoethnography, I share two particular experiences with Salvador Minuchin and how I began to see the world upside down and counter intuitively.

Minuchin, Reiter & Borda (2013) asserted that universities are not training therapists to be comfortable with uncertainty, but rather to fall back on theory when not knowing what to do in an uncomfortable family session. Minuchin, Reiter & Borda (2013) urged therapists to “join in” with the family as soon as possible. He asked, “If not, what theory will new therapists embrace?” (p. 3). Even when the therapist chooses a model from integrative theory, it is still a model, not based on what is happening in the family session at the time (I. Glovinsky, personal communication, September 2018). In addition, Glovinsky stated the following:

This is an important statement and really needs to be considered in developing training programs. I find that students look to be working within a theory as this makes them feel grounded. However, when working with families, going back to a textbook is not the same thing as learning to work with one’s self.

Minuchin (2014) posed the question of why, with the same techniques and exercises, is one therapist successful and the other not? His answer suggested that empathy, compassion, and the self-awareness of the therapists regarding how they fit into their own family system are key (p.4). Many believe this is simply a matter of individual differences. I personally believe this is more of the individual therapist’s conditioning.

With the experience of and training of nonduality, therapists or educators would not be hampered by their own conditioning much of the time. They would have done their personal work before working with clients. They would be able to see what was going on with the patient or client and respond in a nondual manner.

Minuchin used the family members as co-therapists, as well as using a unique technique that he taught the group workers, of which I was one, during his weekly training sessions at Horizon House in the late '60s. He would tell one of the children in the family that they he or she was a crybaby, and then tell the mother she caused the “crybabies” by treating the child like a baby (Minuchin, 2014, p.23). This type of interaction would create a new narrative, and the family would mobilize in their capacity to protect each other. Some of his concepts that I have used since working with him in my late 20s are the following:

- Families are complex social systems containing subsystems with different agendas based on the content.
- Each person has different selves based on a particular situation, and the therapist is only dealing with a partial self at any given time.

This last concept from Minuchin (2014), that therapists are only working with a partial self at any given moment, gave rise in me to the idea that black and white and either/or realities are not the whole picture. Also, I recognized that cause/effect usually did not make sense or feel true. I was able to see systems of dynamics in which blame was not really a truth to be used. Instead, by seeing how each aspect of the system that showed up was relevant, many conflicts began to disappear (Orloff-Falk & Doubleday, 1991).

Because Minuchin had a challenging style, would open conflictual issues, and believed this approach would lead to an experiential shift in the family structure, he could mobilize the family's resources to work toward healing each other. He did not doubt their capability. He was disturbed that the inductive method of reasoning was being replaced by the deductive method. He felt this shift would have "severe consequences" for the families. He did not say what he meant, exactly.

Laing (1967) asserted that psychotherapy can give us the hope of meeting someone authentically. He did not think this was common (p.46). He emphasized the need for a universal theory that would be inclusive of all theories and practices of what the ontological structure of human means (Laing, 1967). This concept intrinsically shows how the interaction and inter-experience of two people works. The concept of inter-experience needs to be able to communicate the relationship between each person's experience within the context of the relationship existing between them (Laing, 1967). Of course, he did not forget the prevailing social system of his times.

I also am looking at nonduality as a universal foundation for healing early childhood trauma by incorporating Laing's teachings into mine--his discussions of the relationship between the therapist and the client as well as the inter-experience of them both. By inter-experience Laing means our experience of each other that cannot be shared, as it is invisible. So, how can I really see your experience of me? In the nondual experience we are sharing the exact same experience at the exact same time, albeit it is invisible. Even as I write this, I see the difficulty of showing how this is possible in words. I do believe it possible to discover a way to document this.

Laing (1967) examined the mainstream of orthodox psychoanalysis and different

theories of object relationships in England, such as Fairbairn, Winnicott, and Melanie Klein's work and the existential schools of psychotherapy, such as Binswanger, Boss, Caruso, and Frankl as examples (Laing, 1967). He also mentioned learning theory, ethology, systems theory, communications analysis, information theory, transactional analysis, object relations, and games theory (Laing, 1967). He was very prolific in his knowledge and synthesis of theoretical perspectives.

Laing (1967) stressed the importance of "shared social collectivities of experience between two people, rather than just the meeting of an ego with an id" (p.52). As stated earlier, but restated for emphasis, Laing contended, "Psychotherapy must remain an obstinate attempt of two people to recover the wholeness of being human through the relationship between them" (p.53). Any method of psychotherapy that keeps the therapist separate from the patient perpetuates the disease that brought the client to the therapist in the first place (Laing, 1967, p.55). I believe the positive attachment of the therapist with the client allows an intimacy that each of us desires/d with our family of origin. Connecting deeply does not mean there needs to be a transference and countertransference that can hurt either the client or the therapist.

Again, I was fortunate to train with Laing during the time I worked for Horizon House in the late '60s. We were apprised that he was a schizophrenic who was going to teach us about schizophrenia. His assertion that we cannot experience another person's experience--"Experience is man's invisibility to man" -- has haunted me throughout my life and has brought about an urgent curiosity and a desire to determine if this statement is true (Laing, 1967, p.18). I have searched for common responses to situations that are the essence of being a human, and a universal healing process that will override

individual experiences. The work of Laing is complementary to my own work on nonduality and has sustained my therapy practice from the moment I met him.

Haley (1963) asserted that for the first 100 years of psychotherapy, trainees primarily learned therapy by experiencing it themselves. The assumption was that having personal therapy oneself created a competent therapist (p.vii). In fact, if there was a “tragic development” with a patient, the supervisor would ask the therapists to examine their reactions, and then send them back into the situation wishing them good luck (Haley, 1965, p.viii). He contended that therapy, like art, needed to be taught in an apprenticeship system (Haley, 1963, p.5). His mentors were unorthodox practitioners like Alan Watts, Erik Erikson, Salvador Minuchin, Gregory Bateson, Braulio Montalvo, Don D. Jackson, and others. He, like other game changers of the late '60s, asserted that the instrument of change in therapy was the therapist. I also share that sentiment.

As a family therapist, Haley preferred to see the whole family in the first interview, so he could see the system in process (p.4). Haley's system is called “brief therapy”. Its success is based on two elements: maintaining its briefness, and focusing on the importance of the family system. Even though the individual is presenting the system, he is seen not just as an individual who can be blamed for the problems being presented (Haley, 1963). Therapists needed to be trained to be directive and active in the application of brief therapy (p.26). Haley (1963) revealed he once flew from San Francisco to Phoenix for supervision from Milton Erickson, who was the only therapist he knew who did brief therapy (p.26). Symptoms focused in the presenting session was another aspect of brief therapy, rather than viewing the past.

He described most contemporary therapists--and by contemporary he meant

1996--as using many different therapy approaches, based on the particular presenting problem (Haley, 1963, p.30). He found it difficult to train therapists in his model of brief therapy, which is a problem-solving method he developed with the support of Milton Erickson. He utilized systems theory and paradox in therapy through the work of Erickson (p.31).

As I read this excerpt and other articles by and about Haley, I remember the impact he has had on my work. Many people are situated in a cause-and-effect way of looking at clients. For example, if their parents were distant and divorced, that could be the reason they had an issue with marriage and commitment (Haley, 1963). Haley's work is akin to nonduality, as it is disruptive to the norms of the individual and his/her family system and created a disruption or pattern interruption in the thinking and conditioning of the client. Whereas Haley was directive with the clients I served at Horizon House in terms of the content of the symptom, I don't know how effective his work was in terms of creating self-awareness and understanding in the client.

Summary

Through my review of the literature, I rediscovered that nonduality is an experience that two people could share without their personal thoughts or history interfering; and, in that space, profound healing can occur (Blackstone, 2008; Fenner, 2007). That does not mean that an illness like cancer will disappear or that a broken bone will miraculously repair itself. Personally, I am able to experience a deep sense of contentment even though my body may be in pain or my personal life is conflictual at that time. Because nonduality as a healing/therapeutic modality is relatively new (Fenner, 2007), this review of literature may seem disorderly. I do not believe that is so. I look

forward to studies relating to unconditioned awareness and nonduality to be of interest to the scholar/practitioner as we discover the language to describe the healing experience.

The first section of the review of literature included nondual teachers and scholar/practitioners; the second group were scholars looking at early childhood conditioning and trauma; the third group were mindfulness/nonduality articles by scholars specializing in early childhood conditioning, and the last group were my mentors from the early and late '60s.

This review of literature is not exhaustive. I find it difficult to leave it and recognize how it has opened my mind in ways I did not expect. For example, I believe there are threads of shared knowledge and experience that propel me forward to continue my research and learning. I do sense that the more I integrate the success and failures of other scholar/practitioners, data from studies focused on early childhood trauma and others in my review of literature, the more I open to the possibilities that unconditioned awareness or nonduality bring to me.

CHAPTER THREE: AUTOETHNOGRAPHY AS METHOD

I have come to drag you out of yourself and take you into my heart.
I have come to bring out the beauty you never knew you had and lift you
like a prayer to the sky.
If no one recognizes you, I do because you are my life and soul.
Don't run away, accept your wounds and let bravery be your shield.
It takes a thousand stages for the perfect being to evolve. Every step of the
way, I will walk with you and never leave you stranded.
-Rumi, *The Inner Camino, A Path of Awakening*
(Holloway, Brierley 2014 p. 35).

Overview

Autoethnography as a method for research allows researchers to examine and analyze their lives in order to support social and cultural change. What happened in our family that allowed for physical and verbal abuse and then silence needs to be shared so that there is an understanding of the times, the medical and psychiatric conditions, and the results of that silence and embarrassment. I chose autoethnography so that I could stop the silence and share the bitter and the sweet results of those experiences in 2018. Although telling the story and examining the results of those times will probably allow for an “aha” with readers, it will not make structural change possible in family relationships until the fabric of our communications also changes. If, for one moment, we could see into the future at the results of our behavior, we might be touched deeply enough to interrupt the patterns we learned and begin to grow our self-awareness immediately.

Research Focus and Data

In this methods chapter, I begin with a description of my first night and my short stay in the Philadelphia Psychiatric Center. This was a crucible 3 weeks in my life where

I felt the experience of being awake without an invisible veil concealing what was right before my eyes. This was not the first time I experienced this awakening. It is also true that when I was a small child I also saw what was right before my eyes. I was taught that what I saw was not what I saw. This created a division in me as to who and what to believe.

My bias about insanity is important for me to put forward. I do not believe there is a genetic tendency toward madness. I believe there is an ancestral consciousness that creates our thinking to be inside or outside the range of culturally accepted thinking. I do not believe there is a direct cause-and-effect condition or situation or a series of situations that can offer an explanation to a mentally disordered way of functioning. If a person is not considered “normal” in the social and contextual culture he or she lives in, that person is often given a medical or psychiatric label. These labels are given by medical and psychological professionals based on the “abnormal,” meaning deviations from behavior of the client or patient they are observing that is worrisome or undesirable. I do believe that a spiritual conversion can occur at a pivotal moment in a person’s life which can look and feel distorted to the person him/herself and his/her world. I am a psychiatric hospital survivor as are many others of my time, the mid-’60s, particularly women with post-partum hormone chaos. (Seager 1960)

After the description of my 3-week stay in Philadelphia Psychiatric Center, I describe some periods in my life that led me to Fielding and autoethnography. And then I present a short description of important times in my life that illustrate why I chose autoethnography as my research method. I am breaking up time periods foremost in my thinking, as I believe they are significant. Although all periods and incidents in my or

another's life may seem significant to determine the future, I believe there are periods of evolutionary growth and the concomitant trajectory that accompanies these periods.

I then describe autoethnography as a research approach that is valuable in social change, justice and feminist perspectives in mental health and why autoethnography is the appropriate methodology for me.

Philadelphia Psychiatric Hospital

It is 1964. I was 25 years old. I had been searching for 6 months to find out what was wrong with me. I could not eat. I weighed less than 100 pounds. I wore my long red hair like a schoolgirl. I smoked cigarettes and drank coffee. I saw many doctors. I admitted myself to Lankenau Medical Center in Philadelphia for a complete checkup. Although I wrote all my symptoms on a large paper bag and, in fact, gave it to the doctor and submitted to many tests, they could not find anything wrong with me. I wondered why I felt like I was dying. I gave birth to our second child in January 1964, a girl. I could not feel much. I took good care of her, but didn't feel connected. I had two children under 2 years old and was 25 years old.

In the 4 months before our daughter was born, the following took place:

- *I had an emergency appendectomy, in my eighth month of pregnancy.*
- *My husband lost his job while I was in the hospital.*
- *His mother died suddenly on Halloween.*
- *We decided to raise his 10-year-old sister.*
- *We had to move from Florida back to Philadelphia and live with my parents until he found work.*
- *I began to have irrational thoughts and couldn't eat.*

- *I went down to 98 pounds and could not feel internally or externally.*
- *I would stick needles in my hands and feet to see if I was alive.*
- *I went from doctor to doctor to discover what was wrong with me.*

I was on my way home from seeing another doctor. My parents were watching my children. I reached my neighborhood and stopped at a stop sign. I could not move. My hands would not leave the steering wheel and my body felt frozen. I took many breaths and I was sure I was dying. I could not cry. I felt as if my blood was drained from me and I became a vampire. I lived day to day without feelings and thoughts that make me human. Finally, I could move my hands and feet and started the car again. I found my way home and parked outside of our garage. I didn't remember how to open our garage door. I was immobilized and felt only the need to lie down and stop thinking. I went into my house. My parents and husband were there. My mother looked at me. I felt nothing. I hugged my two children and wondered how I could feel so separate from them.

I looked at the food on the table. All I could see were the hamburgers. They were all waiting for me. I looked at the hamburgers again and said, "I will not eat my daughter". I didn't see anything strange about that. I could see that they didn't agree with me. I went to our bedroom. Alvin, my husband, came in and said, "We are going to the Philadelphia Psychiatric Hospital." He wanted me to see a psychiatrist. I agreed and went with him right away.

When we arrived, the nurse took me in to see the doctor on call. His name was Dr. Melvin Marcus. He was smiling at me in such a loving way. I cried with relief for the first time in a very long while. I told Dr. Marcus was certain I had cancer and heart trouble, or I wouldn't feel the way I did. I told him I felt terrible and had for a while. I

told him I could not eat or sleep. I told him about my past year. I told him about my emergency operation when I was 8 months pregnant. I told him about my husband's mother dying and our taking in his 10-year-old daughter. I told him how I didn't feel much love toward my children or anyone. I told him about all my fears of dying. I told him how my husband lost his job and I didn't know how we would manage. I don't really remember the words I used to speak to him, however I do remember telling him something was terribly wrong. I remember him examining me and suggesting I stay in the hospital for a few weeks so that we could find out what was wrong and treat the problem.

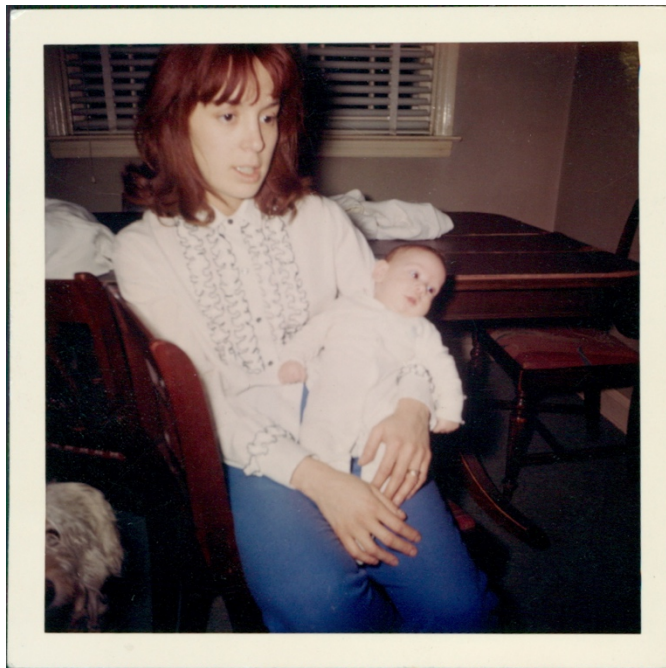


Figure 2. The first night entering Philadelphia Psychiatric Hospital.

I don't remember going to my room, but I do remember my roommate. She told me she was there because of her drinking. I was given a sleeping aid and went to sleep. I don't remember the sequence of events during my stay. At one point I was taken to see

Dr. Cohen. I knew he was important. He sat behind a large wooden desk, much bigger than he was. We spoke for a while. I tried to give answers that sounded normal. He said they would perform tests to determine what was going on with me. He prescribed Valium four times a day. I liked taking it. My anxiety disappeared for short moments.

I was in the hospital for 3 weeks. What I remember about that time seems to glow with a great deal of empathy and love.

A Few Memories

- *Five or six women were sitting around a card table. We were playing cards and sharing. One woman in her 50s told us that she knew there were living creatures in her stomach since she went into menopause. At that time menopause seemed so far away, so very far away.*
- *I went to the gift shop/soda fountain. Behind the counter were two of my slightly older, elegant cousins volunteering. They didn't show any surprise to find me there as a patient. We "chit-chatted." I gave them the fake money for the soda, gave a fake smile and left. I haven't spoken to or spent time with them since that encounter.*
- *We all sat in a semi-circle and there was music playing. A group of women came in and they invited us to stand up to dance to "Do the Hokey Pokey." It is a group dance that I felt forced to participate. I was able to feel embarrassed.*
- *A young man with bright red dyed hair told me how he put shampoo in his father's convertible and took the hose and washed the inside of the car with the top down while his father screamed and screamed at him.*

- *Early one morning a woman was running naked through the rain singing. I found out later she didn't want to get an electric shock treatment that day.*

The most memorable experience was when my parents came to visit. My mother kept telling me the reasons I did not belong there. She made sighs and clicking sounds with her mouth which she did all through my youth. My father looked at me with love in his eyes and together we laughed at some of the people. He was not judgmental or scared. I remembered at that moment who he was.

I walked with a notebook in my hand, long red braids, and no makeup. I was in a school. I remember enjoying my lessons.

And, now I wonder

- How did I get to be admitted to a mental hospital?
- How did I recover, and from what?
- How did I get to be writing this dissertation?
- In the 1960s we were not aware of post-partum psychosis. We were not aware of how the traumatic results of physical and emotional abuse would alter the very fabric of my life even if I found a way to heal myself.

Research Approach - Autoethnography

Autoethnography is a method of arriving at new understandings through a systematic inquiry into oneself. In my experience, it is more challenging than other methods I have explored, including appreciative inquiry, ethnography, participatory action research, photovoice, narrative inquiry, and phenomenology, to name a few. None of those alternative methodologies seemed appropriate for me at this time in my

development. I hope my story may find its way to a teenager or a young mother or father and make a difference in their lives.

Although I experience myself as a conflicted anomaly with fearful thoughts and feelings that intrude in my day, almost every day, an outsider's experience of me is dissimilar from that of my experience of myself through my internal dialogue. If only I were transparent, I would not have to tell my story. Everyone of importance to me that I would meet would already know it and it would be of value to them in their journey. Yet, of course, I am not, and I feel confident my story is not mine alone. It belongs to many women of my time and place in society. This is set against the cultural and social milieu of the Second World War (I was born in 1939) and the American values that came out of that time. I am honored to be alive at this time.

I am glad I am able to share the transformation of the almost exquisite pain of my inside experience to the patterns and moments of my life as a study in resilience and what my mother called a "strong constitution." I name this my abiding will to survive and make a difference in the world. What other research method than autoethnography with its personal acknowledgement, emotionality, subjectivity, and realization that the researcher cannot conceal the researcher's influence on the research can work for me to put my story into words? (Ellis, et al., 2011). Perhaps other scholar/practitioners will read it and decide it is necessary for them to share their story as well. It is no longer appropriate to hide the abusive way any of us were treated. It no longer needs to be embarrassing to tell my story.

Bochner and Ellis (2006) described autoethnographers as people who struggle to overcome adversity in order to support others who read about and use the process the

autoethnographer is describing to look at their own problems and issues and discover their own meaning. Autoethnographers look back at their life experience, describe and analyze that life experience, and offer a way to communicate and tell a story to others that could make a difference

There was a “crisis of confidence” inspired by postmodernism in the ’80s. This occurred after social scientists became increasingly troubled by social science inquiry limited by the ontological, epistemological, and axiological perspectives of that time (Ellis & Bochner, 2000). According to Kuhn (1996) and Rorty (1982), there was recognition that facts and truths were fashioned by the language and paradigms the social scientists used to illustrate them. Over time, scholars from various social science disciplines speculated what research studies would become if the expectation would be that the work would be closer to stories as described above rather than the hard sciences. And they suggested, the pretense of value-free research needed to end and instead the researchers be allowed to share their values in their work (Bochner, 2014). My article written as Judith S. Samuels in 1971 offered that observation without the clarity I have today.

According to Ellingson and Ellis (2008), autoethnography is “a social constructionist project that rejects the deep-rooted binary oppositions between the researcher and the researched, objectivity and subjectivity, process and product, self and others, art and science, and the personal and the political” (p.451). As a method, autoethnography fulfills the role of the process and the product (Ellis et al, 2011). There are many forms of autoethnography, depending on where the emphasis is placed.

1. *Indigenous/native ethnographies*: These develop from colonized or economically subordinated peoples. They look for a disruption in power in the research. This type of autoethnography used to belong to the domain of “white, masculine, heterosexual, middle/upper classed, able bodied Christian” (Denzin, Lincoln & Smith, 2008, p.16).
2. *Narrative ethnographies*: Dissertations presented in story form which integrate the ethnographer’s and others’ experiences in the analysis of others (Tetlock, 1991).
3. *Reflexive ethnographies*: These log and chronicle how the researcher changes during the time of the fieldwork. The ethnographer, in this method, is chronicling his/her life along with cultural members’ lives (Ellis, 2004, p.50).
4. *Layered accounts*: Focuses on the researcher’s experience, alongside of data, abstract analysis, and relevant literature. This form of autoethnography emphasized the procedural nature of research. Although similar to grounded theory, data collection and analysis proceed simultaneously. However, layered accounts use vignettes, reflexivity, multiple voices, and introspection to entreat readers to join the experience of doing and writing research (Ellis, 1991). Layered accounts consider evocative, concrete texts as important as abstract analyses (Ronai, 1995, 1996).
5. Finally, there are *personal narratives*: These are personal stories told by and about the researcher who is themselves the subject and the object and writes suggestive narratives connected to their choice of research and personal lives, (e.g., Berry, 2007; Goodall, 2006; Poulos, 2016; Tillmann, 2009). This

method, although quite controversial to traditional social scientists, seems the most suited to me and my life.

Although I am using scholarly research and non-traditional analysis, I do understand that I am embarking on a possibility of a combination of the five auto-ethnographic methods I have written about with an emphasis on personal narratives.

Ellis and Bochner (2000) have listed almost a page of terms that have been attached to autobiographical research and argued that it seems appropriate now to include under the broad rubric of autoethnography those studies that have been referred to by other similarly situated terms, such as personal narrative, lived experience, critical autobiography, evocative narrative, reflexive ethnography, ethnographic autobiography, ethnography, personal sociology, and auto anthropology (pp. 739–740).

Adams, Jones, and Ellis (2015) declared autoethnography is a research method that uses a researcher's personal experience to describe and critique cultural beliefs, practices, and experiences. It also acknowledges and values a researcher's relationships with others and shows people in the process of figuring out life's solutions and the meaning of their struggles (Adams, Jones and Ellis 2015). "Social life is messy, uncertain, and emotional. If our desire to research social life is true, then we must embrace a research method that, to the best of its/our ability, acknowledges and accommodates mess and chaos, uncertainty and emotion" (Adams, Jones and Ellis 2015, p.55).

Autobiographical genres of writing and research present data in different layers of consciousness and awareness. Autoethnography looks outward first through a focusing on social and cultural aspects of personal experience. Autoethnographers then look

inward, sharing a vulnerable self that is affected by and may move through, detour, and resist cultural interpretations (Ellis, 1999).

Carless and Douglass (2013) declared that history is an illusion, that the history of something does not exist. Rather, it is a fallacy, a fiction. They believe, as I do, that history depends on who is telling the story.

Anderson and Coffin (2013) saw the work of healers, particularly the indigenous shamans with whom they have worked, as awakening the suffering of individuals, so they could come into a deeper relationship with themselves. Awakening suffering in patients in order to embrace and understand the depth of the trauma seems extraordinarily courageous to me. To Anderson and Glass-Coffin (2013) autoethnography is the expression of what lived experience is rather than what is recorded in notes and tapes. In a personal communication with Alan Davis (August 13, 2018) he stated that “when the notes and tapes are authored by the autoethnographers, this distinction disappears.” His point of view is most accurate to me when the nondual experience is lacking in the narrative. I believe when the autoethnographer is able to analyze the data from the memories, diaries, and notes depicting different times and places in the autoethnographer’s life, this distinction is still possible. I intend to show this possibility.

Jacobs (2016) discussed how unintentional trance-based learning from authority figures makes us vulnerable to believing what I call mass consciousness and he calls the “dominant worldview” (p.75). When we are taking direction for our morals and ethics from outside of our natural knowing and leadership, we cannot be attuned to the nondual experience and, therefore, the healing that can be a motivation for our ethical behavior as autoethnographers.

I believe autoethnographers have an ethical responsibility to reflect on their history in ways that do not degrade, humiliate, or in any way cause embarrassment. However, when we write about ourselves, we always involve writing about others (Ellis, 2009, p. 13). There are common ethical dilemmas in an autoethnographical study (e.g., autonomy, beneficence, justice); (Albert & Tullis, 2013, p. 247). Autonomy is freedom from external control and influence. The researcher is someone who is capable of acting with objective morality rather than under the influence of desires. So, in order to propel myself to observe a moral imperative that simultaneously allows me, the principal subject/participant of this study and the narrator of the story of myself, to have integrity, the need for autonomy is essential. Also, a self-reflexive attitude is necessary when examining the past through the lens of the present. To be faithful to the experience and memory of what was without manipulating the memory is a reflexive process and one I care deeply about.

By beneficence, I intend this study to add to the literature that discusses trauma and its consequences and how postpartum psychosis was noted in the '60s. I am also discussing how nonduality can be a universal experience of healing that can underlie many therapeutic methodologies of benefit to clients and the therapist themselves.

I love the idea of justice. The first guideline that propels my work and, at times, confuses me, is to do no harm to myself or others. I am writing about how a young, intelligent, brazen woman of the '50s and '60's was physically and emotionally abused by her father and mother, was self-admitted into a mental hospital for a 3-week period, and was diagnosed with the label of a "schizoid reaction to adult life." I have not been able to find this diagnosis today. I called the Philadelphia Psychiatric Center and

discovered they only keep records for 8 years. I wanted to see what they had written in their notes about me. When I applied for jobs, apartments, or filled out histories when seeing a medical doctor, I lied about whether I had ever been in a psychiatric facility or had been under a psychiatrist's care. Even today when I share that information, I feel ashamed and embarrassed. Writing about this so publicly now is helping me to change how I view that time in my life.

The people in this autoethnography include my family and others who will not be named directly, but are taken from my memory, journals, letters, pictures, and other memorabilia. The autoethnographer also has a responsibility to decide if protecting oneself by how much one discloses during ones autoethnography will support their future work and past life. As well as exposing others, this study can expose me to the judgment of my peers, friends, professors, and so forth as an emissary for nondual therapies and trauma-reducing programs. Will there be harm to me and my status in the therapeutic and academic community? I am soothed by the writings and works of others who came before me who shared their stories. They used their recovery, mental illness, and social and cultural injustices as a narrative to support the healing of others.

Here are just a few of the dissertations I have found in the Fielding Library where the author shares deeply and personally: Burke, E. (2014). *Challenging the silences: A phenomenographic study of how autoethnography is experienced* (Order No. 3635007); Fabris, E. (2012). *Experiences labelled psychotic: A settler's autoethnography beyond psychotic narrative*; Saunders, P. (2018). *Waking up while black: How a Jamaican border-dwelling bredda makes meaning of his camino de santiago pilgrimage*.

This seems like a parallel to the “Me Too” movement, in that the vulnerability of the researcher is allowing the heretofore stigma of what we called mental illness, being bullied, being the other with its labels and code words to gradually leave our shame-filled narratives and be replaced with a discussion more akin to sharing about a broken leg or a knee replacement. Perhaps some of us were experiencing a spiritual convergence rather than a mental illness and the support we received did not allow for the possibility of liberation and freedom from the mores and conditioning of the time.

Two events in our social culture were crucial to my being here at Fielding. In 1954, in a unanimous decision in the *Brown v. Board of Education* case, the Supreme Court ruled segregation in public schools unconstitutional. We never studied this in my high school. My high school was equally divided between people of color and Caucasians. When I began high school, Wilt Chamberlin was our basketball hero. We all looked up to him, no pun intended. As I look back at that time, I recognize there was a separation in our social life, but not in our world at school. Racism as a structured part of America did not really connect with me until later in life.

It was not until 1973 that in *Roe V. Wade*, the Supreme Court decision declared abortions legal based on the right to privacy. These are just two shifts in public policy that occurred before I was 40 that played a pivotal role in my life, at times without my awareness. Until I had an abortion in 1959, I did not know it was illegal. I still have many moments where I question my decision. I am perennially torn between the either/or of whether abortion is murder. I vacillate internally between the horror I still feel when I think of me at 19 years old having to make that decision, letting my parents and family “help” me to abort my baby. *I miss that dream of a child with my lover. Many times when*

the subject of abortion arises, I imagine a bright white light deep in me. I feel less guilt and more understanding of who I was when I made that decision.

Bochner (2014) questioned the use of theories vs. stories. He makes it very clear that he is not juxtaposing theories against stories. Rather, he says “there is nothing as theoretical as a good story” (p. 292). He believes the split between “story and theory is false” (p. 292). His concern, as mine, is that theory becomes an end unto itself when it is “detached, objective, value-free and beyond human consciousness” (p.292). Forgetting and excluding is also a part of the story-making as we are making meaning while remembering and deciding which part of the memory serves the purpose of our narrative. McGlashan (1986) suggests that forgetting the past and the injuries we endured is a form of healing and release, which can keep us sane (p.6).

Since my youth, I have believed that our conditioning creates a veil of sorts that transmits not only our words, but also our history. I find that words are the clothing for the feelings and history of the speaker. My job as the therapist or coach is to listen for it all. The concern I have is that our unconscious biases and personal agenda continue to influence our day-to-day interactions with family, friends, and clients without our awareness. Hence my 10 years of study with teachers and literature in nonduality. Unconditioned awareness cannot impose the therapist’s conditioning on the client as it is without phenomena.

“Storytelling is both a method of knowing and a way of sharing our lives with one another” (Richardson, 1990, p.65). My personal story-telling changes as I evolve. The way I looked at a situation 20 years ago no longer satisfies my current ethics and my truth. How does theory meet story? When we think about a story, an incident that

occurred in the past, do we tell that story with the same attitudes and expression we used when we first shared it? Or, does the continued learning we experience change the narrative so that there is no victim or persecutor, or now there is a new revelation about who is the victim and who is the persecutor?

I wish to keep my history alive knowing it is not only my history. It is all of ours. Whether you are a man or a woman, a child of the '60s or '80s, it makes no difference. These vignettes can reach you as they are the stories of humanity, of being a human. The vignettes, letters, and photographs in Chapter 4 will support and anticipate the future and not just for women, but for all of us.

Summary

Upon my return from the Philadelphia Psychiatric Center, I was hired by Horizon House, a halfway house for mental patients recently returned to society. I was called a research assistant, and over the few years of being there I was offered the opportunity to be an integral part of a study called, "A Community-Based Operant Learning Environment III: Some Outcome Data" (1971). Together with John D. Henderson, I (my name at the time was Judith S. Samuels) conducted the research, wrote the article, and presented it at the Proceedings of the Third Conference of the Association for Advancement of Behavior Therapy as part of a prestigious panel that John D. Henderson was facilitating. The last paragraph of the article states the following:

"Follow-up workers in research studies have traditionally been collectors of data, and not part of the rehabilitation process. Despite efforts to maintain this neutrality, the worker is often perceived by subjects as a caseworker, vocational counselor etc. This is an untenable position, since the rendering of counseling

services would bias results and impair the implementation of the experimental design. Without services, however, the percentage of subjects willing to cooperate may drop considerably. The discharged patient, therefore, tends to view all staff members of mental health facilities as therapeutic agents. One solution to this problem might be a dual role for a follow-up worker...In any case, the existing follow-up procedures are unsatisfactory and must be improved”

(Samuels & Henderson, 1971, p.271).

I knew then we cannot be objective. I didn't know what to do about it. I wanted to have influence with a client and yet I didn't want to manipulate the results. I didn't imagine discussing my thoughts and feelings to the person I was working with at that time.

Autoethnographers realize, along with that young 27-year-old's “aha” moment, that a researcher cannot be totally objective and fulfill his or her research goals. I personally believe it is not possible to let go of our human condition, no matter what the training and supervision. Because of my history as a psychotherapist and a nonduality student and teacher, when reflecting on my work with a client, it became clear to me how my biases affected my work. No matter how much training and studies I have undertaken, my personal thoughts and feelings nonverbally take part in my work. I also believe it is ethically responsible to share and be accountable as to what my biases are as they arise, so they do not unduly influence our results. Of course, we do not know in advance what will show up in a session. Once I felt the strength of my desire to effect the outcome in a way that I believed would be better for the client, I knew I was about to project an unexamined aspect of my history on the client. Each time that occurred, it was easier to discover and easier to stop.

Nonduality is a mature state of consciousness in which it is observed that there is only one and duality is transcended (Fenner, 2015). This became my experience while a student and a facilitator during my therapy sessions, workshops, and programs. It was very important to me to be able to write this dissertation to describe the extraordinary healing effect nonduality has on everyday life. The memories that I share have the thread of alternative knowing which is thinking differently as to what constitutes knowing (Swain, 1972). Should one way of knowing be favored over another?

“What is past is prologue,” spoken by Antonio in *The Tempest*, attempts to convince the character of Prince Sebastian to murder his sleeping father so that he, Sebastian, could become king (Shakespeare, 1605/2004, 2.1. 245-254). With this quote, Shakespeare (2004) seems to suggest that what has happened until that moment gives them permission to commit murder. It means that what has happened before certainly conditions the present and in many cases is the preface to the future. Nonduality by its very nature can change the future by seeing the past through clear eyes (Fenner, 2007) .

With this perspective in our culture, many psychoanalysts, defense lawyers, mental health professionals of all types including psychotherapists would be out of business. I have been sharing this in terms of early childhood trauma and the effect it has on our lives today. Does our early childhood trauma continue to cause us to behave in ways we say we wish to change, or can we be the authors and creators of our own stories from the moment we understand that self-awareness is possible and have an experience of it? Do we need to desire liberation from our history more than we desire the comfort of the familiar, even though it brings us pain and discontent? I believe so. As I have said a few times in this study, for the benefit of all is how I save my own life.

Silence

All of us are born into a conditioned world from an unconditioned unborn emptiness. You may have a religious explanation, a new age answer or just not know or even be disinterested. No matter what!! Just remember back when you knew that you were not separate,

Just sit – just sit – nothing to do. no one to call, just to sit – not write, not read the paper - not use the computer – not earn money. There is a big prohibition in me to have that experience. I want to scream. I feel like I will not survive. Did I feel like that as a child? Looking at conditioning without any attempt to change it, transforms it.

Judith Orloff Adyashanti workshop, 2008

CHAPTER FOUR: MY PERSONAL JOURNEY

Yearning

*Groups of faces string out into darkness and space; eyes jumping like
jumping jacks to catch a trace of a smile. Encourage me or I shall become
a nonentity or death*

A little smile is all I need gasping to catch my breath

(Free Form Writing, Orloff, 1959, New York City)

- The first section of this narrative articulates my journey as a child.
- The second section examines my teenage years leading up to leaving home at 19 years old and moving to Manhattan.
- The third section depicts my time in New York and Vermont.
- The fourth section describes my time in Colorado, my work, and the training I received. I will connect those trainings and how they wove their nondual

threads into my transformation from recovering mental patient to master teacher.

- The last section describes the 12 years from 2006 to Fielding Graduate University.

My Parents' Marriage

When asked about their relationship before they married, my mother said that my father pestered her constantly so she would marry him. My grandmother advised against it because she did not like him. The main reason for her dislike was because he had quit college and was not a professional man. My mother spoke often of Jack, the pharmacist, and how she would have been happy with him. I cannot remember hearing my mother speak of her and my dad's relationship with words of love or romance during her lifetime. On the other hand, each evening when I was a child and we were all in the living room watching "Bonanza" or "The Life of Riley" or any mystery, my father would wax about the glories of my mother. He shared how beautiful she was and how wonderful she was. She would "tsk" him off with the bat of her hand and roll her eyes.

No matter what I felt about him, during those times, my heart would break with continued embarrassment and pain for him. What happened between them that could cause such hurt and frustration? And yet, through it all, they stayed together.

I remember, when I was very young, I read movie magazines and true love stories and decided I would not live the life of my mother. That may account for my serial monogamy. I have been married four times and have had two other long-term relationships for a total of six "marriages." The relationship I am currently in has lasted 31 years so far.

When I was a 45-year-old, I took my mother for a ride along the ocean in California and she asked me, “Do you believe the books where they write about people making love?” Tears fill my eyes even now when I remember my mother asking that question. I felt a tenderness toward her that was unusual. I saw my mother without a veil in my eyes of all the years we were disconnected, and for a brief moment I wanted to protect her from the life she had and the choices she made. I do not remember what I answered.



Figure 3. Mom and Dad on their wedding day

A Loving Tale about My Father and Me at 2 Years Old and Later

My mother told me that when I was 2 years old, my father would come home early from work many afternoons just to be with me. In fact, he would take me for rides in my baby carriage through the park. I wish I could find a picture, or even a fragment of a memory of that time. I have yearned to be a “daddy’s little girl,” which I remember experiencing once at 26 years old, after my 3 weeks in the Philadelphia Psychiatric Center. My dad would come to my apartment, sit on the floor with me and my children, and look at me with tenderness and care. I was not used to that tenderness and would become so uncomfortable that I would excuse myself and go in my bedroom and cry. I still feel uncomfortable when loved fully, and I still cry.

There are moments now in my everyday life when I feel safe. There are moments when I can feel myself and world as one. There are even days when I can relax and see that all is well in the world even with all the present turmoil in our country. I imagine my father before he became ill, and I imagine these times when I was 2 and 3 years old and the joyful times my mother told me that he came home to be with me as a young child. In my fantasy he is happy and wants to do things with my sister and me. He wants to share his knowledge and wisdom with us and teach me things he knows. We fix things together. We play stoop ball. We go to the park and take a walk. These simple actions are those that draw me to men who really enjoy their time with their daughters. These fantasy moments come and go. As I see my father as a man who couldn’t bear his disappointment and his pain, I feel tender. I can let his love touch me, even though he couldn’t share it with me when he was alive.



Figure 4. Judith as a newborn

St. Agnes Hospital

My mother reminded me many times that if I weren't the only child born in St. Agnes Hospital the night she gave birth she wouldn't believe I was her child. Such a strange message for a child. I believed I must be adopted, and I searched and searched for adoption papers. She also reminded me over and over that St. Agnes is a Catholic hospital and the nun on duty taking care of her that night asked her over and over again who should be saved if there would be a choice. My mother told me she would save herself. I am glad she felt that way. Those messages arise in me when choices are difficult. She let me know it was okay to save myself. That sharing of her truth is a thread that runs through this narrative. When the person I am with shares his or her truth, I just relax and feel safe. I so yearn for my whole body to know I am part of a tribe.

My mother told me I was really ugly until I was a year old. Each time I think about that my chest sinks in and I feel nauseous. I was a forceps birth and my head wasn't shaped right at first. I didn't think that affected me. I wonder how many times I

have done the same thing with my children. I search other people's greetings to me as a gauge of how I look. Sometimes, I feel consumed with how I look, and pretend that it is not important. I tell myself that a woman my age has other attributes. I do have those positive attributes that are acquired with age and experience, although, my wisdom tells me it really doesn't matter. I still want my mom to say, "You were such a beautiful baby."

Creedy, Shochet, and Horsfall (2000) concluded that forceps delivery can be as traumatic as an emergency Cesarean delivery. Many women in their study who met the criteria for post-traumatic stress had epidural anesthesia prior to the forceps birth process. I don't know if my mother had epidural anesthesia. I do know that she had a deeply embodied response of fear throughout her lifetime, which could have been due to the fact that when she was 8 months pregnant with me, my parents were in a car accident. As the story goes, the car turned upside down and my mother climbed out of the car unharmed. I believe this is important to share in that when I was 8 months pregnant with my second child, I had an emergency appendectomy and needed medical support in the birth process, so my stitches would not open. This led indirectly to my time in Philadelphia Psychiatric Hospital. Was this a coincidence or an area of ancestral consciousness that hasn't been explored thoroughly? There are many studies on prenatal distress during pregnancy that show there is an increased risk of psychopathology observed in children from those pregnancies. Monk et al. (2016), Murphy-Eberenz et al. (2006) suggested there is a familial link, yet there is not enough evidence to state affirmatively.

Lipton (1998) "reveals that 'environmental signals' are primarily responsible for selecting the genes expressed by an organism" (p.3). This perspective goes against all I

believed until a few years ago. Special attention is paid to the maternal environment, her physiology. He declared the mother's behaviors and emotions profoundly impact the physical development, the level of intelligence of the child, and the behavioral characteristics.

In a personal communication with Alan Davis in July 2019, he stated, "You are creating an argument that the origins of your trauma were not in overt matters of your parents' behavior towards you, but in circumstances of birth that were not apparent or recognized at the time." Could it be that both are true?

Perhaps the car accident and the circumstances of my birth were a landmine just waiting for my parents to step on it. Perhaps ancestral consciousness also played a large part of my conditioning. *My sister was also a forceps birth. There was no car accident before she was born. She was not physically abused as a young teenager. She has been married to her first and only husband for over 57 years with a 3-year hiatus when they were much younger. My sister and I love each other and yet we cannot see the world the same way. It is sometimes difficult for me when we are together to be willing to listen to and experience her perspectives. I have the same feelings I did as a child when my parents presented the world in absolutes and either/or dichotomies. This is not a criticism of my sister or her perspectives. It is a noticing for me of my need to have compassion for that child that continues to live inside of me.*

This nature-nurture controversy continues to pervade our educational system, make our decisions that maintain our entrenched racism and see us as victims of genetic fate (Lipton, 1998, p. 5). Devlin, Daniels, and Roeder (1997) revealed that intelligence was only about 34% based on genes and 66% based on environmental factors. Devlin

(1997) claimed that the prenatal environment was one of the primary factors influencing intelligence.

I believe what Lipton (1998) declared about epigenetics insofar as my prenatal life and the environment I experienced after my mother's car accident may be possible reasons I was so different from the rest of my family. The environmental signals that were expressed during the first two trimesters of my mother's pregnancy were a very happy time from all that she shared with me. Yet, the last trimester, with her accident and the change in my father from being somewhat of an athlete to a serious non-athlete, created a mixed environment. I can go from one emotional extreme to another in a few moments. Because of my work in nonduality, I am able to see what fundamentally is true and what is a trance or reactive state that feels true to others (Jacobs, 2016). Sometimes it is difficult not to become attached to the experience of the moment. Although an illusion, as I discovered, it is very strong and feels inflexible.



Figure 5. Judith and Carole Ann.

Three Years Old

When my sister Carole was born, I stayed with Aunt Ethel, my mother's sister, and Uncle Al, her husband. I was a happy child and felt safe in my Uncle Al's arms. I remember pure happiness with Uncle Al just loving me the way he did. I sat on his lap and watched TV. What a wonderful time. I don't remember when he died. It was only years later that I realized he was my "perfect father". When I remembered that time

I knew I was okay. I didn't remember feeling any anxiety. Looking back, I felt nostalgic for a new beginning. I heard myself say out loud, "Please God, give me another chance."

When returning home after my sister was born, I didn't notice my father, but my mother was there taking care of our home and me and my sister. Like many women of her time, my mother did the housework, the cooking and cleaning up, and took care of my sister and me. My father was gone all day, and from the minute he arrived home he was the center of attention. We wanted to be sure he was happy. We didn't want my father upset. That was a mantra in our home.

The Red Mitten

Once, when I was 3 years old, my mother found me sitting on the stoop crying and quite upset. I had lost a mitten and was so unhappy. I was afraid that the lost mitten would be lonely. She wanted to buy me a new pair of mittens, but I just didn't feel connected to anything but that mitten. I felt panicked. "What would happen to the lost mitten?"

Lane, Ronfard, Francioli, and Harris (2016) declared this is a stage of mentalization, a psychic equivalence where what I believe must be real. Lane, et al. (2016) indicated that children ranging from 4 to 8 years old were able to report whether they could imagine various improbable phenomena, and the results of this study showed that many children knew what was improbable and what was not. I still to this day wonder if the connection I experienced with these red mittens really existed or whether I imagined it.

We all have connections with inanimate objects such as mittens, wedding rings,

pieces of clothing, books, and so forth. As an older child, I wondered if the lost object, even if it was just a mitten, knew it was lost. Does a mitten have consciousness, and can there be an energetic connection to a lost object? What occurs for others when they lose something that is important to them that was inanimate? Lehman, Brauch, Arnold, and Reeves (1995) estimated 60% of young, middle-class children in Western cultures use soft objects such as blankets, pillows, and other stuffed toys to comfort themselves. I am reminded of Linus from “Charlie Brown,” the cartoon, with his blanket and his thumb stuffed in his mouth. Winnicott (1953) asserted that attachment to these objects does not suggest or imply a maladjustment. Most of the data in studies of this type are supplied by parents, not the children themselves.

Even as I write this over 70 years later, I feel sadness. The sadness I felt then, and even now, comes from a strong knowing that when something is out of order in the universe it can be felt even about a lost mitten. I am still affected by acts of kindness and meanness equally. I cry. It is very personal. As soon as I realize how personally I see what is happening, I stop crying. The abuse wound opened up, and I either feel grateful or powerless. I am usually surprised just as when I was a child and young teenager. Since beginning this study, I am less defensive. I tell myself different stories depending on the circumstances so that I can feel “normal”. I still have a deep desire to fit in, to be able to make small talk, to laugh at jokes even though I don’t see the humor. If I pretend, I feel like a fake. This is a paradox that comes and goes depending on how present I am at the time.

After I lost that red mitten, I didn’t play with stuffed toys or special blankets.

Strange as it may seem, this experience aged me in that I no longer believed the world was fair, or that all was good in the world. Although not quite so innocent, I also lay on the grass at night sometimes and watched the stars move. I was just like many children seeking solace from the vastness. I did not know how to express those thoughts. I do not know how I lost this mitten yet understand why this memory has helped shape my life. The mitten is me, lost and lonely. I lost the mitten around the time my mother gave birth to my sister. Perhaps, I felt the loss of the undivided attention of my parents. I don't really know, and each time I read this paragraph I still feel a deep sadness. Perhaps, one day I won't.

Seven to 12 Years Old

When I was 7 years old, we lived in an 800-square-foot row house in Overbrook Park, in Philadelphia. It was one of the best times of my life. The only crimes committed in this primarily Jewish working-class neighborhood were a few illicit affairs. My mother locked the doors and windows each night, and when I would ask her what she was afraid of she denied being afraid and just said it was the right thing to do. I, however, felt afraid when she would lock up our home. I don't know whether the locking of the doors and windows is the issue or the withholding of my mom's feelings. It was just 2 years after my father's heart attack. My world was changing. *I still feel fear beginning to grab my heart when I am alone in my home and evening comes. I often check the locks more than once and with every sound, my thoughts of doom demand my attention. Someone could die! Someone could get hurt!*

It really wasn't important that my mother locked up the house every night. Not

being let into my mother's inner world mattered. I distrusted people who wouldn't share their feelings with me. The nondual experience of unconditioned awareness can be attained in an easy manner when one just says what is true for oneself without justifying, explaining, or attempting to control another's experience. The context of a situation means more to me than the content. By context, I mean the physical, emotional, and spiritual environment that surrounds an experience and gives it meaning.

Why wouldn't my mother say she was afraid? Why not let me know her feelings and thoughts? Without an emotional connection, my home felt irrational. Her behavior sent waves of fright directly to my body. She locked the windows and the doors and checked over and over to be sure they were locked. This behavior combined with the rules I had to follow made fear my constant companion. The only time this fear wasn't waiting in the dark closet of my mind ready to pounce on me was when at 19 years old, I impulsively left home and moved to Manhattan.

My fear of death and being assaulted returned with the birth of my first child and sits in the back of my mind ready when I am unaware of my feelings. I am sure if my mother knew how her behavior would affect me, she would have changed her behavior. If I had known the effect of my behavior on my children, I would do all I could to change that behavior immediately. Without an understanding of how early childhood conditioning can support lifelong trauma, this cycle continues.

My parents warned my sister and me not to go outside alone at night. Sounds wise on the face of it. However, my father spoke of giant rabbits that would jump out of the bushes and attack us. He told me not to stand on my head or I could break my neck.

He told me to not “fly” down the street on my sled. It was dangerous. At the time, I didn’t think or feel much about what my parents told me. In fact, I had outwardly become numb to his words. As I shared in the last paragraph, my fears showed up after I had children of my own. I believe my fears and anxiety are directly related to the fears my parents transmitted to me, fears that they didn’t understand or accept as their own. *What is it that accounts for any of us to be oblivious to our own fears? After I realize what I am feeling belongs to me it is so obvious. I know it is my “shadow.” I know it is projection. I want to be able to see how it works not just understand. I want to watch it happen to me. I want to parse the process, so I really see it in slow motion.*

When I go East to visit my sister, I notice she and her husband, who have been married since they were 18 and 19 years old, are cautious and careful about their life. They are creative and intelligent, and they see the world and people very differently than I do. They believe in right and wrong, good and bad, agreement and disagreement, and I believe they express their opinions as truth much of the time.

I feel at home when I visit them. I go to the beach, take walks, and watch TV in the evenings. I don’t want to go anywhere, or do anything. I love just being there, feeling safe and not required to defend or explain myself. However, I do not argue about her point of view often or impose my point of view into the field, except when I am unaware of myself. And, I am not afraid at night when I am with them. I feel so taken care of like a protected child. They are how I wished my parents would be. It is a paradox, the absoluteness of their views and my deep feelings of safety. That young child alive in me has what she needs: love, and people telling their truth.

Maybe all young children can sense what their parents are feeling. My mom often felt hurt by me. Many feelings were not acceptable in my family. I often heard something along the lines of, “You have no reason to feel sad. I’ll give you something to feel sad about.” When I shared sadness with them, they became accusatory and blameful, saying my sadness was because of my differentness. My mom and dad were deeply hurt themselves and didn’t know what to do with their hurt. Conflict, and very strong opinions, on the other hand, were totally acceptable, and so was raising voices and arguing about facts. I didn’t think much about that as it occurred in most of the families of my friends, and at my grandmother’s home on Larchwood Avenue, in Southwest Philadelphia. My father’s side of the family included his five brothers and two sisters, and every Sunday afternoon we went to visit our grandmother and grandfather, his parents.

Each week, 30 or so cousins, uncles, and aunts all gathered at “Bubby’s” house (my grandmother’s home). My memory of those times brings smiles and joy. We laughed, argued about everything, ate homemade Jewish foods, and returned home happy. Sadness was not allowed there also. We would call G-d on the phone, have skits, sing. We had a wonderful time. We could argue, raise our voices, and challenge uncles and aunts, but never to my father. As I look back on those times, my father was funny and open at times, and yet when my sister or I challenged him he would explode in anger. Looking back again, I can see how hurt he felt when we didn’t take “his word” for whatever we were talking about. Even so, the joy in those gatherings somehow let me know that all could be all right in the world, even my world.

My Father’s Heart Attack

My father had his first heart attack when I was 5 or 6 years old. He was 39 years old. One minute my world was in order, and the next was the end of the world I knew and the beginning of the anxiety and pain that led me to a disconnection with myself for many years. His physical health and the terror of him dying at any moment if he was upset were paramount in our family after his heart attack. I lived in a constant state of what I now call cognitive dissonance (Festinger, 1962) from the time I was 5 years old, culminating with my stay in the Philadelphia Psychiatric Center. Up until the time that I was 5 years old, I could describe my father as loving and kind. My dad was given medication that transformed him into an abusive personality over time. Some of the side effects of the medication, which included Seconal, Prednisone, and a version of Percocet are noted to produce psychotic behavior. I will only list those that I can identify with my father. For example, Prednisone can produce headache, dizziness, trouble sleeping, severe mood swings, personality change. The side effects of Seconal are listed as those to “call your doctor right away”: unusual thoughts or behavior; worsening depression, confusion, hallucinations; anxiety, agitation. In addition to the above listed medications, he also took large doses of pain meds. I do not know which ones, but with Percocet, for example, some of the same side effects are listed as with Seconal and Prednisone.



Figure 6. A moment of happiness –with sadness in my eyes.

Empath

I didn't know I was an empath and could experience a dissolving of the self/other dichotomy. As an adjective, this is demonstrating an ability to understand and share the feelings of another. Because I didn't know I was an empath, with the capacity to experience what was occurring in my family, *I began to believe there was something wrong with me. My mother seemed to be far away, living in her internal world and only coming out when I called for her. I felt so separate and lonely.*

When my mother was 12 years old, her father, educated as an accountant, came home from work with a boil on his neck. His brother, a gynecologist who delivered both my mother and me, lanced this boil without antibiotics (as there weren't any), and my grandfather died of blood poisoning the next day. *When I contemplate the lack of joy in my mother's life I feel so deeply sad. She just wouldn't talk to me.*

My mother refused to discuss her father's death, her mother's death, or any other question I asked about her past, no matter how many times I asked. I was so desperate to know more about her. She would say, "Stop probing. You are upsetting me. I don't want to talk about it." Even today, when someone asks me to stop talking because they are "upset," I feel manipulated. I do stop pushing most of the time now. In the not so distant past I would obsess about it and just keep going.

I felt so scared when my mother proclaimed her upset. I felt guilty and angry at the same time. It seemed no matter what I said or how I said it, neither one of my parents would talk to me personally. This is not about politeness or privacy. We are not separate. I can feel what others are feeling. One time, when in training with Peter Fenner, he asked to meet with me privately. I was sure he was going to tell me how great I was doing. I thought about it all that night. I changed clothes many times that morning before our meeting. I planned to be 5 or 10 minutes late. I was exactly on time. He was sitting on a bench outside the home we were meeting in and looked very inviting to me. I sat down. He took my hand, looked me in the eyes and said, "Judith, I want you to know you don't need anyone to change." What? That wasn't what I expected or wanted to hear. Yes, I do!!! I always have!!! I want people to be open, loving, generous, and kind. I can make it happen. All I have to do is be kind. I went into class in a daze and,

have spent the last few years dealing with my need to have my parents change. As their guide and counselor, many people shared with me they have had similar experiences with their parents. And, maybe it did not seem unusual to them, yet they came to work with me because they couldn't connect deeply with themselves and their families. I couldn't stand the lack of intimacy or true discussion with my family. I felt an emotion and was told that what I was feeling was not what I was feeling. My parents wanted me to be just like them. When I was not, my parents labeled me a phony and continued to ask me who influenced me with "these weird ideas" of mine. I don't believe my ideas were weird. I believe they just took a different turn than those of my family. There was one area in which my mother was very open, and this was about her thwarted ambition to be a history teacher. She was forced to quit high school and go to secretarial school. She longed to be a history teacher, and she read historical novels most of the time. She spoke about this with uncontrolled tears throughout her life. Those were the moments I felt so tender and close. I don't remember having this with my dad except when I returned from Philadelphia Psychiatric Hospital. I do remember a few times when my father's behavior belied his words. We had Sunday morning breakfasts with lots of food and heated conversations. He invited unusual people to these breakfasts-- people of color, people of different religions and ages. He helped my friend obtain an abortion without noticeable judgment; he was very funny at times. I didn't know when the good times would occur or what would make them happen.

Donovan (1997) asserted, "We are all able to perceive others' emotions" (p.460). He also discussed in detail the differences between telepathy, charisma, and empathy. I have never seen myself as possessing telepathic abilities, although the thought crosses my

mind when I think of someone I haven't heard from in a while and he or she calls or texts me. Donovan (1997) did not assert that telepathy is real. He claimed it is as real as other social psychological phenomena which are accepted by the general public, like women's intuition. In terms of a partial definition of charisma as the ability to inspire others, I have been able to inspire my students and clients to achieve their personal goals and visions. Since I was a young child, I have had the ability to perceive the emotions of another and have described myself as empathetic. I wonder when and how I began to understand how patterns of behavior could be predicted and how they affected the future course of relationships.

Donovan (1997) described empathy as a process where one suspends one's own assumptions of reality in order to incorporate those of the other. I know how to do this, and I bring this work into my study, as I believe being able to suspend our assumptions of reality is one of the crucial steps needed to experience nonduality. Strayer (1990) asserted the "core of the empathetic experience is a 'free association' or loosening of self-other boundaries in allowing stimuli impinging on the other to be experienced by the self" (p.227). However, without permission or accepted understanding of trust, this can be a terrifying experience for a child. It seemed so obvious to me, yet it was silent.

Many times, I wondered if sadness and/or fear were bad -- and by *bad* I mean unacceptable in my family. I didn't know much about the feelings I had when I was a child. I didn't know they came and went. I didn't know they were like the wind.

I believed these emotions were who I was. They defined me. Anger, worry, and disappointment seemed to be okay. I did what most children did. I lived in conflict and denial of my experience. I knew what was true. Yet, without permission to discuss it, I

was berated. To this day I am not sure why I was so insistent. I do know that I was very confused about what to believe. And, I didn't feel like a member of my family. I was out there alone! How many of us can identify with this? Probably anyone reading this.

At the same time, I learned how to survive in my family. I learned what was rewarded and what was frowned upon. I learned what could cause anger and fear. I also learned that by riding my bike and reading I could ease my anxiety.

Like my mom, reading was and is freedom for me. Books took me to places I had never heard of and knowledge that was not part of my family culture. I have over a thousand books that I have not read, but could not bear to let them go. As I look back over those few years of memory, I didn't know what to believe. I wondered often how I could "be a fake" and still be part of my family. At some point, during this time, I "forgot" what I saw and knew and chose to be part of my family (Eliot, 2000; Orloff & Doubleday, 1991). I had already developed an internal working model. Rather than be confused, I began to see the world differently than my family. This separation from my family's values and points of view was both isolating and peaceful. Soon after this, I searched the house looking for adoption papers, hoping that I didn't really belong to this family. When I didn't find what I was looking for, I was really disappointed. For a long time, I began to believe my perspective was not mine. I began to feel disconnected from my body.

Cinderella – From a Journal, May 2012

I was 4 years old. My mom read out loud to me many evenings before I went to sleep. After hearing my mother's clear voice reading (she loved books), I relaxed and slept a fairy tale-filled sleep. Although that would change in a year, right then, in my

full-sized maple bed, I slept without fear. That night, when my mom got to the part in Cinderella when her ugly stepsisters were plotting against her with her stepmother, I began to ask questions: Why doesn't Cinderella's daddy stop this? Why are the stepsisters so mean? Why are they so ugly? Why does Cinderella sweep all the ashes and not say no? I remember my mom laughing at me and holding me close.

"Don't you want to be a princess like Cinderella? "It ends happily," she said. "Let's finish the story You'll see." Do her stepsisters go to time out? They are so mean. No time out, I discovered. They are deeply humiliated instead. The prince as we all know looks for the perfect foot. He goes from house to house in this kingdom of his until he finds Cinderella's foot as it slips into the glass slipper. Her perfect foot! Small, shapely, without nails that need clipping or sore from running home when her carriage turned into a pumpkin. And what happened to the pumpkin, I'd like to know. Did Cinderella really want to marry a prince who didn't recognize her right away? Did she want to spend her life with a man who was so foolish? And, what of her misinformed stepsisters? They must have been abused by their parents to act so mean and heartless. And what was with Cinderella that she couldn't tell her dad what was going on at home? Can anyone be so naïve and repressed as Cinderella?

Is it possible there is another story happening that we don't know about? I do remember questioning the black-and-white of Cinderella. I also remember the love and beauty of my mother's face as she looked at me. I remember that time in my bedroom with the door being left open, our small and friendly house without ghosts or evil lurking in closets. I remember that time when I had a baby sister who I played with as my doll baby.

I do remember this time in the beginning of my life when I had opinions about Cinderella and her sisters. No one got angry or lashed out. I do remember Cinderella's stepsisters with their strange faces contorted with anger and hurt. Later, only a year later, I discovered what may have been the cause of their meanness and jealousy.

But, that night, with my mom lying next to me, reading to me, I had no fear. That was to come later. I didn't want to become a Cinderella. I didn't want any of that story. In that story, we cut off our toes, our heels, our noses, and loose saggy skin. We pretend we don't care if the Prince sees us as desirable, if he appreciates our particular beauty.

Clarke (2000) spoke to the Grimm Brothers' version of Cinderella, first published in France in 1697. He described how the "evil" stepsisters attend Cinderella's wedding to the prince, and as they leave the ceremony birds pluck their eyes. In Sexton's (2001) version, Cinderella becomes a housewife whose husband didn't even recognize her. I have felt like this in relationships. At times, I couldn't tell if my husband, or the man I was with at the time, didn't notice whether I was there or not.

Trolley Car

My mom and I took buses and trolley cars in Philadelphia to go everywhere. Whether it be my mom taking me to an acting class at the Settlement House in South Philadelphia or a movie or shopping trip, we took a trolley or a bus. *I loved the trolley. I enjoyed seeing all the shades of people, how they dress, speak, all the languages.* We went for orthopedic shoes, as I was pigeon toed. My feet were fluoroscoped every 6 months. *My mother and I were on our way home from the shoe store. I didn't know my father's time in bed was because he had a heart attack. I didn't know he might die. I didn't know this trip to the shoe store was the last time my mother would be care-free,*

and that fear would lay in wait and would bleed in from every crevice of our home. I didn't know I would lose my direction. I was 5 years old; we had just left the shoe store and were on our way to the trolley stop. My mother stopped short and just stood on the curb looking around as if she were lost. I remember feeling confused and afraid and kept pulling on her hand. Finally, she looked at me and her trance state disappeared, and we went on our way. I watched her very carefully from that time on and felt responsible for her emotional state when I thought about it.

Today, I believe that when she suddenly stopped that day, she went back in time to when her father died suddenly of a lanced boil on his neck. I don't think she ever returned from that memory. After that moment on the curb, her life with my father changed and my life with my parents as I knew them was gone. From that day, we were a different family--always watching, waiting for something to happen to my father. "From that day the trauma becomes kindled in our central nervous system and becomes easier to ignite each time the memory is evoked" (Glovinsky, personal communication, July 2019).

Elementary School

My feelings and beliefs about connection continued as I moved into elementary school. As I look back to that time the tone my mom used conveyed a warning for us to be careful. When I remember that time in our lives, I can see my dad in his bed, just lying there healing his heart. At that time, heart attacks were treated by having the patient lie in bed for a long time. Throughout my elementary school education, I lived in two parallel worlds. After my father had a heart attack, I remember not being allowed to make any noise, and I was constantly being warned not to disturb him or he might get upset. I

know my mother was very frightened. I also know she was not aware of being frightened. She was just constantly warning me to behave so “daddy” didn’t get upset or die. I really believed that I could make my father die by being naughty. I know adults today who believe their thought about something can make it happen. Nothing I say to them makes a difference in their belief.

The theme of being a powerful agent in a life or death situation followed me the rest of my life and still does to this day. When my father died in 1983, his heart was healthy. He died from diabetes complications where his foot had to be amputated. He spent the last year of his life not leaving his green chair that was in front of the TV, not being able to care for himself in any way. He was not able to cook or make coffee. I don’t remember a time when he did dishes or cleaned up after dinner. I didn’t experience a partnership in their relationship, and I struggled in my own right to find a partnership that could withstand the test of time. It took four marriages and many years; however, I finally was able to find it with my fourth partner who I have now been with for 31 years.

A Chance for Private School and Abuse

My time at Beeber and Lamberton Elementary School was spent reading, observing, and riding my bike. At that time, my father began to act strangely. If it were today, I would know he was hooked on prescription drugs and I may have been able to give up my attempt to connect with him in a rational way. I might have asked for help from my relatives. As it was, the craziness was just the way it was. I believe my father was chronically ill from the time of his heart attack.

Families coping with chronic illness become co-regulating systems (Repetti, et al. 2009) where the issues and stresses of one family member affect the well-being of

another member of the family. In a personal communication, Ira Glovinsky asked (September 2018), “Are we not always co-regulated systems, sometimes better and sometimes worse? We are always regulating each other from moment to moment. We can be in or out of sync, attuned or mis-attuned, contingent or non-contingent or anti-contingent, but it is all under the heading of co-regulation.”

Well, that was true for me. Although these articles were not published until 2009 and 2016, it seems like they could have been published in the mid-’40s when I was in fourth or fifth grade. *Neither my father nor my mother ever admitted that my father was hooked on pain meds and sleeping pills although he acted it out each evening. Maybe that wasn’t even in their minds. I cannot even imagine what my mother was going through when my father acted strangely each evening. For example, he would crawl on the floor toward her and tell us all how beautiful she was. His voice congealed into the sound of a muffled gargle. My sister and I were grossed out and just looked at each other.*

When my fifth-grade teacher asked my parents to tea at her home near Haverford Avenue, in mainline Philadelphia, I was really excited. She recommended that I be sent to a non-traditional Quaker school, the Friends School in Philadelphia, and she would arrange to help my parents with the finances. Based on what she was observing in class and the way I thought about things, she believed I would be better understood in a private school rather than a public school. She told my parents I was gifted, both intellectually and emotionally. However, my parents said no.

Der, Batty, and Deary (2009) discussed the links between high IQ in youth and health at age 40. They declared that a high IQ in young adulthood was associated with a

loss of physical health and morbidity at age 40. My illnesses did not begin until age 55 when I discovered through a dream that I had lung cancer. I believe that if there was no physical and emotional abuse in my childhood, I would not have gotten ill with lung cancer and digestive disorders later in my life. Some examples of physical abuse were throwing me to the floor and kicking me, punching me with a closed fist all over if I confronted my father and told him I did not agree with him. Some examples of emotional abuse are my mother saying, “I hope you have a daughter just like yourself and then you will be sorry,” and my father saying, “You will destroy the world.” An example of sexual abuse was calling me a harlot. I didn’t know what that word really meant.

I don’t know if they were attempting to hurt me, or just control my behavior? I knew it wasn’t right. My mother would close the windows and the blinds so the neighbors couldn’t hear or see. I see this as abuse-related trauma. One day during a visit to my parents’ home as an adult, P.L., a neighbor who was a Holocaust survivor, called out to me. We hugged. She apologized for not helping me when I was a child crying out for help. I bless her every time she shows up in my memory. Anda, et al. (2010) asserted in their study of adverse childhood experiences (ACEs) that adverse childhood experiences may be associated with an increased risk of lung cancer, particularly premature death from lung cancer. Furthermore, Anda et al. (2010) suggested that the increase in risk may only be partly explained by smoking, suggesting other possible mechanisms by which ACEs may contribute to the occurrence of lung cancer. What a shock to find articles that connect ACEs with lung cancer. Scientists are now looking at the connection between genomics, proteomics, metabolomes, and microbiomes.

Additionally, Crisma, Saurel-Cubizolles, and Romita (2001) presented data from 510 women who showed that sexual violence from a parent to a daughter produces long-term effects such as low self-esteem, depression, suicide attempts, eating disorders, gastrointestinal and gynecological disorders, and more. In the same study, they asserted that physical violence alone may or may not have those effects. Yet, without my childhood experiences and the resultant physical and emotional illnesses which were similar to those symptoms caused by sexual abuse, I would have taken a different journey and would not be writing this dissertation. I anticipated that once I found an appropriate answer as to reasons for the brutality, I would leave my self-doubt, despair, and sadness behind me, and like all fairy tales live happily ever after. That did not happen. At the same time, I gained compassion and learned how to be assertive. I found deep joy in learning and developing my creative spirit.

Ramona

My parents did not allow me to participate in sports or gymnastics. They were terrified of me getting hurt. So, I read and read. My favorite book was *Ramona*, an 1884 American novel written by Helen Hunt Jackson set in Southern California after the Mexican American war. It is the story of a mixed-race Scots-Native American orphan girl who suffers serious racial discrimination and hardship. She was considered illegitimate and she was my age.

I still cannot get her out of my mind. At times, I thought she and I were the same person. Being told I was not to trust people that were not Jewish led me further into exploration of other religions, poetry, and fiction that showed we were really one, and led me away from traditional Jewish observance. The idea of being “the chosen people,”

but only if one is a man, did not make sense to me. I have always wanted to be someone else. Ramona is my alter ego. She is the brave, compassionate, undying aspect of me.

Being Jewish in Philadelphia

I believe being a secular Jew without spiritual guidance from my family had much to do with the Jewish experience in the '40s in Philadelphia. Growing up in a non-observant Jewish family, I knew that Judaism stressed the importance of community and home (Yuval-Davis, 1994). Yet, I did not have anyone with whom to discuss my family situation. When I told family members, they did not believe me. School officials thought I was making up my situation as an excuse. Jewish fathers were supposed to treat their daughters in ways that led their daughters to a successful marriage, to a career that would include marriage and taking time to raise children, and to being treated with deep respect and unconditional love. Remember, this was 1952 in Philadelphia.

I do have a joyful memory of lying on the grass on my front lawn and talking with God many nights. I would ask why I was being treated the way I was. Why couldn't I feel safe in my home? Why just me and not my mother or sister? No answers. No answers. Just silence and the sounds of the night. I don't question why me anymore, except when I am really feeling sorry for myself.

My mother and father were still stunned by the Holocaust and their family's exodus from Russia and Poland. They had not fully adapted to America. They projected a veil of anti-Semitism on the non-Jews that we encountered each day both in school and work. They felt obliged to remind me every day about how Jewish people were being blamed for Jesus's death. Jacobs and Dimarsky (1991) asserted there was a tragic combination of violence against women and anti-Semitism that arose out of the context

of stereotypes about Jewish women in America: the “Jewish mother” and her daughter becoming a

“Jewish princess.” This contributed to the inward self-hatred of the Jewish immigrant in the early part of the 20th century (Frondorf, 1998). In most cases the literature discussed marital violence between men and women, not fathers and daughters. Yet, in my case, both my parents and my sister participated in the abuse toward me by blaming me for my father’s behavior.

One day after a Saturday service at my synagogue, I went up to the rabbi and asked him what I needed to do to become a rabbi. The look on his face, the smile that even to this day I cannot describe, and his pat on my back broke my heart. Little did he know the astringent effect his behavior would have on my future. If I remembered his name, I would call him and let him know how powerful that moment was for me. He told me I could become a rabbi’s wife and help my husband. I remember his mouth moving and a strange tunnel-like roar going through my body. “I want to be a rabbi, not a rabbi’s wife.” I always thought of my mother at times like this. She was a wife and I did not want to be like my mother.

After that meeting with the rabbi, I did not go back to a synagogue until much later in my life when I decided to become bat mitzvah. I was 65 years old and spent two years preparing. A bat mitzvah is a religious initiation ceremony for a Jewish girl aged 12 years and one day. This ceremony then asks the girl to live the Jewish laws and customs that are for women. From that time on, around when I was 15 years or so, I was on a search for truth, for what was really true. Although I knew I was not alone, and I knew

that from daily experiences with life – friends, family, school, books, animals, even insects -- something had ruptured in my faith. It has never returned, as I am not as innocent as I was back then. For the sake of the truth, I took the oath to follow the laws of the Jewish woman, yet did not keep them.

I began to read books about Catholicism, Buddhism, and other religions. Yet there was no way I could deviate from my belief in the strength of connection. *How could this rabbi talk, and I react with such strong feelings?* I didn't really know him. He was telling me what were facts at that time.

I was 15 years old and wondered how emotions passed from one person to another. Did I have that power? Was it because he was a rabbi, a man of God? Did God hear what he was saying to me? Did God know my body felt such heat as I walked away? Why didn't lightning strike me as I thought such mean thoughts (i.e., violent thoughts). I remember vowing to change my thinking. I didn't know how to do it and stay in the world I grew up in. It took moving to New York City and meeting a very diverse group of people.

Taking Sides – The Beginning of Nonduality

In middle school, I could not see how one side, the South or the North, in the Civil War was better than the other, with the exception of the rejection of slavery. How could one side be all right and the other all wrong? I vaguely remember discussing the Civil War and understanding only how each side felt rather than the reasons for the conflicts. Were we not all one country? Were we not all human? By that time, skin colors and ethnic differences were becoming apparent to me. Yet, I felt so curious to touch, be

close, to be friends, to understand how and why we looked so different. Could one tenth of an inch of skin divide us so dramatically? Could I ever understand what made one human judge another? This is true for me today.

It was only after high school and moving to New York City that I realized that I grew up in a racially divided world. Even though I was friendly with people of color in high school, I was not socially in the same circles. I didn't question the separation. It didn't appear as an issue for me. The violent uprisings that occurred in other parts of the country didn't touch me or my friends. I lived in a neighborhood of White people until I was 19 (which was in 1958), where people rarely, if ever, questioned what racial issues were occurring in other parts of the United States.

I began to understand, after moving to New York City with its diversity and ease of movement between White and people of color, what was occurring in the rest of the United States. My building on Riverside Drive at 87th was integrated, and many nights, the residents of that building sat on the stoop and we discussed Black and White relations. At first, I was very uncomfortable having these conversations.

Yet, I remembered when I was a small child my father used to send me to the movies on Saturday afternoons with his Black employee and thought nothing of it. Yet, this was different. This was equality, and I began to see what now I would call White privilege. Not then.

Epstein (2011) discussed how Oakland, California was seriously impacted by integration and civil rights, and how Oakland fought the resistance to that. I am only familiar with public school in Philadelphia in the 1940s. There were no people of color in

my elementary schools. This was before civil rights issues became prominent in discussions. In my family, we had people of color in our lives through my father's business, and they were part of our Sunday breakfast. I had no idea of the turmoil and anguish they were experiencing and did not for many years. Being at Fielding Graduate University for the past 3 years has opened my eyes to the institutionalized racism and unconscious bias that we all possess. My racial and gender biases and prejudices are still becoming apparent to me.



Figure 7. Teen-aged punk.

Teenager

I associate the solidity of the change in my family upon my becoming an adolescent, and I became aware of the power of being a sexual being. I enjoyed the attention I received from boys and men alike. I enjoyed posing in front of a mirror and, in fact, I had a strange pleasure upsetting my father with my sexuality. At first, he would tell me to go back upstairs and remove my make-up. I would answer back and do all I could to force him to accept me and allow me to explore the world. He would not!

These arguments and my baiting of my father led to more serious abuse. I was a poster child for physical and emotional abuse. The regular physical beatings, violent outbursts, misogynist and sexually abusive name-calling by my father, and the blaming and attempts to stop my voice by my mother, have deeply affected my life in many different ways. One of many positive results is how it provided me with the motivation and resilience to continue to grow and develop. In fact, I am obsessed at times to understand how people come to the conclusions they do and how those conclusions lead to their choice of behaviors. Although many of our choices are bound by conditioning, I am always searching for tools that will support thinning that conditioning in myself and others. I imagine a world where we all are being the self that is not wounded.

One evening after a screaming physical fight between me and my father, I ran upstairs to my bedroom where my younger sister Carole joined me. I heard my father on the telephone with the police. Within a few moments they were at the door. I covered my head with the sheet and didn't want to talk with the police. They came upstairs to my room and told me my father wanted me arrested so that I would behave. I was 15 years

old. They offered to help me by taking me out of the house to a relative or a friend. I pretended that I was crazy that night. I knew what I was doing when I covered my head with a sheet. I knew the police took my side and I liked it. I realized I could call the police and make things happen. Even though I was pretending I was crazy, I later realized I was deep in a reaction and thought I was all-powerful. My reaction to the abuse was that I began to stop experiencing my feelings. I became numb and hardened the armor surrounding my heart. I was so afraid.

My father enlisted the force of the authority of the police. At the time, I couldn't leave my sister. I did eventually leave her to move to New York when I was 19 years old. I asked her to come with me. Of course, she didn't, and I don't know what I would have done if she had said yes. I still have misgivings about that. It has only been over the past 5 or 6 years that my sister and I have been able to discuss this severing of our intimacy and bring it back into our lives. I don't know if our respective history in our family can allow our intimacy to overtake our wounds. As I stated earlier, my sister married at 18 years old and lived a life dedicated to family and staying near our hometown. She took care of our mom most of the time. I didn't want anything to do with my mom for a long time. Such a different path than mine.

The verbal and physical abuse went on for many years. My innocent faith was gone. Yet, the abuse was so often and so efficient, I became immune to the physical pain. In fact, I didn't feel much at all. Many victims of childhood abuse repeat the abuse in their adult life. Some marry a person who seems kind on the outside, yet that person

is beset with his or her own demons. The ability of the abused/abuser to not feel what is occurring in the present is startling, and yet I spent many years not feeling what was going on around and inside of me and saw it as being strong.

A poignant memory stands out and yet it is not unusual. It is only unusual in that my boyfriend at the time was involved. We went to a movie with another couple and went to a restaurant afterwards for a late-night snack. I called my home and let my parents know we would be a little late. I had a very early curfew. My parents knew where I was and that we were just having a bite to eat.

I felt divided into two people: the fun, in love with my boyfriend, teen having a shrimp cocktail, and the prepared-to-confront-my-parents, genderless, powerful, superhero. This was a common experience for me, this division of selves. I would appear tough while a small child cowering inside of me depended on my acting tough.

We arrived at my doorstep, 20 minutes late, and my boyfriend Gerry walked me to the door as boys did at that time and we kissed a few times. My father opened the door and grabbed Gerry's sport coat and attempted to tear it off his back. This was the first time I had witnessed him be violent toward another person and I was very confused. My father chased Gerry down our street. I don't remember where the other couple was. They could have been in the car or we could have dropped them off before coming to my house. I don't remember how Gerry got away. When my father came back into the house, he was out of breath and told me I couldn't see Gerry again. I challenged him right there and said I would see Gerry, and he couldn't stop me. When I remember that night, I am aghast. Didn't the neighbors hear and see what was going on? My father was a big man at that time. There he was, chasing a young teenaged boy/man down the narrow street

where we all knew each other, and no one said or did anything until many years later, as I said previously.

His attack on me (and himself) that night was worse than usual, and my mother attempted to stop him from physically hurting me. *I hit him back over and over again. I ran up the stairs to go to my bedroom and we kept hitting each other. He fell to the ground, unable to catch his breath. I was sure he would die and again I was deeply divided. I wanted him to die and also wanted him to recognize who I was. I was his daughter, the one he came home to when I was a young 2-year-old and took to the park and loved. That night, my mother demanded that I go and find his nitroglycerine pills. She was so frightened and trembling. I stood there for about 20 seconds and could not move. Finally, I got the pills and gave them to my mother. "If he dies tonight, it will be your fault" she said to me. I hear those words over and over when I remember my teenaged years. I have heard myself say them when one of my children or my partner is really angry at me. I think I could die if they don't stop that behavior. I believed her and I didn't believe her at the same time.*

I repeated the sentence, "Fathers, Jewish fathers do not treat their daughters that way" over and over to myself, interspersed with hearing my mother say, "it is your fault if he dies." Hyland, et al. (2013) declared that even after controlling for variables such as mother's age, education, income, race, religion, depression, use of drugs and alcohol, mother-father relationship, and intimate-partner violence, physical punishment occurring more than two times in a month increases the risk of adult psychiatric issues. I believe this relates to the dichotomy that I lived with much of my life. How is it possible for someone who loves another treat that person with meanness or contempt? How does the

person who is being treated in this way tolerate the hurt? Yet, we are all part and parcel of this hurtful behavior. When I want to be with someone, I have the nuclear-strength mistaken belief that I will be the one to make the difference. I will be the special one.

Beatings, fighting, curfews, threats, and harsh words continued even after I finished high school and enrolled for night school at Temple University in Philadelphia. I paid for my own education, and worked at the Warwick Hotel as an assistant to Adele Downey, whose title I do not remember. I didn't think I could move out of the house and certainly didn't think I could go to school and afford to pay for an apartment. The anti-Semitism that was undercover at that time showed up when Ms. Downey took some of us to a conference hall in the Poconos called the Shawnee Inn, which was owned by Fred Waring's family. He was a bandleader and was on television. His group was called Fred Waring and the Pennsylvanians. Although I only found references to rumors that he was anti-Semitic, I had the direct experience of being turned away from his resort (Waring, 1997, p. 248). I can only guess it was because of my name.

However, Adele Downey did not accept this and told the clerk at the registration desk that the entire group would leave if they didn't give me the same courtesy as they did everyone else in the group. I was stunned. I saw her as a hero and understood at that very moment that the circumstances of a situation were just circumstances. The way Ms. Downey responded gave me permanent courage to speak up and not tolerate behaviors that promote racism, prejudice, or threatening behaviors of any nature. This inspiration from Adele Downey and other female managers I have been fortunate to work with was such a gift. I searched Philadelphia obituaries and other information about Adele Downey and have not been successful in my search.

Journal Entry, 1985

It is impossible to be without relationships. I used to sit in my father's chair by the window watching mosquitoes drinking my blood in-between my fingers and wonder if my father would know that I was sitting in his chair. When he was home, no one was allowed to sit in his chair. It faced the newest model TV set, had remote-control device on its arm, and was the centerpiece of our living room. We jumped out of that chair when we heard his car door slam every night at 7 o'clock.

Sitting in that chair gave me feelings of power. I would boss my younger sister around without guilt and with great feelings of calm. As my father walked up the steps, my mother would jump off the sofa and would seem to have dinner appear out of nowhere exactly as he reached the door.

My mother warned me all the time about talking to strange men. She created an aura about men that turned them both into G-ds and unnecessary and frightening objects, both at the same time.

Teenager, Continued

Soon after that experience with Adele Downey, I was out on a triple date and the other two women were dental hygienists. They told me they were looking for a third roommate to move with them to New York City. Three weeks later I was at Penn Station with my suitcase on my way to New York City for an adventure I could never have foreseen. My 19-year-old heart was full of fear and excitement; my mind was open. Nothing could stop me. At least, so I thought. That time (1958) was the height of the beatnik movement, and I was in New York City -- the Big Apple.



Figure 8. Beatnik days in the big city.

One Life

A tender smile, a soft caress, a finger moving slowly,

How can I be denied?

Softness of a warm rhythm

Arms wrapped for motion,

One arm grasping the air holding one life

(Judith Orloff, 1958)

New York City

Most of my life in New York was a wonderful adventure and mystery. I had no doubt I was living the life I was born to live. I had roommates, lovers, interesting people to talk with, a terrific job. I worked for Revlon in the eye makeup department. My boss

was a beautiful woman who taught me so much about business and being a woman. I was really happy. My roommate's grandfather was the editor of *The Daily Worker*, and her parents made furniture and designed fabrics. She drank scotch and milk and her sunglasses she wore in the evenings were white-frame harlequin shaped. I was on top of the world again, even after my abortion and breakup with my lover.

And then, one night while I was preparing dinner for us, I opened the door of our apartment and there was an escaped inmate from Sing Sing, a maximum-security prison. This man knew my roommate from their high school days and had been her boyfriend. He was wearing makeup and had bleached blond hair. He pointed a gun at me. During the time he was in our apartment he threatened me, tied me up, and when my roommate came home he acted out sexually with her. *I didn't feel afraid. I was watching a movie. In fact, a lot of my life at that time was watching a movie. We went in a taxi that morning to the bank to give him our money. We escaped from the taxi and went directly to my roommate's uncle and called the police. From that time until he was caught, the police protected us. We bought them a bottle of scotch after they caught him.*

During the time he was at our apartment, I felt paralyzed and didn't move. I did what he said. He had a gun. I felt like I was in a dream and didn't feel anything akin to fear at that time. After we escaped his threats, I still went to work and acted as if what was happening was just exciting. When I told the story, that is what it was, a story. That time was emotionally very similar to what occurred in my family when I was physically abused. It was how I survived in my family during our fights. I stopped feeling pain. I wasn't afraid of being killed. It was as if it never happened. In fact, as much as I am uncomfortable sharing this, an aspect of the story was exciting. As I have said previously,

it was only when I had my first child that these fears materialized into everyday obsessional thoughts.

Two other incidents in New York were transformational. My father worked in New York during the week and came to dinner at my upper West Side apartment one night along with two students from Columbia. Our dining room table was an ironing board and we sat on the floor eating spaghetti and meatballs, and my father was so wonderful. It was reminiscent of when he came to my apartment after I left the mental hospital and sat on the floor with me and just hung out. *These two friends from my building were at Columbia University and were Cuban. They flew back to Cuba on the night Batista was overthrown. They invited my roommate and me to go back with them. They were so excited. Although I was avant-garde, I was too afraid. Inside this cool chick was a frightened girl who was still bound by middle-class values and a deep desire to be safe.*

I also had an abortion with the first grown-up love of my life because he wouldn't marry me. Again, I left the relationship as if it hadn't happened. Much later on I



Figure 9. Adult Judith and Mother.

realized what happened and how much this affected me. I then met my first husband, got married, and had my first child. Through many difficult circumstances, we moved back to Philadelphia. My ability to function in the everyday world began to suffer. I lost interest in almost everything.

Philadelphia

As a young woman I did not understand or even consider that my behavior would affect my children as they developed. Verny and Kelly (1982) asserted the fetus becomes aware and conscious between 3 to 6 months in the womb. “Research has shown that children who have suffered traumatic events often display impairment in attachment, biology, affect regulation, dissociation, behavioral control, cognition, and self-concept” (Croyle, 2013, p. 1).

This study is significant, as it recounts my personal/experiential account of a young mother suffering from postpartum psychosis, without a diagnosis or anyone to talk with. Also, there was a lack of a social and political justice environment that could support my healing. No one knew what was wrong with me. O’Hara (2009) described postpartum blues as “mood lability, irritability, interpersonal hypersensitivity, insomnia, anxiety, tearfulness, and sometimes elation” (p.1259). O’Hara described the experience of postpartum blues as if he were inside my experience of myself. Karraa (2014) asserted that at the other extreme of these common postpartum blues is postpartum psychosis, which is seen as life threatening and affects one to two women per 1,000 births globally (p. 52). She called this a psychiatric emergency requiring immediate medical attention.

In my eighth month of pregnancy with my second child, I needed an emergency appendectomy, my husband lost his job, his mother died, and we took on raising his

11-year-old sister. When I awoke from the operation, there was a priest at the next bed offering prayers to a woman who had lost two children that morning. He told her she might feel better knowing that because her two children were so young, they would go directly to heaven. This scene in the hospital spontaneously arises when I feel frightened for myself or my family.

At the time, I was terrified listening to the priest. He eventually came to the foot of my bed and asked if I wanted him to pray for me. Again, I felt so afraid, and didn't share what I was really thinking. It has been very difficult for me to share my real thoughts and feelings. It has been easy to share my reactions. "No, I didn't want him to pray for me. After hearing what he told the woman in the next bed who had just lost her two children, I wanted him gone. I don't remember what I said or what I felt. When I think of it now, I am outraged. What audacity! How the hell does he know anything about losing a child. I was 25 years old. I gradually became anorexic and watched my weight plummet. I weighed 98 pounds by the time my daughter was born in January 1964.

From the time of her birth I was obsessed with dying. I didn't see how I could care for two children, myself, and my husband. I stopped eating, except for grapefruit and drank coffee all day long. I didn't have a job in an unfamiliar world. I was living on the outskirts of Philadelphia in a walk-up two-bedroom apartment feeling no connection with my family or husband including my children. Within 4 months I was admitted to a mental hospital in Philadelphia. I was unable to stop thinking that I or one of my children would get hurt, be sick, perhaps get shot by an assassin like John Kennedy. *I believed I was totally responsible for what happened to my children. It would be my fault. I couldn't stand the responsibility. I realize I believed my mother when she told me I could cause my*

father's death by my behavior. I had just transferred that fear from my father to my children.

It was there, in the Philadelphia Psychiatric Hospital, that I "woke up" for the first time and had my initial experience with nonduality. I experienced my heart opening in a spontaneous love I had never felt before, love for everything and everyone. I also experienced that I was an aspect of a connected system of invisible silver and gold strings connecting me with all that is. Although my family thought this was part of the illness that brought me to the hospital, and perhaps they were correct, I was able to access that experience from time to time from that point onward to now.

To speak about that experience in everyday terms: As I mentioned above, Dr. Melvin Marcus was the admitting resident psychiatrist for that evening. I was so deeply and thoroughly frightened; all I could feel was his eyes meeting mine, as I provided my list of "issues" (and there were so many). *I was so suspicious of being judged by everyone and at that moment I couldn't see any change in his expression. He just was there, and I felt so accepted and complete. I knew at that moment, I was all right, no matter what was in store for me.* As I connected with many of the patients I spent time with at the hospital, I didn't see them as crazy. I see all of us as incapable of cutting through the morass of language and judgments we were subject to in our families and communities by ourselves. We need each other.

Horizon House – My Healing Continued

My healing continued as I did not feel so separate from all that I knew, and upon leaving the hospital I applied for a job as a research worker and was hired at Horizon House, a halfway house for mental patients. Horizon House in the late '60s offered me an

alternative to emotional abuse and violence. It offered me a chance to go to “school” as an apprentice and participate in meaningful work.

It was the late '60s. I was 27 years old, separated from my husband, leaving for work early in the morning with most of the fathers from the row house street, on Lankenau Road in Philadelphia. My children were left in the able hands of an au pair to get them off to school, clean the house, go food shopping, and more. I was home by 5:30 each evening, proud to have such a meaningful job, and thought nothing of leaving my children with a caretaker.

I would not say I was selfish or uncaring, I just never thought about it. I had put my pain in a corner of my heart. Again, as in New York, I was living a life I loved. I rarely thought about my childhood at that time. I was so filled with myself. How is it that outwardly I participated in life with such zest after being hospitalized? I had a therapist I was seeing many times a week. I had a mother's helper. I didn't feel like someone's wife and I felt love for my children. I believed by separating from my husband, I was temporarily freed from my feelings of being controlled. I didn't know then how much trauma was living in my cells always ready to see a big rabbit out there to attack me.

Horizon House is where I began to learn my craft. Because we were in the midst of the Vietnam conflict and there was an opportunity for young men to choose alternative service and say they were conscientious objectors, our staff was a connected, anti-war, caring conglomeration of dedicated, idealistic, young people who believed in the Horizon House vision.

Horizon House was founded in 1947 by former hospitalized patients and had been called Fountain House. Fountain House became Horizon House in 1952 and Irvin D. Rutman became the executive director. It was one of the country's first half-way houses for people with chronic psychiatric illness. He was a true pioneer in psychosocial programs. Psychiatric rehabilitation was a relatively new school within the profession. Horizon House allowed paraprofessional people like myself to have hands-on relationships with the newly released patients. There was vocational rehabilitation, group work, life skills, and social skills. Some of the ex-patients made breakfast and lunch each day and cleaned the inside and outside of the building. It was part of the vocational rehabilitation program (Dorfman, 1988, p. 256). In fact, when I had my company, Educational Discoveries, I continued to support the rehab of people in the community by hiring them for the same services. It meant so much to me.

It was there at Horizon House, I, an ex-mental patient myself, was given extraordinary training to work with ex-mental patients. I was given the opportunity to participate in a research project comparing three different modes of treatment (Samuels & Henderson, 1971). The article I co-authored with John Henderson was presented at a behavior therapy conference in Washington, DC in 1971. I trained and worked with some of the masters of that time, such as Irvin Rutman, R. D. Laing, Salvador Minuchin, Marty Seligman, and Jay Haley, to name a few. I also was invited to weekly poker games with these great teachers. Boy, was I feeling special!

While working with Jay Haley, I presented two cases to Dr. Haley, in particular. One concerned a young man who enjoyed wearing women's underwear. Another component of the case was his sexuality, which wasn't clear. Dr. Haley supervised all of

the paraprofessionals in a training program, once a week. He supported me with this client by first asking me to take “John” to a department store. I chose Lord and Taylor located on City Line Avenue in Bala Cynwyd, Pennsylvania, as I shopped there often, and my mother worked there. Once there, I was to go with him to the women’s lingerie section and ask him to pick out the underwear and sleepwear he found appealing. I was very uncomfortable. I didn’t think I could do that. I felt embarrassed and I worried I would see my mother who worked there. I vividly remember doing what I was asked to do. I can see myself walking in the store and, like a zombie, approach the lingerie section. I felt so frightened to go with this young man and publicly be there with him while he did this. I felt sick to my stomach and judged the entire experience.

“What was I doing?” I didn’t have the insight to know what would happen on this assignment. I had never done anything like this before. And, I doubted that Jay Haley knew what he was doing. I also had repulsive feelings and pornographic thoughts of men having sex while pretending they were women. Well, we did it. In fact, he bought some women’s underwear. We left the store without anyone recognizing me. To this day, I am affected by how confused I was and yet willing to follow Haley’s instructions. I appreciate that about myself and it has set a tone for my teaching of nonduality in action.

Much of Haley’s methods at that time came from studying with Salvador Minuchin and his acceptance of Milton Erickson’s work on hypnosis. Erickson asserted, “Whatever the behavior offered by the subjects [sic] it should be accepted and utilized to develop further responsive behavior” (Erickson, 1952/1980 p. 155). This may seem like an unfinished statement. In nonduality accepting what is has no agenda. When I am able to be with the moment just as it presents itself to me, there can be instant

peace or enhanced anxiety. I don't know what to expect. No matter what occurs, I experience a connection with myself that widens the river of life (Fenner, 2007). However, for me, he suggests that we need to accept "what is" for the purposes of what can be. By following the support of Jay Haley without compromise, we utilized Erickson's "confusion" technique. He taught me to accept the person's reality; lead the work with the appropriate pacing; and see "resistance" as a lack of appropriate pacing by the therapist.

I continued to work with John. (a pseudonym) I remember that one day at Lord and Taylor, John was at the counter with his few pairs of women's underwear. We didn't discuss it at that time. I see this experience as a disruption to the conditioning that hid John (a pseudonym), who had armor protecting him from hurt and pain with his family. This therapeutic intervention stayed with me during my career as a psychotherapist and a CEO of a leadership development company. I used this experience in my development of my disruptive learning technology.

As I write this I see the young man being able to bond with his family. In a nondual therapeutic relationship, therapists put themselves in the client's heart and mind, and openly share their own experience at that moment. The therapist knows that thoughts and feelings are not who we are and can share intimately without fear of hurting the relationship. Our norms in terms of LGBTQ are different today. The therapist educates the client to know that thoughts and feelings are not the truth of who we are and can share intimately without fear of hurting the relationship (Fenner, 2007). I wasn't capable of doing that at the time.

My learning after this first opportunity was about me and my deep prejudice about gay men and gay sex. I believed at that time it was against nature and yet my relationship with John was not negative. I did believe homosexuality was “wrong.” I began to discuss these biases with Dr. Haley. He supported me to examine my own family system and see where these communications originated. I began to remember the times I was attracted to girls and how ashamed I felt. As I explored my family system, that shame eventually disappeared and those attractions I had toward women were like a sacred chamber that I had for myself.

The second case I was privileged to work with was with a long-time hospitalized man, most likely in his early '50s, who was sure he would be besieged by germs if he touched anything that touched the floor or a tabletop. He sat in a rocking chair in a corner of the day room. The television was turned on, but he wasn't watching it. Dr. Haley asked me to take him in the staff bathroom and wash my hands the entire time. Again, I didn't understand the reasons for this, but decided to do it without compromise. Haley (1996) asserted that therapists can become interested in ideas rather than the practice of championing change for the client. He was anything but passive in his training of us. And so, I went into the staff bathroom and began to wash my hands over and over again. In my mind was the thought that my hands would get red and be chapped, yet I certainly went ahead with the challenge. I will call the client “Bill.” Bill watched me doing this and was quiet for a while. After about 15 minutes, I clearly remember him telling me he thought I was crazy. When he did, I stopped and dried my hands and took him back to his rocking chair. We were able to laugh about my “craziness” after that. As

with the other client, I don't have a memory of what occurred in our relationship after those particular times.

Laing (1967) theorized that mental illness could be understood as a reaction and result of our struggle with different identities, in effect, a divided self. He didn't believe it was a purely medical disease. Schizophrenia, in particular, was the result of the identity defined for us by our families mixed with our authentic identity. When our experiences of ourselves are fundamentally different from our family's experience of ourselves, our internal landscape becomes fractured. That began for me when my mother told me over and over again that I couldn't possibly be her daughter because I was so fundamentally different from her. Laing (1967) believed love was a force of violence just as the parents of that child were also subjected to this force and their parents before them. This violence force of love was really one of destruction. Laing (1967) declared this initiative was on the whole successful.. In other words, the love children received was not love that was pure. It carried much of their parents' ancestor's negativity with it.

R.D. Laing was invited to Horizon House and offered a 2-day training to the staff in understanding schizophrenia. Being that I had recently been in Philadelphia Psychiatric Hospital and didn't understand what schizophrenia was, I was both afraid of being called out by Laing as "one of them," and deeply grateful for the training opportunity.

Physically, he was bent over a bit, and strikingly handsome. While doing my research on Laing, I discovered that he was an alcoholic, had many children with many different women. and was not a responsible parent. When he introduced himself, he explained he had been diagnosed as schizophrenic. However, I cannot find a source for

this. He was going to take us on a “trip” back to infancy, and during the time we spent going back in time he asked us to remember what that time was like and what our family communicated to us about what was true or false about ourselves and them. We lay on the floor or sat in chairs, whatever we preferred, and followed his instructions. We waited for him to come to us.

The opportunities presented to me at Horizon House taught me to see all people as equal and whole, no matter their outward behavior and diagnoses. My alienation from my family continued. Working with experts and pioneers in the mental health field taught me more than I could integrate at that time. I felt so important and inadequate at the same time. I believe that dichotomy that I just shared shows up in most of my clients and participants. It doesn’t need to be important and inadequate; it can be any competing feelings experienced at the same time.

I saw myself as the title of his book, *The Divided Self* (Laing, 1955). My memory of Dr. Laing is powerful, and because of the 2 days I spent with him I found myself pulled in the early ’80s to become a primal therapist in Burlington, Vermont at the Center for Change.

University of Pennsylvania

I married again, this time to a professor at the University of Pennsylvania. Soon after that I received an offer to work in the President’s Office at the University of Pennsylvania. I decided to leave Horizon House. I knew I wanted to finish college, and by working at the University of Pennsylvania I could take classes at no cost. I began to feel like I needed a degree. Leaving Horizon House was just another continuation of my pattern of skipping out of what I called an insurmountable conflict. Now, looking back, I

see an overwhelmed adolescent, powerless to make a difference and living in an either/or reactive space. After being an executive assistant in the office of the University President, Martin Myerson, I wrote a grant for a new program as part of the Action Agency to be brought to the University of Pennsylvania. I was promoted to the position of the Director of the University Year for Action program, which was VISTA in colleges and universities.

I was no longer an assistant to an administrator; I was the administrator. I taught a class in the Urban Studies Department and the course name was Poverty of the Soul . We read Dickens' *A Tale of Two Cities*, Baldwin's *Notes of a Native Son*, and other articles that the Action Agency recommended for White students who were working in Black neighborhoods. I didn't have a bachelor's degree; I was 29 years old. I certainly did not know how to do this. My support in the office from my manager was outstanding. He trusted me to do the right thing, and if I didn't he taught me how to decide what the correct action was for me to take. Fortunately for me, the skills I had learned at Horizon House gave me the ability to understand diverse people. I was given the personal and professional freedom to work with students from all over the country. Again, just as at Horizon House, I was awarded opportunities that seemed like a gift from the Divine. Also, I took courses in the afternoon and early evening that moved me further academically.

Vermont – A New Reality

My move to Vermont was precipitated by my second divorce, a new man, and the invitation by Steward LaCasce to partner with him and a few others to create a college without walls in Burlington, VT. It was 1972, and it was called *Vermont College of*

Community Involvement (VICI). It became Burlington College, received its accreditation, and was a haven for students and faculty who would not fit in a traditional college environment. How thrilling it was for me to be part of an alternative college. I was 33 years old, divorced twice, and was beginning a new life in Vermont with my children and my new partner. And, he was an extraordinary man. I wasn't able to be intimate personally at that time. I wanted other people to behave in a way that created a good feeling in me, albeit temporary. I felt good. I didn't have the language or awareness to open up in a relationship or accept love the way it was given. I hadn't the experience of being loved just for the sake of being loved. I still believed I could start a new life by leaving my old one behind. I wasn't aware that circumstances were not the only contender for decision making. I didn't know that love had to start inside of me in order for it to be shared with another person. It was only a few years ago I asked for his forgiveness for how I behaved. He gave it.

The first class I taught at VICI was called Male and Female in a Sexist Society. My students were a motley crew including prison guards, dropouts from traditional colleges, and veterans. Having taught a year at University of Pennsylvania in the University Year for Action Program, I had some confidence and was full of myself. I look back and find it inconceivable that I co-taught a course with a biologist which consisted of readings, poetry, wading in creeks and rivers, exploring life, and using Gestalt methods of therapy to pull it together. I wouldn't do that now. I knew very little about biology and Gestalt therapy. How could I have mustered up the willingness to do this? After teaching these classes, and my participation in all that I could, I became the director of counselling services, which catapulted my career as a psychotherapist. I

remember meeting with students in a small office and calling on the wisdom of Laing, Rutman, Minuchin, and Haley. I thought I was invincible. I certainly was a tough cookie. I didn't understand how important it was for me to become self-aware with a meditation or other spiritual practice to learn more about myself. Yet, even with my self-doubt I added value to Burlington College and the students. I received my MEd in 1976 from University of Vermont with a focus on learning. Ever since that time, I was always curious as to how people learn.

I left VICI after a serious confrontation with the president of the college and the board. I learned that I went too far in my perspective and my negativity. During that period of my life, I believed my opinions to be the truth. I remember sitting on a bench with the other faculty who agreed with me waiting for the board meeting to end. It ended poorly and all of us were hurt and embarrassed. Many of us felt obliged to resign our position. I feel so sad writing about this. I still feel the pain of my behavior and how it affected others. With self-awareness and a nondual perspective, I would have had the courage to meet privately with the president and share my concerns. I know we would have worked it out. I was invited to give the convocation speech for Burlington College September 2015. The school closed suddenly in May 2016.

I became a member of a primal therapy collective called Center for Change. I left my partner again and remarried again. My new third husband was one of the therapists at the Center for Change. I knew before the wedding I was making a horrible mistake. I felt like I couldn't help myself. He had two small children who lived with us every 2 weeks for 2 weeks. I grew to love the children. After we divorced, I moved again, this time to San Diego. When I look at my professional development, it is at odds with my

personal growth. I learned what Laing was teaching back at Horizon House from a professional perspective, and after I learned the primal therapy methodology at Center for Change, I started my own company with six other people: three psychotherapists, a massage/energy therapist, and a psychiatrist. We called our enterprise The Carriage House Associates in Health. I bought a building with a gift of \$2,000 from my mother and became the president of our practice.

This was the mid-'80s. We did workshops where people experienced their past lives. I didn't have that experience. We focused on women's and men's workshops and saw ourselves as the pioneers of innovative therapy practices where the therapist is no longer separate and objective but rather is in relationship with the clients. We began a training program and trained students to be therapists. We gave them a certificate at the end of their training. We were not licensed to be a school. We didn't care and neither did the students.

I had married one of the psychotherapists I met at the Center for Change. After a few years this marriage also began to fall apart. My husband had become addicted to cocaine and I knew I needed to divorce him. Again, I saw my only option was to move somewhere new. Why did this continue to occur? *Why did I have to leave every job, marriage, relationship? I did not leave my children.* However, up to this time, they needed to continue to change schools, move with me to wherever I was going. *I knew this way of being with my children was wrong. Why did I do it? I continued to put myself first and my fears continued. Why was I still afraid at night? Why? Why? Why?* My children were almost on their own at this time. I do remember thinking how the responsibility for their life fell to me. Their father remarried, moved to Maine, and had two more children.

I was torn as a mother. I needed to work; I needed to take care of my children; I wanted to be free and I wasn't. This responsibility still eats at me. I still feel obligated to my children. I cannot see myself untying those strings.

A Fly Cries

My experience in Vermont with a spider and a fly has had a lasting effect on me and is as powerful as when I took acid and felt the earth move. I lived in a log cabin in Vermont for many years. *One night, during a forever snowstorm, while reading on a cushion-covered window seat, I heard screaming. I was alone in the cabin and became so frightened I could not feel the distance from my mind to my body. Who could be screaming like that? I could not breathe, thinking an intruder was in the cabin and imagining the scream came from me. It was not. It was coming from a fly that was being killed by a spider.* Shelomi (2014) asserted that flies do scream. I did not know that was possible. As I looked up from my window seat, I saw a spider killing a fly that had fallen into the web she had spun. *In that scream, I could not tell the difference between a child, a grown-up intruder, or a fly. How can that be? I could not tell the difference. How could we not be connected?* My deep knowing that we are one has informed each choice I made. Hearing a fly cry haunts me even today. I was offered an opportunity to go into business with four very successful men. My job would be teaching seminars. I left Vermont as soon as I could. At this point my daughter was at University of Colorado and my son was at Tampa University. I was really free to leave. The only consideration I had was I had been a stepmother to my husband's two children. I still left. I still cannot tolerate my leaving them. Saving my own life was all I could think of at that time.

Reality

When reality begins to float into darkness

Try to find a cold wall.

It takes you all the way back and forces you to heal the call of the night

Voices pounding in your brains,

crying

wailing

sickness

death

Drums pounding in your ears that are sensitive even to the note of the violin that is too

high for us to fathom.

Bodies full of passion – drained

Hearts filled with love – stained

And minds--minds wanting peace – how do they find it? – through death – or brave life?

*Can you face ignorance, or must you continue to float – Try – oh God, please try to come
down.*

(Judith Orloff, sometime in the '60s or '70s)

San Diego - A Paragraph

Before I moved to Colorado I spent a few years in San Diego after leaving my third husband. My business partner and her husband and small child also moved with me. This was an opportunity for me to lead workshops in the United States and Australia. We began our company, Choices International, Inc. We received help from successful

businessmen who put money and time into our company. Being personally in demand, and having people want to study with me led me to crash emotionally over and over again. I felt so lonely. I was about 46 years old. I still lived with my fear of being alone, my fear of having an escaped killer find me and slash and rape me. Sometimes I believed my mind and at other times I did not. I traveled to Hawaii, Colorado, back to Vermont, and did programs in California. I met wonderful people and didn't take a moment to sit quietly to discover what was going on in my inner landscape. *I still didn't know that I was projecting on my students; I still didn't know how wounded I was. I loved being in California and living this wonderful life, even though each evening, when alone, I was frightened. We all lived together for a while. When they went off as a family for the weekend, I was hurt that I wasn't asked and behaved just like my mom. I locked and checked the doors and windows over and over. Why couldn't I heal my fear?*

I cannot describe why I kept leaving where I was, except I was on a treadmill that began when I was 19 and felt if I didn't get out of my house, I would die. In my final move from California to Colorado, my business partner believed California was going to experience a serious earthquake. She felt she had to leave. I didn't feel I could stay without her. I did not believe I could be really successful by myself. I believed I needed a team. I believe we all need a team to create synergy and success. I didn't know that I did not have a free choice in all my leavings. Rather it was either leave a man, a place, a situation, or else something horrible would happen.



Figure 10. Choices International in local newspaper.

Move to Colorado

I moved to Colorado and merged our company, Choices International, a personal growth company, with a company that presented seminars on financial literacy. We merged with Quantum Education and created Educational Discoveries. We became a leading financial literacy and leadership development company. This move was made with my best friend and business partner and her family, the same person who moved to San Diego with me.

I won't describe the joys and sorrows of Educational Discoveries in this study. Perhaps, at a later time, I will write an article on growing your company with Emotional Intelligence. We were able to grow Educational Discoveries enough to have an annual revenue of \$8,000,000 and we sold it to Provant for \$8,500,000. I survived an internal coup and became president of the company. At one point we employed 75 people (including contractors) and worked with Fortune 100 companies offering leadership development franchise wide programs. Some of our clients were Bank of America and Fleet Bank, Caterpillar, Ernst and Young, AT&T, and many more. On 9/11, the wife of one of the founders was on a plane that crashed in Pennsylvania. The training industry collapsed, and our stock went from \$26 a share to 11 cents. My retirement move was no more.



Figure 11. Marriage to my soulmate.

How can I do this? How can I marry a man so young? I feel so crazy. I feel pulled and pushed to do this. I am so in love. So in love. In 1989 I married my fourth and final husband/partner and we adopted a Cambodian orphan directly from an orphanage when he was 3 1/2 years old. He was deaf, mute, and had brain damage from banging his head over and over again to relieve his emotional pain. That journey will be the subject of a memoir that I will write after I complete this EdD.

My time in Colorado from 2000, when we sold Educational Discoveries, until 2016 when I enrolled at Fielding Graduate University has been one of the most creative periods of my life. My career has been transformational as to the nature of what I have learned about myself, how change occurs, and how this can be shared with individuals, families, and organizations. Based on the applicability of my research, these change models can serve as a theory of change for early childhood trauma.

Discovering Nonduality and Peter Fenner

Selling Educational Discoveries directed me to nonduality. I didn't have to work for a few years and didn't know what was next. *I knew something was not quite right in*

my work. I found that I had an agenda with my students and clients. I didn't understand that resistance was an integral dimension to the process of healing. I thought I did. I also asked questions that were really not part of the healing process. I notice others doing the same thing now. I knew I wanted to be clear when I worked with people. I found myself attempting to influence their decision without telling them. I didn't feel good about this.

I went to an evening introduction to a Peter Fenner workshop called Radiant Mind. I felt so uncomfortable. He didn't speak for minutes at a time and everyone sat patiently waiting for something to happen. When people did speak they sounded like parrots on a spiritual search. I asked him what it was like for him with so many people wanting his approval. He just laughed. He didn't answer me. I didn't know I could not answer someone. I felt so obligated. I always needed an answer for everything. I signed up that evening. Thus began my journey into unconditioned awareness.

Some of my teachers were Peter Fenner, Candace O'Denver, Adyashanta, Rupert Spiro, and many others. I also read as many books as I could digest about this ancient practice. Yet the books felt like I was eating the menu and not the food. And, many of the teachers, although wonderful to be with, didn't include working with feelings. I wanted a spiritual life that included feelings. I was told over and over again that wasn't possible. Unconditioned awareness was devoid of phenomenon. I wanted the relative experience of being Judith Orloff while having the nonduality experience of unconditioned awareness at the same time. I set out to discover if that was possible. I met other nondual practitioners while I was presenting papers at the Science and Nonduality Conferences in 2011, 2012, and 2013. I focused on nondual relationships, was a founding board member

of the Center for Nondual Awareness, and served on a panel to discuss nonduality at the California Institute of Integral Studies in 2012. We called our not-for-profit Radical Love Foundation, and we called our profit company Radical Choices. The following is a letter from one of the participants in one of our Radical Love workshops:

Sometimes that concept of Radical Love gets lost and to find it again on almost the last day of 2015 was such a blessing and wonderful way to start 2016. Thank you Judith for your teachings and your love. I will never forget when we first met at the workshop in San Francisco and how that workshop led me to consider marriage differently. Marrying...has brought my life to a place of fullness, trust and deeply feeling held. May health and happiness abound in your life and those you love in the coming year and always.

Since 9/11, I have wanted to create something that could deeply influence people from all cultures and points of view. I felt sheer joy when I *made a difference in another's life. When that didn't happen, I looked more deeply at myself. Was I pretending to know and trying too hard instead of just being present? Was I caught in my own trauma and not aware of it? Was I wanting power? Was I competing?* Those personal questions are a taste of my process of inquiry.

I believe this study detailing these transformational experiences through the lens of a woman in the mid 20th century, together with early childhood trauma and our cultural and social mores, can offer new insights into results leading to resilience sustainability. I also wish that nonduality may be considered as a possible foundational inquiry for those who read this.

Pure logical thinking cannot yield us any knowledge of the empirical world; all knowledge of reality starts from experience and ends in it. (Einstein (1954, p.271)

The Last Twelve Years, 2006-2018

Being a student, studying with masters in their field, being willing to learn and not know everything, being comfortable with not knowing, realizing the way I look at the world informs my ability to love. Letting go of holding myself and others hostage. These have been some of my challenges during this time.

(Notes from a journal during Peter Fenner Training, Judith Orloff)

These last 12 years were instrumental in leading me to Fielding and auto-ethnography. Once I had an experience of unconditioned awareness that was not drug inspired and I could experience nondual awareness regularly, I knew others could also. I was in a healing process and wanted to be with others in the same process for themselves. My spiritual practice of nondual living began in 2007 working with Peter Fenner in his Radiant Mind programs. I accomplished quite a bit. I co-created a nondual community. I co-founded the Radical Love Foundation, a non-profit organization that offered programs in Awakened Relationships and Natural Leadership. I ate, slept, and was married to this search for my own healing. I was financially supported in this venture, and I was able to offer these to people of dissimilar backgrounds. We also created a for-profit company, Radical Choices, which attempted to be a business, but to no avail. It literally was too much for me to accomplish, and I wouldn't stop and take stock of what was true. Looking back at my documents, files, papers, and programs, it is obvious we were the research and development arm of a company that didn't exist. I wasn't clear on my own

expression of the principles I was teaching. This became all that mattered for a time. How can nonduality be experienced while having feelings when there are no phenomena? I am still testing and working with that question. If, in fact, I can show the possibility of nonduality in action, then I am optimistic this model can be used for healing early childhood trauma.

With the support of our team and participants in these classes, we were granted a patent for a six-step experiential learning process which we called Disruptive Learning Technology (p. 166), ? a Nondual Action Based Mindfulness process. What a mouthful! That was the name we gave it so we could be taken seriously. Although this was personally rewarding, I wanted to be able to reach larger groups. I want to leave a legacy with my work. Although I could physically stop, I was unable to rest. I feel very fortunate to be able to satisfy my being with leaving this legacy.

I decided to study the work of scholar/practitioners and others in the field of education and psychology. I focused on finding a doctoral program where I could learn the skills and methods to present my work as a unified whole and discover if and how nonduality works as a core healing modality. And I did. I enrolled in Fielding Graduate University's EdD program, in August 2016.

My Healing

Some of the themes that run through my life are: my four legal marriages; my two other partnerships; life as a single mother of two birth children; my time in Philadelphia Psychiatric Center; my adoption of a child from a culture I knew nothing about; my professional and personal successes and failures; my personal healing of my

childhood trauma; and my desire to learn and teach and to make a difference in people's lives including my own.

Again, I draw our attention to the state of women in the mid-1950s and the lack of medical research into post-partum conditions. I also draw our attention to how physical and verbal abuse received as a child can have predictable future results no less serious than sexual abuse. If we were able to prevent or stop this through an educational model, our society would be healthier, and, therefore, more productive. Teaching children who and what they really are could give them more opportunities to experience unconditioned awareness and greater relaxation.

I wanted freedom from my demons more than I wanted comfort. My journey up to now has brought me experiences that propel me to continue on my path while staying embodied in my everyday life as a wife/partner, mother, grandmother, student, practitioner, friend, and scholar.

My early childhood trauma dissolves a bit more each moment I am present, here. I can rest in unconditioned awareness. I test and retest the six principles to be sure of their accuracy on myself and my few clients. Uncovering memories during this year and a half has been a key catalyst in this journey. Writing down my experiences and finding sources that support or have a different perspective has been a primary opening of the armor that has surrounded my heart. Perhaps a seventh principle could be added which has to do with keeping a journal during your healing process.

EXIST

*Heart stop beating, mind stop thinking, You're not supposed to be alive. Lips,
stop trembling, Eyes, stop seeing, You're not supposed to be alive.*

EXIST, EXIST, EXIST

Control emotion keep it penned as a prisoner in a cell.

Hide compassion, be an actor, never, never, yell.

EXIST, EXIST, EXIST

(Judith Orloff, Sometime in the 60's Philadelphia)

CHAPTER FIVE – NOW WHAT? CONCLUSIONS AND RECOMMENDATIONS

We now know that memories are not fixed or frozen like Proust's jars of preserves in a larder, but are transformed, disassembled, reassembled, and recategorized with every act of recollection.
(Sacks, 2013. *Hallucinations*, p.154)

My dissertation is a panoply of memories which appeared in my consciousness as threads and patterns of a self-woven tapestry which focused on the research question of the dissertation: How does the use of nonduality as a therapeutic tool assist in the amelioration of early childhood trauma? The memories that I experienced occurred when I sat quietly, took walks, paced, cried, fought with myself, and felt serene joy. I have been researching and writing for a year and a half. I listened to my mind remind me of trauma, childhood, parents, family and then, out of nowhere, resilience. When I shared some memories with my sister, her experience of those moments was different from mine. I felt frightened when she didn't believe me. I wondered if I made up my memories.

Writing from memory created a re-experiencing and re-conceptualizing of the different person I was/am/will be. While retrieving the memories, and reliving them, I experienced a disharmony in how I lived and still feel that at times today. Creating a clear tapestry of early childhood abuse was difficult. I stitched and restitched until I could read it aloud and ask my committee to read it. Many days during this time. I wished I had pursued a different type of study such as in-depth interviewing.

Writing a dissertation dealing with personal early childhood trauma revealed I am courageous, and I can persevere. There were moments during these past years at Fielding

when I wasn't sure. For example, my recovery from a knee replacement accounted for more pain than I had anticipated. I lost 2 months of writing and thinking. Another example is the experience of isolation that coincided with the completion of my classes--where I had contact with other students--to the pure aloneness that came with daily writing. I also discovered how much I wanted approval before I would go on. Who would give it to me? I tried my family, friends, and even strangers to no avail. No one wanted to listen to a paragraph or two without the context that created this work. I paced and walked and realized this was my contribution. I needed to own it. Eventually I did.

Memories

If this dissertation had a different focus, such as the paranormal, I would share different memories, such as

- Discovering I had lung cancer through a dream
- Having visitations by my father and friends as they die – showing up in a vision and leaving me the message that all is okay
- Seeing what might be called ghosts at different times

These events in my life have always been part of my story. Yet during the dissertation time, these memories did not appear as relevant. So, memories, from my vantage point of this present time are called to the surface of my consciousness when needed, when searched for, upon awakening in the morning, after a good cry or laughter, and they are not necessarily factually true. Tulving (1983) argued that “access to, or actualization from, the episodic memory system tends to be deliberate and usually

requires conscious effort” (p. 46). Mandler (1985) agreed, yet conceded that “much of everyday memory experiences are in fact spontaneous” (pp. 102–103). Post-traumatic stress-based memories tend to arise seemingly out of nowhere (Rasmussen & Berntsen, 2011). And, although my memories were the same, my narratives and stories from the past were experienced differently based on my ability to abide in unconditioned awareness when I was writing them.

Suggestions for Future Research

One of my recommendations is to conduct further research and study to determine whether early childhood trauma memories occur similarly as PTSD spontaneous memories. I believe not only do these early childhood memories occur spontaneously, but the aftermath of the experiences over time easily determines the future behavior of the person experiencing the memories. And for me, my clients, and students, there does not seem to be any apparent circumstance that might bring this spontaneous memory to the surface at that time. In other words, we cannot generalize as to what circumstance brings the memory to the surface. This merits research that goes beyond the circumstance to the core experience of the trauma while gently moving through what I called Disruptive Learning Process, and now call Nonduality in Action, I can say anecdotally, and generally speaking, I have seen this work with clients and organizations. . I have not done any formal research at this time; that research is a future interest and desire.

The American Psychological Association has suggested further research on memories of abuse to determine how accurate or inaccurate recollections of events may be created. This includes:

- Research to decide which clinical techniques are most likely to lead to the creation of pseudo memories, and which techniques are most effective in creating the conditions under which actual events of childhood abuse can be remembered with accuracy;
- Research to determine how trauma and traumatic response impact the memory process;
- Research to learn which people are more susceptible than others to memory suggestions. (<https://www.apa.org/topics/trauma/memories>)

Although these seem like worthy endeavors, I do not see how this research can make a positive difference in the healing process. However, I do expect researchers to use the data in their continued search for truth. Without the self-awareness that nonduality and self-awareness trained practitioners bring to the healing process, the outcomes would most likely continue to be devoid of the clarity that comes from unconditioned awareness (the practitioner) meeting unconditioned awareness (the client).

The worst part of holding the memories is not the pain. It's the loneliness of it. Memories need to be shared. (Lowry, 1993, p.193)

Implications for Practice

Fenner (2007) declared the only experience two people can absolutely share is unconditioned awareness, that of nonduality. After 12 years of study and practice, my team and I created a six-step process that promises to achieve the experience of nonduality in action for most people who practice it. This process is not intended to achieve the deepest self-healing that nondual practitioners like Judith Blackstone, Peter Fenner, Brian Thériault, and Rupert Spiro aspire to. However, by following the steps and doing the assignments offered, this nonduality in action practice becomes a way of living. Over time, new behaviors become the norm and the benefits to both the practitioner and the client increase. The Kabbalah teaches that with any spiritual practice and meditation, action for the benefit of others is the most beneficial to the actor (Rader & Rader, 1989).

Another recommendation is that elementary school teachers and administrators include curriculum to address quieting the mind of children who are overwhelmed by the demands placed on them at this time. This could be in the form of teaching kindness, compassion and other results of nonduality. I will also look at this at a later date.

Nonduality in Action Process: Dissolving Trauma



Figure 12. Six steps to nonduality in action.

Definition of the six steps in the nonduality in action process

I will define the steps of this process and then will give an example of how it works to achieve a nonduality in action way to live.

Insight: being mindful of where our beliefs, values, and points of view originated, without believing our truth is the truth for others. Conscious knowledge of one's feelings, motives, and desires.

Personal Responsibility: knowing that you are the owner of your thinking, emotions, behaviors and mindset. It means that you are responsible to choose your behaviors from unlimited options. Simply put, it is the key to creating desired results, and, therefore, to success.

Authenticity: being genuine, whole, and connected. Having a presence that is experienced by others as congruent.

Integrity: keeping and honoring our commitments no matter what we think or feel. Keeping our word is simply doing what we say. If we cannot keep our word, we let the other person know and make sure he or she is complete.

Clear Mind: occurs when our insight, personal responsibility, and integrity are occurring simultaneously. At that time, there is a free choice, not one made out of a reactive type of response. There is more than an either/or alternative to any situation.

Awakened Action: allows us to know what actions to take. Awakened action points to truth, and is extremely flexible and contradictory. Each situation is unique, and each person is unique. Only when we are relying on a clear mind can we use awakened action. Clarity informs and points us to what makes sense in each moment with each situation.

The nonduality in action process does not attempt to go into the depths of internal transformation described by Nixon (2013). It is meant to touch the traumatic experience lightly while reaching deeply, simultaneously allowing the student or client to attain self-discovery without provoking another reopening of the trauma. This distinction between self-discovery and provoking the original trauma depends on what and who brings about the “aha.”

Below is a pictorial diagram (Figure 13) of the process I just described. Although this process is taught in orderly steps with accompanying “exercises” so that the student or client can experience the different steps in the process, the result for the student is systemic.

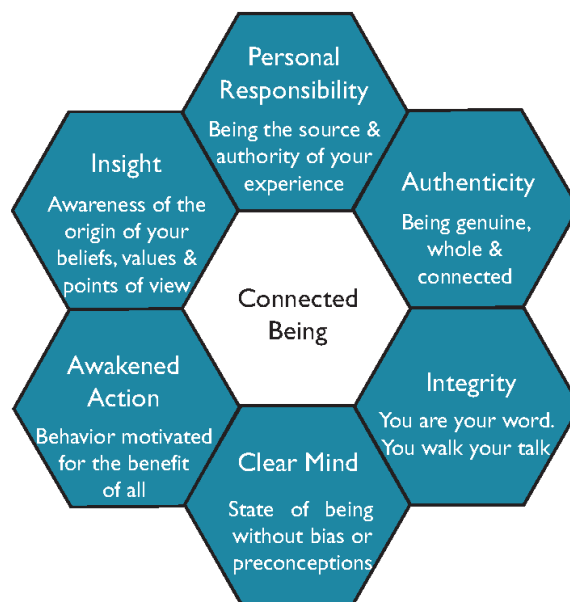


Figure 13. Another view of six-step process.

These principles began to make sense and became clear as I studied with Peter Fenner as a participant in his training. I noticed that many of the students in the programs could meditate for hours until they experienced unconditioned awareness or nonduality. I

could not. After a while, I needed to take my insight and take action. I followed the Six-Step Process without thinking about the steps.

. This process has created shifts in individual and group worldviews from fixed static positions to a fluid, worry-free state that dissolves harmful recurring thoughts. Over time, this worry-free state becomes more natural and easier to access. Even if the thoughts do not go away, they are recognized as debris needing to be swept away..

As these principles are followed, they create new habits and a new internal map. This includes living a life with a vision of profound health, allowing early childhood trauma to be released and dissolve. This occurs when each step is followed until it is obvious that awakened action is required. The person can then view him/herself positively. The choices s/he makes are chosen anew from a myriad of options rather than conditioning.

Example of Nonduality in Action

I will go through one example of the first two principles and the assignments that bring about self-awareness:

Insight: Session 1: I met with a client, a family, through videoconferencing. They began to tell me their story. I listened until something they say just stuck in my mind and I was unable to move forward. For example, in a meeting with a couple within the past 6 months, one of the partners had an affair with someone he worked with. When his wife said she knew something was going on and confronted him with it, he denied everything. So, I heard the issue of trust rather than the affair. She knew he was not telling the truth, yet she did not believe herself.

Why was that important in my six steps? It told me that both partners needed to have insight as to their behavior that created the affair. I know that sounds unusual to say. Yet, I see the couple as a system, and each action in the system causes the entire system to shift. Although the content of an affair can be titillating, exciting, and sad, I believe the foundation of a partnership, particularly in marriage, is trust in oneself and one's partner.

I gave them an assignment which was to notice when they are not being honest all the way. Only notice. They were not to make any changes in their behavior.

Insight: Session 2: He shared he was afraid to tell his wife his truth about almost everything. He reported he felt left out and criticized when he did share. She shared how angry he was with her when she asked questions about him and other women. He also said his family did not speak about anything personal. I asked them if they saw how they were active participants in his affair. He said yes and she said no. I gave them another assignment. I asked her to notice how much time she spent thinking about the other woman. Before we ended the call, she said she could not help thinking about the other woman. It is mostly what she thought about when not working or being with their children (two teens who knew about the affair). I suggested she was keeping this affair going. He had blocked the other woman from his social media accounts and the wife continued to monitor the other woman's social media. I suggested another assignment which was they not discuss content when they shared with each other. Instead of saying, "I felt sad because you..." I asked them to just say they felt sad. They agreed to this. They reported they both saw how they were responding from their history and gave many

examples. I noticed they were not angry with each other at the end of the meeting. I forwarded the recording of their meeting for them to review during the week.

Personal Responsibility: Session 3: The wife was angry at her husband. The woman he was having an affair with passed her on the street and “was brazen and didn’t even look away.” Her husband felt shame and realized he has felt an undercurrent of shame since a child. The wife did not find a connection from her history and what was happening for her now. Judith: *How do I let this be? How do I not tell her what I see? How do I support her to see for herself?* I recommended she meet with the ex-lover and let her know how she felt. When I suggested that, she expressed anger and began to share her history admitting this happened to her mother and father. In the middle of her sharing, her husband gave her some feedback through holding her hand and looking at her tenderly. I gave them the assignment to continue not sharing content but continue just sharing feelings. They let me know they were going away for a few weeks and felt much better about each other. At this point they each saw how they contributed to the affair.

I recognize this will take more than six meetings. I let them know we have reached our third principle: authenticity. If the work continues as it has up to this point, they will see how their early childhood trauma precipitated the affair, the discovery of the affair, the obsessions on both sides, and how the affair is secondary to the lack of trust they have recently made conscious. I do not expect them to live a non-duality lifestyle.

I do expect them to retain what this opportunity brought them and perhaps I will see them at another time in the future. Neither one had ever been to a therapist. They appreciated

my honesty, my “not beating around the bush,” and the openness we shared. I hope this gives a flavor of my work at this time.

Final Reflection

This autoethnographic study has been revealing both in going deep into early childhood trauma while understanding my personal truth about my own trauma. I am a resilient survivor of consistent physical and emotional abuse as a young teen. During this process, I realized as well how much my parents loved me. I am able to see how the medication my father was prescribed for his pains and heart attack changed him and my mother, as did my misdiagnosis of mental illness that put me in a mental hospital change me. Understanding the toll on my mother and my father shifts my perception of the past, the present, and my continued dreams for the future. I no longer feel like an abused child. The subtlety and nuances of a person’s life bring richness to their stories and their lives.

Giorgio (2009) stated, “The call of narrative is the inspiration to find language that is adequate to the obscurity and darkness of experience” (p.270). I believe we need to shine our light of awareness on our painful memories and begin to dispel the darkness that I feel/felt. The memories that arose and found their place in this study were the ones that made meaning of how and why my life was a mixture of pain and inspiration. My memories wove this narrative into a meaningful fabric for my life as well as the lives of others. I pray social and cultural healing can be garnered from my experiences as they touch many others.

In living a nondual lifestyle there is the capacity of new beginnings and the lessening of our negative ancestral consciousness. In writing this dissertation, I have found the memories that were diffused are those I focused on. New memories are already

asking for attention. My father helping my friends. My mother showing kindness to me without questioning my motivations. This brings new ideas for papers and work. As I continue my work, other memories will show up and those are the ones that will diminish as they are explored in their core. In that way my childhood trauma will continue to gradually dissipate.

I am appreciative of Fielding Graduate University and my amazing committee for supporting this work and for their concern with social justice and inclusion. I finish this endeavor in a state of grace and appreciation.

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